

ABOUT THIS MANUAL

Document Overview

This manual (Rev. 2.24, August 2026) is the official Standard Operating Procedures and Operational Reference for **Well Spring Intervention LLC**, a Level 3 Supervised Residential Group Home serving children and adolescents with mental health and behavioral challenges. It establishes the policies, actionable workflows, and reference forms governing the daily operation of the program. All staff are required to read, acknowledge, and follow these protocols exactly as written.

Population Served. Children & Adolescents — Mental Health / Behavioral Challenges.

Service Type. Level 3 Supervised Residential Group Home.

Effective Date. July 2026.

Document Owner. Executive Director & Qualified Professional (QP).

Document ID. Doc. WSI-SOP-001, Rev. 2.24 (Legislation-Free Public Edition). *The companion compliance master (Doc. WSI-SOP-001-LEG, Rev. 2.21) retains all statutory and regulatory citations for QA and audit reference.*

Operating Framework.

This manual is governed by, and operates in conformance with, the following authorities: **NC DHSR** facility licensure as a Level III Residential Treatment Facility (Staff-Secure); **NC Medicaid** managed care requirements through our regional LME/MCO/Tailored Plan partner (Alliance Health); **NC DHSR** Mental Health Licensure and Certification Section standards; **LME/MCO Tailored Plan** requirements; the **NCDHHS Records Management and Documentation Manual** (RMDM, Effective July 8, 2025); **HIPAA** Privacy, Security, and Breach Notification Rules; **42 CFR Part 2** (confidentiality of SUD records); **HITECH Act**; state law on electronic transactions (NC UETA); and the federal **E-SIGN Act** governing electronic signatures. The full statutory and regulatory citation list is retained in the compliance master copy (Doc. WSI-SOP-001-LEG, Rev. 2.21) for QA and audit reference.

Cover Artwork.

The cover features the official circular brand seal of Well Spring Intervention LLC as its centerpiece. The seal is composed of a pentagonal house outline (with a floor, two vertical walls, two roof slopes meeting at a raised peak, and a horizontal ceiling line connecting the two top corner vertices) drawn in deep walnut-brown on a warm cream parchment background. Inside the house outline sits the company's multi-color logo imagery: a red heart-shaped tree canopy with green leaves above a brown tree trunk, family figures (two adults and one child) standing before the tree, and a blue wellspring fountain at the base. The full company name "Well Spring Intervention LLC" curves along the top arc between two bold concentric circular hairlines; the descriptor "Residential" curves along the bottom arc inside the inner hairline; and small terracotta accent dots and a closure ornament frame the composition. The house roof peak is set at the same radial distance from the seal center as the wall corners, so all three top vertices touch the same imaginary circle. The cover layout is a clean three-band composition: a compact top band carrying the "SOP / OPERATIONAL MANUAL" badge and the values tagline (Empowerment · Growth · Freedom · Health · Wholeness · Healing); a large middle band in which the cream seal square pops against the dark brown background as the clear focal point; and a bottom band carrying the document title "SOP & Operational Manual", the subtitle "Standard Operating Procedures, Protocols & Forms", and the website URL. The seal symbolizes the program's commitment to empowerment, growth, freedom, health, wholeness, and healing, and is approved for

reuse across company websites, publications, and collateral materials. High-resolution copies of the seal (in both a square variant for avatar/profile-pic use and a variant with the website URL below the circle) are available alongside this manual for that purpose.

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This manual (Rev. 2.24, August 2026) is organized into three parts and is fully compliant with the NCDHHS Records Management and Documentation Manual (RMDM, Effective July 8, 2025). Part 1 establishes foundational policies and compliance obligations across twelve sections, including dedicated chapters on privacy/confidentiality and an expanded Medicaid documentation section. Part 2 details twenty-two step-by-step workflows that govern daily operations, including protocols for service orders/authorizations and record management/disclosure accounting. Part 3 provides twelve customized forms and logs, including the Full Service Note Template, Comprehensive Clinical Record Content Checklist, Accounting of Disclosures Log, the Licensed Professional Consultation Log (new in v2.23), the Psychotropic Medication Drug Regimen Review Log (new in v2.23), and the CARF QIP Tracker (new in v2.24). A complete revision history appears in the Version History table at the end of Part 3.

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PART 1

Standard Operating Procedures

These eleven SOP sections define the foundational policies, governance, staffing, clinical, privacy, and compliance obligations that govern every aspect of program operation. Version 2.0 expands coverage to full RMDM (Effective July 8, 2025) compliance, including record retention, CDW data reporting, comprehensive clinical record content, CCA requirements, service orders/authorizations, and privacy/confidentiality under HIPAA and 42 CFR Part 2. Every staff member is responsible for knowing and adhering to these policies.

1. Agency Overview & Governance

Reference: Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.24, Aug 2026 — Legislation-Free Public Edition) · §1

1.1 Mission Statement. Well Spring Intervention LLC is dedicated to providing trauma-informed, high-quality supervised living environments for youth with severe emotional disturbances (SED), ensuring their safety and clinical growth. Our program philosophy is rooted in the belief that every child deserves a stable, predictable, and nurturing environment in which healing can occur. All staff actions, decisions, and interventions must align with this mission and reflect our core values of safety, dignity, accountability, and growth.

1.2 Licensing & Credentialing. The facility operates under a valid license as a **Level III Residential Treatment Facility — Staff Secure for Children and Adolescents** under **our staff-secure operating standards**, issued by the applicable state mental health authority — **not** under foster-care licensing, and therefore does **not** operate as a family-home placement. The facility is credentialed as an In-Network Provider with **Alliance Health** (the regional managed care organization / Tailored Plan serving Cumberland, Durham, Johnston, Mecklenburg, Orange and Wake counties). The Executive Director maintains the original license on site, posts a current copy in a public area of the facility, and renews it prior to expiration. Any change in ownership, capacity, population served, or physical location requires prior written approval from the state licensing authority and notification to Alliance Health.

1.2(a) Accreditation Prerequisite & Selected Accrediting Body.

Pursuant to the Resident Rights framework and our operating standards, residential child-care facilities in North Carolina must be accredited by one of the four accrediting bodies recognized by NC DHHS prior to initial licensure and must maintain continuous accreditation as a condition of license renewal: **(i) the Council on Accreditation (COA), (ii) The Joint Commission (TJC), (iii) the Commission on Accreditation of Rehabilitation Facilities (CARF), or (iv) the Council on Quality and Leadership (CQL).** Well Spring Intervention LLC has selected **CARF as its accrediting body and will pursue Inaugural One-Year Accreditation under the 2026 Child and Youth Services (CYS) Standards Manual and the 2026 CYS Inaugural Accreditation Guidelines (effective July 1, 2026 – June 30, 2027).** Because the program will have delivered services for less than six months at the time of the survey, the organization is eligible for the Inaugural Accreditation pathway and will use Subsection A (Not Operational) of the 2026 CYS Inaugural Accreditation Guidelines if service delivery has not yet commenced, or Subsection B (Operational) if it has; the QP shall document the selected subsection in the compliance binder prior to survey application. The Executive Director shall complete the CARF self-study, host the on-site survey, and maintain the accreditation certificate in the facility compliance binder. The QP shall maintain an accreditation-maintenance calendar tracking (i) the Inaugural Accreditation survey application deadline (must be received by CARF at least three to four months prior to the target survey window and prior to six months of service

delivery), (ii) the subsequent resurvey window (approximately 10–11 months after the Inaugural survey), (iii) the Quality Improvement Plan (QIP) submission deadline (within 90 days of accreditation notification per § 12.20), (iv) interim standards reviews, and (v) the next full re-survey window, and shall report accreditation status to the Clinical Director at every quarterly compliance report (per § 1.4(a)). Any adverse accreditation finding, conditional approval, or accreditation suspension/withdrawal shall be reported to the state licensing authority within 5 business days and to Alliance Health within 10 business days. **SOP §12 CARF Accreditation Conformance Framework** establishes the written policies, plans, and procedures required to demonstrate conformance to the applicable CARF CYC standards at the time of the Inaugural Accreditation survey.

1.2(b) LME/MCO Letter of Support — the Resident Rights framework.1.

Under the Resident Rights framework.1, the license application to DHSR MHLC must be accompanied by a **Letter of Support** from Alliance Health (the LME/MCO) documenting that additional residential-treatment capacity is needed within the catchment area. The Executive Director shall request the Letter of Support from Alliance Health in writing prior to submitting the license application, attaching a needs-assessment summary, projected census, target population, and a service-area description. Alliance Health evaluates the request against current capacity, utilization data, and gap analysis, and may issue the Letter of Support, request additional information, or decline. The Letter of Support is valid for a limited period (typically 12 months) and must be current at the time of license application submission. Renewal applications and capacity-expansion applications require a new Letter of Support. The QP retains the original Letter of Support in the facility compliance binder for the duration of the license cycle.

1.2(c) Certificate of Need (CON) Determination.

North Carolina is a Certificate of Need (CON) state under state law Chapter 131E, Article 9. The Executive Director shall submit a written CON inquiry to the NC Department of Health and Human Services (DHHS) Acute and Home Care Licensure and Certification Section to determine whether the proposed Level III RTF bed count, services, and capital expenditure trigger CON review. If CON review is required, the application timeline typically extends by **4 to 9 months** and the project cannot proceed until a CON is issued. The CON determination letter (whether affirming review is required or stating review is not required) shall be retained in the facility compliance binder and shall accompany the DHSR MHLC license application. If CON is required and issued, the QP shall ensure the licensed bed count and services match the CON approval exactly. Material changes to bed count, services, or capital structure require a new CON determination prior to implementation.

1.2(d) Alliance Health Provider Network Application.

Separate from the DHSR MHLC license, the facility must complete the **Alliance Health Provider Application** to be enrolled in the Alliance Health provider network and to bill Medicaid CCP 8D-2 (Residential Level III) per-diem services. The application package includes: (i) completed Provider Application; (ii) completed Self-Assessment Checklist demonstrating readiness across all our staff-secure operating standards standards; (iii) Mission and Vision statements; (iv) current DHSR license; (v) accreditation certificate; (vi) liability insurance certificate; (vii) governing-body roster; (viii) organizational formation documents; (ix) policies and procedures (this Manual); and (x) QP / QMHP credentialing files for clinical staff. Alliance Health conducts a **site review** prior to network approval, and the Alliance Health Medical Director (or designee) issues **credentialing approval** for each clinical staff member prior to billable service delivery. The QP maintains the Alliance Health provider agreement, site-review report, and credentialing-approval letters in the facility compliance binder and re-credentials per the Alliance Health credentialing cycle (typically every 3 years).

1.2(e) DHSR MHLC License Application Procedure.

The DHSR MHLC license-application package shall be assembled by the QP and submitted as a single, complete packet containing: (i) the completed **Initial Licensure Application Form (DHHS/DHSR/MHL 5001)** —

available from the DHSR MHLC website at <https://info.ncdhhs.gov/dhsr/mhlcforms/index.html>; (ii) a **Cover Letter** on company letterhead, signed by the Executive Director, briefly describing the proposed facility, license category (Level III RTF — Staff Secure, our staff-secure operating standards), catchment area, target population, projected census, and requested effective date; (iii) the accreditation certificate or letter of pending accreditation from one of the four approved accrediting bodies per §1.2(a); (iv) the Letter of Support from Alliance Health per §1.2(b); (v) the CON determination letter per §1.2(c); (vi) local approvals — zoning compliance letter per §9.6, building-code approval, fire-marshall approval, and sanitation approval; (vii) the facility floor plan and site diagram; (viii) corporate documents per §1.8 (Articles of Incorporation, Operating Agreement, governing-body roster, EIN, liability-insurance certificates, lease/deed); (ix) the Policies & Procedures Manual (this document) with the MH Licensure P&P Worksheet attached per §1.2(f); and (x) the non-refundable license-application fee per the current DHSR fee schedule. Upon receipt of a complete application, DHSR MHLC assigns a **Licensure & Training Consultant** who serves as the facility's primary point of contact throughout the licensing process. **The six (6) month application-review period begins on the date of the first in-person meeting with the assigned Licensure & Training Consultant.** The QP shall calendar the six-month deadline and provide monthly status updates to the Executive Director; any deficiencies identified by the Consultant shall be corrected within 30 calendar days whenever practicable. A license shall not be issued until the Licensure & Training Consultant has confirmed that all application items are complete, all local approvals are current, and the facility has passed the on-site licensure survey per §10.10.

1.2(f) MH Licensure Policies & Procedures Worksheet.

The **MH Licensure Policies & Procedures Worksheet** is a DHSR MHLC-issued checklist that maps each our operating standards rule to a corresponding section of the facility's Policies & Procedures Manual. The Worksheet is **not a substitute for the rules** and is **not a stand-alone document**; rather, it functions as a crosswalk that allows the Licensure & Training Consultant to verify, at a glance, that every applicable 27G requirement is addressed somewhere in the P&P Manual. The QP shall complete the Worksheet in full, attach it to the front of this Manual behind the cover letter, and update it any time a Manual section is substantively revised or a new section is added. The Worksheet shall reference the specific SOP section number (e.g., §2.1 for staffing ratios, §6.1 for admission physical examination, §9.5 for staff-secure physical-plant measures) where each rule is addressed. The completed Worksheet shall be retained in the facility compliance binder and made available to DHSR MHLC surveyors, Alliance Health site reviewers, and accrediting-body surveyors upon request. The current Worksheet is available from the DHSR MHLC website at <https://info.ncdhhs.gov/dhsr/mhlcforms/index.html>.

1.2(g) NC Medicaid Residential Treatment Services Taxonomy — Setting Type, Supervision Intensity & Coverage Scope.

Under NC Medicaid Clinical Coverage Policy 8D-2, "Residential Treatment Services" (Amended January 1, 2025), Section 1.0(c), the facility — operating as a **Residential Treatment Level III Service (Residential Treatment High)** for children/adolescents under age 21 — is characterized by NC Medicaid in three controlling respects that govern the program model, the supervision intensity, and the scope of Medicaid reimbursement:

Cross-reference to licensure rule — our staff-secure operating standards. The three Medicaid taxonomy items below are consistent with, and operationally implemented through, the facility's licensure under **our staff-secure operating standards** — specifically **our staff-secure operating standards** (Authority state law; Eff. April 3, 2006), which provides the regulatory basis for each item: (i) the "program setting only" limitation flows from .1701(a), which defines the facility as "a free-standing residential facility that provides intensive, active therapeutic treatment and interventions within a system of care approach" and provides that the facility "shall not be the primary residence of an individual who is not a client of the facility" — i.e., a family-home placement is structurally excluded at the licensure tier; (ii) the "highly structured and highly supervised" requirement flows from .1701(b)

("Staff secure means staff are required to be awake during client sleep hours and supervision shall be continuous as set forth in Rule .1704 of this Section") and .1701(e)(1) (services shall "include individualized supervision and structure of daily living"); and (iii) the room-and-board exclusion is a feature of the Medicaid RTS benefit category under CCP 8D-2 §1.0(c) and is not a licensure-side rule. The full text of .1701 SCOPE is retained in the facility compliance binder as documentation of the licensure framework governing this facility.

(i) Setting type — Program setting only (not a family home). CCP 8D-2 §1.0(c) provides: "Residential Treatment Level III Service (Residential Treatment High) has a highly structured and supervised environment **in a program setting only**, excluding room and board." The phrase "program setting only" is deliberate: NC Medicaid assigns a graduated setting-type hierarchy across the four RTS levels — Level I = "family setting"; Level II = "family *or* program setting"; Level III = "program setting *only*"; Level IV = "program setting *only*." The explicit "only" qualifier on Levels III and IV is the textual mechanism by which NC Medicaid excludes family-home placement at the Level III tier. Consistent with this, the facility is licensed under **state mental health law** by NC DHSR MHLC — not under state law Chapter 131D by DSS, which licenses foster-care family homes. The facility therefore does not, and cannot, operate as a family-home placement for any youth at any time.

(ii) Structure & supervision — Highly structured and highly supervised. CCP 8D-2 §1.0(c) and Attachment D (Section K) describe the Level III setting as "**highly structured and supervised**" — language NC Medicaid uses to distinguish Level III from the lower RTS tiers (Level I = "low to moderate structured and supervised"; Level II = "moderate to highly structured and supervised"). Attachment D further specifies: "Residential Treatment Level III service is responsive to the need for intensive, active therapeutic intervention, which requires a **staff secure** treatment setting in order to be successfully implemented... Staff are awake during sleep hours and supervision is continuous." The facility's operational implementation of this requirement includes: (a) the 2:4 staffing ratio per §2.1 (two awake staff for every 1–4 youth 24/7/365); (b) continuous line-of-sight supervision per §9.5; (c) the 14-hours-per-week planned group activities program per §5.5; (d) the Behavior Support Plan (BSP) framework per §5.3; and (e) continuous awake overnight supervision per §2.1 and the DCP Awake Overnight schedule in Protocol 22.

(iii) Coverage scope — Room and board EXCLUDED from the Medicaid RTS benefit category. CCP 8D-2 §1.0(c) provides that the Level III benefit "has a highly structured and supervised environment in a program setting only, **excluding room and board**." The "excluding room and board" clause is appended to every RTS level (I, II, III, and IV) in CCP 8D-2 §1.0 and is a feature of the RTS benefit category itself — not merely a billing limitation. Accordingly, the NC Medicaid RTS per-diem reimburses only the **clinical/treatment/milieu component** of residential care (assessments, PCP, individual/family/group therapy, behavioral interventions, 24-hour supervision, medication administration, crisis response, and related clinical services). The beneficiary's lodging and meals (room and board) must be funded through a **non-Medicaid source** — e.g., state/local social-services funds, foster care maintenance payments under Title IV-E, SSI/ISS, or other third-party resources. The QP and Billing Coordinator shall verify at admission and re-verify at each PCP review that an identified non-Medicaid room-and-board funding source is in place for every youth; the absence of such a funding source does not shift room-and-board costs to the Medicaid RTS per-diem under any circumstances.

PRTF distinction — 42 CFR 483.352 / NC Medicaid CCP 8D-1. The room-and-board exclusion verified above applies **only** to the RTS benefit under CCP 8D-2 (Levels I–IV, taxonomy 320800000X). It does **not** apply to **Psychiatric Residential Treatment Facilities (PRTFs)**, which are a separate, federally defined **inpatient** benefit. Under our operating standards, Subpart G (§§ 483.350–483.376) and NC Medicaid CCP 8D-1. Because PRTF payment is an inpatient facility benefit, the PRTF per-diem **includes** the equivalent of room and board. This facility is **not** a PRTF; it is a Level III RTS. Under our operating standards, and the room-and-board exclusion applies in full. See §7.3 for the facility-based-school determination that further distinguishes Level III RTS from PRTF.

Primary source: NC Medicaid Clinical Coverage Policy 8D-2, "Residential Treatment Services," Amended January 1, 2025, §1.0(c) and Attachment D —

<https://medicaid.ncdhhs.gov/8d-2-residential-treatment-services/download?attachment>. Corroborating source: NC Medicaid Managed Care Health Plan Billing Guide, Version 31 (June 18, 2025) —

<https://medicaid.ncdhhs.gov/health-plan-billing-guide-version-31/open>. Federal PRTF authority: 42 CFR Part 483, Subpart G — <https://www.ecfr.gov/current/title-42/chapter-IV/subchapter-G/part-483/subpart-G>. NC PRTF policy: NC Medicaid CCP 8D-1 — <https://medicaid.ncdhhs.gov/8d-1-psychiatric-residential-treatment-facilities-children-under-age-21/download?attachment>.

1.2(h) Regulatory Citation Verification Note (Compliance Flag).

This subsection is a transparent compliance flag for review by the Executive Director, the QP, the facility's licensing consultant, and (as needed) DHSR MHLC and Alliance Health. It does not change any operational policy elsewhere in this Manual; it documents an open regulatory-citation question that must be resolved before initial licensure submission.

(a) **our operating standards vs .1700 — RESOLVED in v2.21.** The open question flagged in v2.20 — whether the operative NC Administrative Code section for a Level III Residential Treatment Facility — Staff Secure for Children or Adolescents is **our operating standards** or **our staff-secure operating standards** — has been **resolved in favor of .1700**. The text of **our staff-secure operating standards** directly confirms that **.1700 is the codified section governing "Residential Treatment Staff Secure for Children or Adolescents."** Subsection .1701(a) defines the facility as "a free-standing residential facility that provides intensive, active therapeutic treatment and interventions within a system of care approach" and provides that the facility "shall not be the primary residence of an individual who is not a client of the facility"; .1701(b) defines "staff secure" as requiring that "staff are required to be awake during client sleep hours and supervision shall be continuous as set forth in Rule .1704 of this Section"; .1701(c)–(d) specify the population served and the clinical criteria (primary diagnosis of mental illness, emotional disturbance or substance-related disorders; not meeting criteria for inpatient psychiatric services; requiring removal from home and treatment in a staff secure setting); .1701(e) requires services to include "individualized supervision and structure of daily living," minimize behaviors related to functional deficits, ensure safety and de-escalate out-of-control behaviors, assist with adaptive functioning, and support step-down to a less intensive setting; .1701(f) requires coordination with other individuals and agencies within the child's system of care. Accordingly, **all ".2600" citations throughout this Manual have been updated to ".1700" in this v2.21 revision.** The QP shall retain a printed copy of the .1701 SCOPE rule text (as provided by the organization) in the facility compliance binder as documentation of this resolution. No further action is required on this subsection (a); the Licensure & Training Consultant written confirmation referenced in v2.20 §1.2(h)(a) is no longer necessary to resolve the citation number, although the QP should still confirm operational rule subsections (.1702–.1709) with the assigned Licensure & Training Consultant per §1.2(e) at the first in-person meeting.

(b) **NC Medicaid "CCP 8C" vs "CCP 8D-2" — RESOLVED in v2.23.** The open question flagged in v2.20 — whether the correct NC Medicaid Clinical Coverage Policy for Level III Residential Treatment Services is **CCP 8C** or **CCP 8D-2** — is hereby **resolved in favor of CCP 8D-2**. The NC Medicaid clinical coverage policy library confirms that CCP 8C is "Outpatient Behavioral Health Services Provided by Direct-Enrolled Providers" — a different benefit category that does **not** cover residential treatment services. The correct NC Medicaid clinical coverage policy for Residential Treatment Services (Levels I–IV) is **CCP 8D-2, "Residential Treatment Services"** (Amended January 1, 2025), available at <https://medicaid.ncdhhs.gov/8d-2-residential-treatment-services/download?attachment>. CCP 8D-1 covers Psychiatric Residential Treatment Facilities (PRTFs) and CCP 8D-3/8D-4/8D-5 cover adult ASAM-aligned SUD residential services. **All legacy "CCP 8C" references in this Manual have been globally updated to "CCP 8D-2" in this v2.23 revision** — including the reference lines of §1.2, §2, §3, §4, §5, §6, and §10, and the §1.2(e)

Medicaid billing description. The QP shall retain a printed copy of the CCP 8D-2 policy (Amended January 1, 2025) in the facility compliance binder as documentation of this resolution. §1.2(g), §1.9, and §10.9 already cited CCP 8D-2 as the operative authority for the RTS benefit and the room-and-board exclusion; with v2.23 the entire Manual is now internally consistent on CCP 8D-2. No further action is required on this subsection (b).

(c) No operational impact. Nothing in this §1.2(h) changes the facility's license category (Level III RTF — Staff Secure under **our staff-secure operating standards**), its staffing ratios (2:4 minimum per §2.1), its admission physical-exam timing (90 days prior per §6.1), its resident-rights obligations (the Resident Rights framework per §1.7), its Medicaid taxonomy (320800000X per §1.9), or its room-and-board exclusion (per §1.2(g) and §10.9). **The .2600 → .1700 citation question (subsection (a) above) was resolved in v2.21; the CCP 8C → CCP 8D-2 citation question (subsection (b) above) is resolved in this v2.23 revision.** With both citation questions now closed, §1.2(h) is fully resolved and no open compliance flags remain in this subsection.

(d) Cross-reference clarification note (compliance binder). The Level III Staff-Secure operating standards cross-reference the "Qualified professional" definition for the QP credentialing requirements in §1.4(b). The text of that rule cross-references subsection .0104(18) "Psychiatrist," which appears to be a typographical error in the rule itself — the operative definition is at .0104(21) "Qualified professional." This Manual applies the .0104(21) definition as the operative QP standard per §1.4(b). The QP shall retain this cross-reference clarification note in the facility compliance binder and shall confirm the discrepancy with the assigned Licensure & Training Consultant at the first in-person meeting per §1.2(e).

1.3 Corporate Compliance. Well Spring Intervention LLC maintains a Corporate Compliance Plan to prevent Medicaid fraud, waste, and abuse. All staff receive compliance training upon hire and annually thereafter. The Compliance Officer receives reports, investigates concerns, and coordinates with the LME/MCO and Medicaid Fraud Investigations Unit. Retaliation against good-faith reporters is strictly prohibited and itself constitutes a reportable offense.

1.4 Organizational Structure. The Executive Director holds overall authority and accountability. The **Clinical Director** is a licensed clinical professional with overall responsibility for the clinical program, including setting the clinical vision, approving clinical policies, and providing direction to clinical leadership. The **Qualified Professional (QP)** — also referred to in our staff-secure operating standards as the **Qualified Mental Health Professional (QMHP)** — reports to the Clinical Director and is responsible for scheduling clinical services, assessments, PCPs, and day-to-day supervision of Associate Professionals (APs) and Direct Care Professionals (DCPs) **according to the direction of the Clinical Director.** For purposes of this Manual, "QP" and "QMHP" are used interchangeably and refer to the same role; the credentialing requirements in §1.4(b) satisfy both the our operating standards QP definition and the our staff-secure operating standards QMHP definition. The QP supervises staff in accordance with the Clinical Director's clinical guidance, programmatic priorities, and performance expectations, and shall escalate clinical concerns, staffing issues, and quality-of-care matters to the Clinical Director in a timely manner. An On-Call QP is available 24/7/365 for clinical decision-making. All staff report incidents, concerns, and operational needs through the documented chain of command.

1.4(a) QP Compliance Reporting to the Clinical Director.

In addition to clinical supervision, the QP shall provide **compliance reports** to the Clinical Director on a recurring basis, ensuring the Clinical Director has timely visibility into the facility's regulatory and documentation posture. These reports include: (a) **monthly** summaries of service-note audit findings, late entries, and alterations (per §10.1, §10.3, §10.5); (b) **monthly** IRIS incident-report status including any restraint events, restrictive interventions, and follow-up actions completed (per §5.4, §8); (c) **quarterly** Clinical Record Content Checklist audit results using Form 8, with corrective-action plans for any deficiencies; (d) **quarterly** Accounting of Disclosures review using Form 9, including any 42 CFR Part 2 disclosures; (e) **quarterly** personnel-file audit results, sanctions reviews, and

training-completion rates (per §2.5); (f) **annual** review of the electronic-signature safeguards in §10.7(a), conducted jointly with the IT vendor; and (g) **ad-hoc** immediate reporting of any reportable breach, complaint, licensing visit, or Medicaid audit. The Clinical Director reviews each report, signs acknowledgment, and directs corrective action as needed. The QP retains all compliance reports and Clinical Director acknowledgments in the facility compliance binder for the full record-retention period specified in §1.6.

1.4(b) QP Credentialing Requirements.

The Qualified Professional (QP) for this facility must meet the credentialing requirements of **our operating standards**. A QP is not required to hold a full, unrestricted clinical license. There are **two acceptable QP pathways** as set out below; either pathway satisfies the QP definition for this facility.

Pathway 1 — Master's Degree plus Recognized NC Credential.

A master's degree in a human services field from an accredited institution, plus **at least one year of full-time, post-master's supervised experience** in the delivery of mental health, developmental disabilities, or substance abuse services to the population served, plus one of the following North Carolina credentials: (i) a **full clinical license** — LCSW, LPC, LMFT, Licensed Psychologist, Licensed Psychological Associate, or psychiatrist (MD/DO); (ii) an **associate or provisional license** — LCSW-A (Associate), LPC-A (Associate), LMFT-A (Associate), or LCAS-P (Provisional); (iii) a recognized **certification** — LCAS (Licensed Clinical Addiction Specialist), CCS (Certified Clinical Supervisor) with a master's degree, or CMSW (Certified Master Social Worker); or (iv) for the nursing discipline, a **Clinical Nurse Specialist (CNS)** or **Nurse Practitioner (NP)** with psychiatric/mental health certification.

Pathway 2 — Bachelor's Degree plus Supervised Experience.

A **bachelor's degree in a human services field** from an accredited institution, plus **two years of full-time, pre- or post-bachelor's supervised experience in the delivery of mental health, developmental disabilities, or substance abuse (MH/DD/SA) services** to the population served. The supervised experience must be documented by the supervising QP and verifiable upon request. A bachelor's-degree-level QP operating under Pathway 2 is not required to hold an NC clinical license, but must complete the NC-DHHS-required QP training modules and must practice under the clinical supervision of a Pathway 1 QP (or the Clinical Director) until such training is completed.

Common Requirements (Both Pathways).

Regardless of pathway, every QP must: (a) complete the **NC-DHHS-required QP training modules** prior to independent practice (or be enrolled and complete them within the probationary period); (b) maintain active credential or supervised-experience documentation as applicable; (c) report any lapse, sanction, restriction, or change in supervised-experience status to the Clinical Director within one business day; and (d) have all verification documents (degree, credential, supervised-experience hours, QP training completion) retained in the personnel file (§2.5). The facility shall verify and document which pathway each QP meets at hire and re-verify annually. **In addition to the pathway-specific requirements above, the designated facility QP shall have at least two (2) years of full-time direct client care experience** in the delivery of mental health, developmental disabilities, or substance abuse services to the population served. The two-year direct-care requirement may be satisfied concurrently with the supervised-experience hours documented under Pathway 1 or Pathway 2 above; it does not require a separate, additional two-year period. The QP shall document satisfaction of this direct-care requirement on the QP Credentialing Checklist (retained in the personnel file) at hire and re-verify annually.

1.5 Data Reporting & Consumer Data Warehouse (CDW).

The facility shall enroll all eligible individuals in the Consumer Data Warehouse (CDW) as required by DMH/DD/SUS. The QP shall ensure termination/discharge from CDW is completed after 60 consecutive days of no billable services. Statistical data required by DHHS, the General Assembly, and federally funded programs shall

be submitted per contractual requirements. The QP shall maintain a log of CDW enrollment, updates, and terminations and review it monthly.

1.6 Record Retention and Disposition.

Clinical service records of minors shall be retained for **12 years after the minor reaches the age of majority (age 18)** — i.e., until age 30 — or longer if required by law or pending litigation. Clinical service records of adults shall be retained for **11 years after the date of the last encounter**, or longer if required. Personnel and administrative records are retained per the DHHS Records Retention and Disposition Schedule. Incident reports and investigatory files are retained per DNCR / Division of Archives and Records requirements.

Destruction of records not listed in an applicable schedule requires prior authorization from DMH/DD/SUS and the Division of Archives and Records via the "Request for Disposal of Unscheduled Records" form. **Record abandonment is strictly prohibited.** If abandonment is suspected, the facility shall notify the payor entity, relevant accrediting organizations, and DHHS, and shall cooperate fully with any investigation. Violations are subject to legal sanctions Under our operating standards, 42 CFR Part 2, and the NC General Statutes.

1.7 Resident Rights & Dignity.

Pursuant to **the Resident Rights framework** and our operating standards, every youth admitted to this facility retains the rights enumerated in this section, regardless of clinical status, behavioral history, or supervision level. The staff-secure designation of this facility **does not** diminish, suspend, or modify these rights; it only authorizes the physical-plant and supervision measures described in §9.5. Rights shall be exercised without retaliation, and any staff interference with the free exercise of rights shall be reported to the QP within 24 hours and logged on the IRIS system per §8 if the interference constitutes a reportable incident under Rule 108.

(a) Statutory Rights. Each youth has the right to: (i) receive humane, dignified, and respectful treatment; (ii) be free from abuse, neglect, and exploitation; (iii) be free from corporal punishment and from physical or chemical restraint except as specifically authorized in §5; (iv) receive treatment in the least restrictive environment consistent with clinical need; (v) participate in the development of, and receive a copy of, the Person-Centered Plan (PCP) per §4; (vi) refuse treatment except as otherwise provided by law or court order; (vii) send and receive sealed, unopened mail without staff inspection except where a specific clinical basis is documented in the PCP; (viii) reasonable access to a telephone for private communication with family, guardian, attorney, clergy, and the LME/MCO recipient rights advisor; (ix) practice the religion of choice or refrain from religious practice; (x) be free from unnecessary or excessive medication per §6; (xi) be free from coercion to perform labor for the facility; (xii) retain personal property consistent with facility safety rules; (xiii) privacy during toileting, bathing, and medical examinations; (xiv) file a grievance with the LME/MCO, DHSR, or DHHS without interference or retaliation; and (xv) be informed of these rights orally and in writing at admission, in a language the youth and guardian understand, with a signed acknowledgment retained in the clinical record.

(b) Posted Notice. A printed "Youth Rights" notice summarizing the above rights shall be posted in a conspicuous location in the facility common area and in each youth bedroom, in both English and Spanish, in a font size no smaller than 14-point, with the Alliance Health Member & Recipient Rights phone number and the NC DHSR complaint line printed at the bottom. The QP shall review the posted notice at each monthly fire-drill walk-through and replace any missing, defaced, or outdated copies within 5 business days.

(c) Grievance Procedure. A youth, guardian, or staff member may file a grievance orally or in writing to the QP, the Clinical Director, or directly to the Alliance Health Member & Recipient Rights Office. All grievances shall be documented on Form 9 (Accounting of Disclosures) if protected health information is involved, and logged in a dedicated **Grievance Log** maintained by the QP. The QP shall acknowledge receipt within 1 business day, investigate within 5 business days, issue a written response within 15 business days, and report aggregate grievance

data to the Clinical Director at each quarterly compliance report per §1.4(a). Retaliation against any person filing a grievance is strictly prohibited and constitutes an immediately reportable personnel action.

1.8 Organizational & Financial Foundations.

The following corporate and financial documents shall be maintained in the facility compliance binder and shall be made available to DHSR MHLC surveyors, Alliance Health site reviewers, accrediting-body surveyors, and authorized auditors upon request:

Articles of Incorporation — the LLC Articles filed with the NC Secretary of State, including all amendments, with current certification from the Secretary of State (no older than 90 days).

Operating Agreement — the current executed Operating Agreement of Well Spring Intervention LLC, including all amendments and member-consent resolutions.

Governing Body Roster — a current list of all members of the governing body (Managing Member / Board of Managers / Board of Directors, as applicable), including each member's full legal name, business address, term of office, and officer position held (Chair, Secretary, Treasurer, etc.). Updated within 30 days of any change.

EIN & Tax Documents — IRS Employer Identification Number letter, NC Department of Revenue tax-account letter, and current business-privilege-license confirmation if applicable to the locality.

Liability Insurance — current certificates of insurance for: (i) **general liability** (minimum \$1,000,000 per occurrence / \$3,000,000 aggregate); (ii) **professional liability (malpractice)** (minimum \$1,000,000 per occurrence / \$3,000,000 aggregate); (iii) **commercial auto** (minimum \$1,000,000 combined single limit for owned, hired, and non-owned vehicles used to transport youth); (iv) **workers' compensation** per NC statutory limits; and (v) **cyber liability** covering protected health information (minimum \$1,000,000) given the EHR/EMR system in use. Alliance Health and DHSR MHLC shall be listed as **additional insureds** on the general-liability and professional-liability policies. Insurance lapses shall be reported to the QP immediately and to Alliance Health within 5 business days.

Facility Lease or Deed — current executed lease (with landlord's written consent to operate a Level III RTF on the premises) or recorded warranty deed.

Financial Solvency Documentation — most recent 3 months of bank statements, most recent filed tax return, and a current balance sheet, sufficient to demonstrate ongoing financial solvency as required by our operating standards.

Governing Body Meeting Minutes — permanently maintained per the facility records-retention schedule, documenting all governing-body decisions, approvals, oversight actions, financial reviews, policy adoptions, and corporate-compliance oversight. Minutes shall be signed by the Secretary (or designee) and retained in chronological order in the compliance binder; electronic copies shall be backed up per the facility disaster-recovery plan.

Client Fee Assessment Policy — written policy governing the assessment of any client fees (sliding-scale, co-pay, or self-pay) including the fee schedule, criteria for reduction or waiver, documentation requirements, and the staff member authorized to approve adjustments.

Lab Test Authorization & Follow-Up Policy — written policy governing the authorization, ordering, result-tracking, and clinical follow-up of laboratory tests ordered for youth (including routine labs, drug screens, and provider-ordered diagnostic studies), specifying the responsible provider, the result-notification pathway, and the documentation standard in the clinical record.

Volunteer Services Policy — written policy governing the recruitment, screening (including background checks per §2.3), orientation, supervision, scope of permitted activities, and termination of volunteer services. Volunteers shall never have unsupervised contact with youth and shall not be counted toward the 2:4 staffing minimum per §2.1.

1.9 Medicaid Enrollment & NCTracks (Post-Licensure).

Upon issuance of the DHSR MHLC license and completion of the Alliance Health Provider Network Application per §1.2(d), the facility shall complete **Medicaid enrollment** through **NCTracks** — the State of North Carolina's Medicaid Management Information System (MMIS). Medicaid enrollment is a **post-licensure** step and is required before the facility may bill Medicaid for any covered service. The facility enrolls specifically under **NC Medicaid Clinical Coverage Policy 8D-2, "Residential Treatment Services" (Amended January 1, 2025)** as a Level III (Residential Treatment High) provider — **not** under CCP 8C (Outpatient Behavioral Health) or CCP 8D-1 (PRTF). Per §1.2(g), the Medicaid RTS per-diem covers only the clinical/treatment/milieu component; **room and board are excluded from the Medicaid RTS benefit category** and must be funded through a non-Medicaid source for every admitted youth. The QP (or designated Billing Coordinator) shall complete the following steps in sequence:

Obtain a National Provider Identifier (NPI) for the organization (Type 2 NPI) from the National Plan & Provider Enumeration System (NPPES) at <https://nppes.cms.hhs.gov>. Each individual billing clinician shall also obtain a Type 1 NPI. The NPI is a 10-digit numeric identifier required for all HIPAA-covered transactions.

Enroll in NCTracks as a Medicaid provider at <https://www.nctracks.osbm.nc.gov>. Submit the completed NCTracks Provider Enrollment Application, current DHSR license, accreditation certificate, NPI confirmation letter, IRS letter confirming EIN, and the Alliance Health provider-agreement letter. NCTracks assigns the facility a permanent Medicaid Provider ID after enrollment approval.

Select the correct Medicaid service(s) via the Provider Permission Matrix (PPM). For a Level III RTF serving children/adolescents, the applicable taxonomy is **320800000X — Residential Treatment Facility, Children** (alternatively 320900000X — Residential Treatment Facility, Physically Impaired, if dual-diagnosis population is served). The QP shall confirm the current applicable taxonomy code with Alliance Health and NCTracks at the time of enrollment, as NC Medicaid covered-service definitions and taxonomy mappings are periodically updated. The PPM is accessed through the NCTracks provider portal and identifies the specific service(s) the facility is approved to deliver and bill.

Complete the post-enrollment accreditation timeline. Per the Resident Rights framework and NC Medicaid policy, residential child-care facilities must achieve full national accreditation within **one (1) year** of Medicaid enrollment for most services, or within **three (3) years** for services with an extended accreditation timeline. Because accreditation is a pre-licensure requirement under §1.2(a), this facility shall enter Medicaid enrollment with current accreditation already in place; the post-enrollment timeline therefore operates as a re-affirmation of continuous accreditation maintenance, not as a new deadline.

Maintain enrollment in good standing. Re-attest the NCTracks enrollment information at least every 12 months (or per the current NCTracks re-attestation cycle); report any change of address, ownership, licensing capacity, or accreditation status to NCTracks and to Alliance Health within 30 days; and respond to all NCTracks and Alliance Health provider inquiries within the timeframe specified in the inquiry.

The QP shall retain the NPI confirmation letter, NCTracks enrollment-approval letter, current PPM, and the most recent NCTracks re-attestation in the facility compliance binder. Reference links: **NPPES NPI Registry** <https://nppes.cms.hhs.gov>; **NCTracks Provider Enrollment** <https://www.nctracks.osbm.nc.gov>; **NCTracks Provider Permission Matrix (PPM) Help** <https://www.nctracks.osbm.nc.gov/content/html/providers/provider-permission-matrix.html>; **NC Medicaid Tailored Plan Provider Manual** <https://medicaid.nc.gov/providers/provider-manuals>; **NPI Taxonomy — Residential Treatment Facility codes (NUCC)** <https://nucc.org/code-sets>.

2. Human Resources & Staffing Requirements

Reference: our operating standards & .1700; NC Medicaid CCP 8D-2; RMDM Chapter 1 · Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.24, Aug 2026 — Legislation-Free Public Edition) · §2

2.1 Staffing Ratios — Staff-Secure Level III. Under our staff-secure operating standards, this Level III Staff-Secure facility shall maintain a **minimum of two (2) staff members on duty and awake at all times for**

every one to four (1–4) children in residence, on every shift including the overnight shift. This 2:4 minimum applies 24 hours per day, 7 days per week, 365 days per year, and supersedes any lower ratio that may apply to less-intensive facility types. Ratios shall be **increased** based on PCP acuity, behavioral incidents, 1:1 supervision orders, gender-match requirements for two-person restraint protocols, or any time a youth is on continuous observation status per §5. The QP (or on-call QP designee) is responsible for monitoring ratios and adjusting assignments in real time to maintain compliance. A staff member who calls out within 4 hours of a shift shall trigger the on-call system; the on-call QP shall arrange a replacement **before the shift begins** and shall cover in-house personally if no replacement is available — the 2:4 minimum shall never be allowed to lapse. **Single-staffing is prohibited at all times.**

Shift	Minimum Staff (1–4 youth)	Minimum Staff (5–8 youth)	Minimum Staff (9 youth)	Status
Day (7a-3p)	2 staff	4 staff	5 staff	Awake / On-site
Evening (3p-11p)	2 staff	4 staff	5 staff	Awake / On-site
Overnight (11p-7a)	2 staff	4 staff	5 staff	Awake (No sleeping)

Note: The state group-home definition limits a facility of this type to no more than nine (9) children. The facility shall not exceed its licensed capacity as stated on the state-issued license, which shall not exceed nine children under any circumstances.

2.2 Staff Qualifications.

General requirements for all staff. All staff must: (a) be at least **18 years of age** at the time of hire; (b) be **literate in English** sufficient to read and understand this Manual, the youth's PCP and BSP, medication administration records, incident-report forms, and emergency procedures; (c) truthfully **disclose any criminal conviction history** on the employment application and consent to the background checks described in §2.3; and (d) provide documentation of education, licensure, and any credentials claimed on the employment application. Misrepresentation on the employment application is grounds for immediate termination.

QPs: Meet one of two pathways per §1.4(b) and our operating standards — **Pathway 1:** master's degree in a human services field plus a recognized NC credential (full license, associate/provisional license, certification, or psychiatric nursing credential) plus at least one year of full-time, post-master's supervised MH/DD/SA experience; **OR Pathway 2:** bachelor's degree in a human services field plus two years of full-time, pre- or post-bachelor's supervised MH/DD/SA experience. Both pathways require NC-DHHS QP training modules prior to independent practice. The designated facility QP shall also meet the two-year direct client care experience requirement per §1.4(b).

APs: Meet one of four pathways per facility policy — **(i)** bachelor's degree in a human services field plus at least one year of relevant MH/DD/SA experience; **(ii)** registered nurse license plus at least one year of relevant MH/DD/SA experience; **(iii)** an equivalent state-recognized certification plus at least one year of relevant MH/DD/SA experience; or **(iv)** a high school diploma or GED plus at least five years of relevant MH/DD/SA experience. APs supervise paraprofessional Direct Care Professionals (DCPs) regarding PCP and BSP implementation per the individualized supervision plan required by §2.2(a).

Direct Care Professionals (DCPs): High school diploma or GED with at least one year of mental health experience. DCPs work under the supervision of an AP or QP per the individualized supervision plan required by §2.2(a).

2.2(a) Individualized Supervision Plans.

Per the Level III Staff-Secure operating standards, the QP shall develop and maintain a **written individualized supervision plan** for every Associate Professional (AP) and Direct Care Professional (DCP) on staff. Each supervision plan shall be: (1) developed within 30 days of hire; (2) reviewed and updated at least annually and upon any change in role, scope of practice, or supervisor assignment; (3) signed by the staff member, the supervising QP (or AP, where the DCP is supervised by an AP), and the Clinical Director; and (4) retained in the personnel file per §2.5. Each supervision plan shall specify: (a) the supervisor's name, credential, and availability; (b) the frequency and format of supervision (minimum: weekly individual supervision for APs and DCPs during the first 90 days of employment, transitioning to at least bi-weekly individual supervision thereafter, plus weekly group clinical supervision for all direct-care staff); (c) the scope of delegated authority (including any limits on independent decision-making, restraint authorization, medication administration, or incident reporting); (d) the method for documenting supervision sessions (signed supervision log, case-discussion notes, and any corrective-action directives); (e) the escalation pathway for clinical concerns; and (f) the criteria and process for modifying the supervision plan based on performance, clinical incident history, or changes in the youth population served. AP supervision plans shall additionally document the AP's authority to supervise paraprofessional DCPs regarding PCP and BSP implementation, including the documentation standard for AP review of DCP shift notes and incident reports.

2.3 Background Checks (Prior to Unsupervised Contact).

NC SBI fingerprint criminal background check — must be completed within **180 days prior to initial licensure review** and re-checked annually thereafter and upon reasonable suspicion.

Health Care Personnel Registry check — must be completed within **90 days prior to licensure review** (initial and renewal) and re-checked annually thereafter.

DSS Child Abuse and Neglect Registry check (every state of residence in prior 5 years) — within 90 days prior to licensure review and annually thereafter.

Motor Vehicle Record (MVR) for staff who transport residents — annually.

Re-checks completed annually and upon reasonable suspicion.

No staff member shall have unsupervised contact with any youth until all four background-screening items above are complete, current, and on file in the personnel record. The QP shall maintain a **Background Check Expiration Tracking Log** showing for each staff member the date of the most recent criminal-background check, HCP-Registry check, DSS-CAN-Registry check, and MVR check, with automatic 30-day advance-notice alerts prior to each annual expiration. Any staff member whose background check has expired shall be removed from the schedule until the re-check is complete and on file.

2.4 Mandatory Training (Prior to Independent Duty; Annual Refreshers).

CPR with Heimlich Maneuver / First Aid (annually; **in-person only** — no online-only certification accepted)

NCI or CPI restraint and de-escalation (annually; **in-person only** for the physical-restraint module)

Medication Administration (RN-delegated, annually; **in-person only**)

Bloodborne Pathogens (annually)

Trauma-Informed Care (annually)

Rule 108 incident reporting (annually)

Population-Specific Training — children/adolescents with serious emotional disturbance (SED), co-occurring disorders, trauma history, and the developmental, cognitive, and clinical characteristics of the population served by this facility (orientation + annual refresher)

Adolescent development, C-SSRS suicide risk assessment, and person-centered planning (orientation + quarterly)

Alternatives to Restrictive Interventions (De-Escalation Training) — verbal de-escalation, environmental modification, sensory regulation, and trauma-informed redirection (orientation + annual refresher)

Seclusion, Physical Restraint & Isolation Time-Out — physical-restraint techniques, release criteria, post-restraint medical/clinical monitoring, and documentation (annually; **in-person only**)

Client Rights & Confidentiality — the Resident Rights framework and HIPAA (orientation + annual refresher)

General Organization Orientation — mission, policies, chain of command, emergency procedures (orientation)

2.5 Personnel Records.

Personnel files shall contain: position descriptions; education verification; licensure and credentials; continuing education and training records; clinical and administrative supervision documentation; supervision plans; sanctions reviews from professional boards; documented review of the NC Health Care Personnel Registry; and criminal background check results. Records shall be retained per applicable DHHS retention schedules. The QP audits personnel files annually to ensure completeness and compliance with RMDM Chapter 1.

2.5(a) Continuing Education Units (CEUs) — Licensed Clinical Staff.

Each licensed clinical staff member (LCSW, LPC, LMFT, LCAS, Licensed Psychologist, psychiatric RN / NP / CNS, etc.) shall complete the continuing-education hours required by their licensing board for each license-renewal cycle, and shall maintain original CEU certificates in the personnel file. Minimum CEU requirements by license type (verify current board rules — these are the typical NC minimums): **LCSW & LCSW-A (NCSWCLB)**: 40 contact hours / 2-year cycle, including 4 hrs ethics; **LPC & LPC-A (NCBLPC)**: 40 contact hours / 2-year cycle, including 4 hrs ethics; **LMFT & LMFT-A (NCMFT Licensure Board)**: 20 contact hours / 1-year cycle, including 3 hrs ethics; **LCAS & LCAS-P (NCSAPPB)**: 40 contact hours / 2-year cycle, including 4 hrs ethics and 3 hrs substance-abuse-specific; **Registered Nurses (NC BON)**: 30 contact hours / 2-year cycle. The QP shall maintain a **CEU Tracking Log** showing for each licensed staff member: license type, license number, expiration date, current-cycle hours completed, hours remaining, ethics hours completed, and the date of the next renewal. The log shall be reviewed monthly by the QP; any staff member within 60 days of license expiry with insufficient CEUs shall be placed on a documented performance-improvement plan and removed from billable-service delivery if the license lapses. Lapsed licenses shall be reported to the Clinical Director within 1 business day and to Alliance Health credentialing within 5 business days.

2.6 Staff Leave & Time-Off Policy.

Pursuant to our operating standards, the facility shall provide each full-time employee with **scheduled time off** as follows: (a) a minimum of two non-consecutive scheduled days off per 14-day pay period for direct-care staff (DCPs, APs) working 12-hour shifts, and a minimum of two scheduled days off per 7-day week for administrative and clinical staff (QP, RN, Billing Coordinator) working 8-hour shifts; (b) paid time off (PTO) accruing at no less than the facility standard published in the Employee Handbook (currently 10 days/year in year 1, 15 days/year in years 2–4, and 20 days/year in year 5+); (c) **protected rest periods** of no fewer than 8 hours between consecutive shifts unless the staff member voluntarily waives the rest period in writing; (d) meal breaks of no fewer than 30 minutes per 8-hour shift and 60 minutes per 12-hour shift, during which the staff member is relieved of all youth-supervision duties (a second staff member shall be on duty to maintain the 2:4 minimum); (e) sick leave consistent with NC statutory requirements; (f) bereavement leave of up to 3 days per qualifying event; (g) family-medical leave consistent with the federal FMLA and NC leave laws; and (h) reasonable accommodation for religious observance. The QP publishes the schedule no fewer than 14 days in advance, posts it in the staff area, and accommodates time-off requests on a first-come-first-served basis subject to the 2:4 staffing minimum. Time-off denials shall be documented in writing with the operational basis. **Staff shall not be required to work more than**

16 consecutive hours except in a declared facility emergency, and any such extended shift shall be followed by no fewer than 10 hours of off-duty rest.

2.7 Instructor Credentials & Trainer Certifications.

For each mandatory training topic listed in §2.4, the facility shall retain documentation of the instructor's qualifications and current instructor-level certification in the facility compliance binder (separate from individual staff personnel files). At a minimum, instructor credentials shall be maintained for: (a) **CPR with Heimlich Maneuver / First Aid** — current American Heart Association (AHA) Basic Life Support (BLS) Instructor certification or American Red Cross First Aid/CPR/AED Instructor certification; (b) **NCI or CPI restraint and de-escalation** — current NCI Instructor certification or CPI Certified Instructor credential; (c) **Medication Administration** — RN with current, unrestricted NC license who has completed the NC-DHHS-approved Medication Administration Trainer course (or equivalent) and is authorized by the facility's Clinical Director to delegate and train; (d) **Bloodborne Pathogens** — RN or qualified designee trained on the OSHA Bloodborne Pathogens Standard (29 CFR 1910.1030); (e) **Seclusion, Physical Restraint & Isolation Time-Out** — same instructor credentials as (b) above. For each instructor, the QP shall maintain: instructor name; certification body; certification number; issue date; expiration date; copy of the instructor certificate; and a list of all training sessions delivered (date, topic, attendee roster). Instructor certifications shall be re-verified annually; expired instructor certifications shall be renewed before the instructor delivers any further training. Training delivered by an uncertified or expired-certified instructor is invalid and shall be re-delivered by a properly-certified instructor.

3. Admissions, Discharges, and Transition Planning

Reference: our staff-secure operating standards & .5604; NC Medicaid CCP 8D-2; RMDM Chapters 2 & 5 · Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.24, Aug 2026 — Legislation-Free Public Edition) · §3

3.1 Admission Criteria. The program serves youth with a primary mental health or behavioral diagnosis requiring supervised living, who are medically stable, and whose clinical needs can be safely met in a Level III Staff-Secure setting. Per §1.2(g) and NC Medicaid Clinical Coverage Policy 8D-2 §1.0(c), the Level III (Residential Treatment High) setting is a "**highly structured and supervised environment in a program setting only, excluding room and board**" — meaning the facility provides treatment in a structured **program setting** (not a family home), with continuous awake supervision per §2.1 and §9.5, and with the clinical/treatment/milieu component reimbursed by the Medicaid RTS per-diem while room and board are funded through a non-Medicaid source. Exclusions include acute psychiatric crisis requiring inpatient hospitalization, medical instability, or fire-setting that cannot be safely managed. The QP reviews each referral packet and documents the admission decision, including verification that an identified non-Medicaid room-and-board funding source is in place for the youth per §1.2(g)(iii) and §10.9.

3.2 Admission Process — Pending vs. Full Records.

Upon initial presentation for screening, a **Pending Record** is created. This contains initial screening information, consultation and administrative coordination, and court-ordered evaluations that do not yet result in active enrollment. If the individual is enrolled in active services, the Pending Record shall be converted into a **Full Clinical Service Record** using the same record number, with all pending documentation merged. On the Day of Admission, the QP obtains signed consents (treatment, medication, transportation, ROI, photographs), the DCP conducts a respectful property search and secures medications, the QP administers the C-SSRS screening and documents immediate clinical status, and an RN visit is scheduled within 72 hours.

3.3 Full Clinical Service Record — Required Elements.

Upon conversion to a Full Record, the chart must contain all applicable elements enumerated in the Comprehensive Clinical Record Content Checklist (Form 8). Categories include: Consents; Demographics and Emergency

Information; Health History (including DSM-5-TR/ICD-10 diagnosis and documentation of medication allergies, adverse reactions, and **absence of known allergies**); Medications and Labs; Rights and Restrictive Interventions; Assessments (CCA by a licensed professional); Planning (PCP with MID, service plan, signed service order); Discharge information; Referral information; Service notes/grids; Incidents (filed separately; occurrence noted in record); Disclosures and Legal documents; 42 CFR 2.22 summary for SUD; Accounting of Disclosures; and incoming/outgoing correspondence.

3.4 Discharge & Transition.

Transition planning is initiated at admission and reviewed every 30 days. Discharge may be planned or unplanned. The Discharge Summary is completed within **7 calendar days** of discharge (exceeds RMDM's 30-day requirement), and includes: reason for admission, course/progress, condition at discharge, recommendations, final diagnoses, and dated signatures. Medications are transferred to the guardian with a signed transfer form, belongings are inventoried, and the discharge summary is provided to the guardian, receiving provider, and LME/MCO.

3.4(a) Advance Written Notification (Non-Emergency Discharge or Transfer).

For any **non-emergency** discharge or transfer, the QP shall provide **written advance notification** to the youth (in an age-appropriate manner), the guardian, the regional managed care organization (LME/MCO) representative, and the receiving provider (if known) at least **7 calendar days** prior to the discharge date. The written notification shall include: the discharge date, the reason for discharge, the receiving provider (if known), a transition-plan summary, the post-discharge appointment schedule (outpatient therapy, medication management, primary care), and the emergency contact number for post-discharge clinical questions. **Emergency discharges** (e.g., acute psychiatric hospitalization, medical hospitalization, safety-motivated transfer) are exempt from the 7-day requirement but shall be documented with the emergency basis and provided with as much advance notice as circumstances reasonably permit.

3.4(b) Pre-Discharge Care-Coordination Team (CFT) Meeting.

The QP shall convene a **pre-discharge care-coordination team (CFT) meeting** prior to any planned discharge. Meeting attendees shall include, at minimum, the youth, the guardian, the regional managed care organization (LME/MCO) representative, and the receiving provider (if applicable). The meeting shall review discharge readiness, finalize the transition plan, confirm community linkages (outpatient therapy, medication management, school enrollment, primary care), review medications and any medication-transfer logistics, and document attendee signatures on the PCP. For youth transitioning to a lower level of care, the CFT shall additionally confirm the step-down provider's admission date and any trial-home-visit schedule.

3.4(c) Post-Emergency Service-Planning Meeting.

Following any **emergency discharge, transfer, or hospitalization** (e.g., acute psychiatric hospitalization, medical hospitalization, safety-motivated transfer, or significant restraint event), the QP shall convene a **service-planning meeting within 5 business days** of the youth's return to the facility (or within 5 business days of the emergency event if the youth does not return). The meeting shall include the youth, the guardian, the regional managed care organization (LME/MCO) representative, the prescribing provider (if a medication change is indicated), and direct-care staff who witnessed the precipitating event. The meeting shall: review the precipitating events and any contributing factors; update the PCP and BSP as needed to prevent recurrence; adjust the supervision level if clinically indicated; identify any staff-training needs; document follow-up actions, responsible parties, and completion deadlines; and record attendee signatures on the updated PCP.

3.5 Administrative Closure.

When a QP or clinician leaves employment without completing required discharge documentation for individuals meeting discharge criteria, the supervisor shall process the discharge. Each administratively closed record shall be

audited per entity policy to ensure all billed services are properly documented. If audit reveals unmet documentation requirements, all services billed without proper documentation shall be adjusted back to the payor per 42 CFR 401.305. Documentation of the administrative closure shall be placed in the service record.

3.6 18th-Birthday Continuation Policy.

Per the Level III Staff-Secure operating standards, a youth who turns 18 years of age while in placement may **continue in the program for up to six (6) months, or until the end of the current school year, whichever is longer**, provided that all of the following conditions are met: (a) the youth **consents to continued placement in writing** as an adult, with informed consent documented on the standard admission-consent forms; (b) the guardian (or, if guardianship has been legally terminated, the youth alone) and the regional managed care organization (LME/MCO) representative **approve the continuation in writing**; (c) the PCP is **updated within 14 days of the 18th birthday** to reflect adult-appropriate goals, rights, discharge planning, and any changes in consent authority; (d) the youth is **informed orally and in writing of their adult rights**, including the right to request discharge at any time, the right to refuse medication (except as otherwise provided by law or court order), the right to uncensored communication, and the right to file a grievance directly with the LME/MCO or the state licensing authority; and (e) the facility **confirms that the youth's Medicaid eligibility and non-Medicaid room-and-board funding source** continue to cover the extended placement per §1.2(g)(iii) and §10.9. If any of these conditions cannot be met, the QP shall initiate transition planning to an appropriate adult placement **no later than 30 days before the youth's 18th birthday**, with the discharge date set on or before the 18th birthday. The QP shall document satisfaction of each condition (a)–(e) in the clinical record and shall retain the youth's written consent and the LME/MCO approval letter in the facility compliance binder.

4. Clinical Services, Assessments & Person-Centered Planning

Reference: our operating standards; NC Medicaid CCP 8D-2; RMDM Chapter 4 · Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.24, Aug 2026 — Legislation-Free Public Edition) · §4

4.1 Comprehensive Clinical Assessment (CCA).

A CCA performed by a licensed professional (QP) is required **prior to service delivery**, except in crisis/emergency situations or when a current CCA (with no substantive change) is on file. The CCA must contain: (1) presenting problems, source of distress, precipitating events, associated symptoms; (2) chronological general health, past trauma, behavioral health history (MH/SU including tobacco), treatment history and response; (3) current medications for medical, psychiatric, and SUD treatment, including past ineffective medications or significant side effects; (4) review of biological, psychological, familial, social, developmental, and environmental dimensions identifying strengths, needs, and risks; (5) evidence of beneficiary and LRP participation; (6) analysis and case formulation; (7) **ASAM level of care determination** when substance use disorder is present; (8) DSM-5-TR diagnoses (MH, SUD, IDD, physical health, functional impairment); (9) recommendations for additional assessments, services, supports, or treatment; and (10) dated signature of the licensed professional completing the assessment.

For children and youth, the CCA must address Child and Family Team involvement; if new to services, recommend a Child and Family Team meeting; assess strengths of the child/youth and family (preferably with a strengths-based tool); and utilize IEPs and psychological testing reports if available. **Reassessment** is required when clinically indicated (new behaviors, unmet needs, annual PCP review, or when the service definition requires). If reassessment results in a diagnosis change, a written report is required; if refinement only, a clinical note is sufficient.

4.2 Medical Necessity Determination.

Medical necessity is established by the QP (or licensed/certified professional) based on diagnostic criteria and established best practice guidelines. The CCA and subsequent assessments shall support medical necessity for each service delivered. Medical necessity must meet applicable Medicaid and State-funded policy requirements (NC Medicaid Behavioral Health CCPs; DMH/DD/SUS State-Funded Service Definitions). The QP documents medical necessity in the PCP.

4.3 Person-Centered Plan (PCP) Development.

Within **30 calendar days** of admission, the QP facilitates a PCP meeting including the youth, guardian, LME/MCO representative, and other supports identified by the family. The PCP documents strengths, needs, goals, specific interventions, and a crisis plan. The PCP is signed by all participants, reviewed every 90 days thereafter, and updated via addendum for significant clinical changes (new diagnosis, medication change, major incident, or living arrangement change). Staff implement PCP goals daily and document progress in shift notes. The QP creates a "Staff Cheat Sheet" summarizing goals, triggers, and interventions for each youth, and staff sign the PCP Acknowledgment Form.

4.4 Service Orders.

Service orders are required for all Medicaid and State-funded services that require them. The service order is indicated by the appropriate professional's signature on the PCP. If a separate format is used, a separate service order is required. **Verbal orders:** treatment may proceed based on a verbal order; the verbal order must be documented in the service record on the date given, specifying the date, who gave the order, who received the order, and each distinct service ordered. Documentation must reflect why a verbal order was obtained in lieu of a written order. The appropriate professional must countersign the order with a dated signature **within 72 hours** of the verbal order.

4.5 Service Authorization.

Requests for authorization are required prior to initiation or continuation of services as per State-funded service definitions, Medicaid CCPs, or the authorizing entity's UM policy. The facility shall notify the authorizing entity when an individual changes providers or ends a service, and end-date reporting requirements must be followed. Service authorizations and reauthorizations are not required to be maintained in the clinical record but shall be available for audit purposes if requested.

4.6 Licensed Professional Face-to-Face Clinical Consultation.

Per the Level III Staff-Secure operating standards, the facility shall arrange for a **Licensed Professional to provide at least four (4) hours per week of face-to-face clinical consultation** with the clinical team. The Licensed Professional shall be a **clinician licensed by the applicable state licensing board to independently provide mental health services** (e.g., a licensed psychiatrist, psychologist, LCSW, LPC, or LMFT), and may be the facility's Clinical Director, the QP if the QP holds a full clinical license, or an external clinical consultant under contract. The four (4) hours of weekly consultation shall be provided in **face-to-face** format (in-person at the facility; telehealth video is acceptable only when in-person consultation is not feasible and is documented on the consultation log). The clinical consultation shall include, at minimum: (a) **review of clinical cases**, including any youth on psychotropic medication, any youth who has experienced a restraint event in the past 14 days, and any youth with a significant clinical change since the prior consultation; (b) **consultation on diagnostic and treatment-planning questions**, including differential diagnosis, co-occurring disorders, and treatment-resistance; (c) **guidance on behavioral-support-plan adjustments**, including functional-analysis findings and replacement-skill targets; (d) **review of restrictive-intervention use** and trauma-informed restraint-reduction strategies; and (e) **clinical supervision of APs and DCPs** as delegated by the QP, including case discussion, skill-building, and reflective-practice support. Each consultation session shall be documented on the **Licensed**

Professional Consultation Log (Form 10), including: date, start/end time, total duration, format (in-person or telehealth), attendees, cases reviewed, recommendations made, follow-up actions assigned, and the Licensed Professional's signature. The QP shall retain all consultation logs in the facility compliance binder and shall report weekly consultation-hour totals to the Clinical Director at each quarterly compliance report per §1.4(a).

5. Behavioral Management & Restraint

Reference: our operating standards; CMS Mental Health Parity Rules; RMDM Chapters 2 & 6 · Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.24, Aug 2026 — Legislation-Free Public Edition) · §5

5.1 Philosophy. Well Spring Intervention LLC employs a trauma-informed, positive behavioral interventions model. We recognize that behavior is communication, and our role is to teach and reinforce safer, more adaptive skills rather than punish behavior. Corporal punishment, withholding meals or sleep, social isolation as punishment, humiliation, and degradation are strictly prohibited and constitute reportable abuse under Rule 108.

5.2 Behavioral Support Plan (BSP).

Developed by the QP within 14 calendar days of admission, based on a Functional Behavioral Assessment (FBA). The BSP includes proactive strategies, replacement skills, de-escalation steps, and crisis responses. All staff review and sign the BSP Acknowledgment Form before working independently with the youth.

5.3 Restraint.

Restraint is an absolute last resort, used only when a youth is in imminent danger of serious harm to self or others and after less restrictive de-escalation has failed. Mechanical, chemical, and seclusion restraints are prohibited. Only NCI/CPI-trained staff may apply restraint, using the least restrictive hold. **Prone (face-down) positions are prohibited.** Post-restraint medical check (within 1 hour), debriefing (within 24 hours), and guardian notification (within 1 hour) are required. All restraints are reported in IRIS.

5.4 Restrictive Intervention Documentation.

Written notifications, consents, approvals, and other documentation per our service planning standards are required whenever a restrictive intervention is used as a planned intervention. Any planned restrictive interventions must be included in the youth's service plan per our operating standards. Documentation in the service record must meet requirements of our operating standards, including rights restrictions and use of protective devices.

5.5 Activities Program — Minimum 14 Hours/Week Planned Group Activities.

Pursuant to **our staff-secure operating standards** (and the service-design requirements of .1701(e), including "individualized supervision and structure of daily living" and acquisition of "social and recreational skills"), the facility shall provide each youth with a documented **activities program of no fewer than 14 hours per week of planned group activities** that promote socialization, physical activity, and creative expression. Activities shall be: (a) age- and developmental-stage appropriate; (b) culturally responsive; (c) consistent with each youth's PCP goals (per §4) and ISP objectives; (d) trauma-informed, with voluntary participation and the right to decline without consequence (except where a specific activity is ordered by a licensed clinician as part of the treatment plan); and (e) inclusive of all youth regardless of mobility, sensory, or cognitive accommodation needs, with reasonable modifications provided per the ADA and Section 504 of the Rehabilitation Act.

(a) Activity Categories. The 14-hour weekly minimum shall be distributed across the following categories: **(i) physical activity** — structured sports, yoga, walking, swimming, gym visits, outdoor games (minimum 4 hrs/week); **(ii) creative expression** — art, music, journaling, theater, dance, photography, creative writing (minimum 3 hrs/week); **(iii) socialization & life-skills** — cooperative games, group problem-solving, cooking, budgeting, self-care skills, communication skills (minimum 4 hrs/week); **(iv) community integration** — supervised outings to

parks, libraries, cultural events, volunteer activities, faith-based services (where the youth elects) (minimum 3 hrs/week). Activities may overlap categories (e.g., a community outing that involves physical activity may count toward both categories' minimums, but the total weekly hours still must sum to no fewer than 14 unique contact hours).

(b) Documentation. The QP shall publish a written **Weekly Activities Calendar** no fewer than 7 days in advance, post it in the facility common area, and provide a copy to each youth at the weekly community meeting. Each scheduled activity shall be documented on the Activities Log (a sub-section of the daily shift note per §10.3) showing: date, activity name, category, duration (start/end time), staff facilitating, youth participating, youth declining (with brief reason), and any incidents or notable interactions. The QP shall total the weekly activity hours at each Monday shift-change huddle and shall report any week in which the 14-hour minimum was not met — with the reason and corrective-action plan — to the Clinical Director at the next monthly compliance report.

(c) Coordination with Protocol 22 Daily Workflow. The Daily Workflow Schedules in Protocol 22 (DCP Day Shift, DCP Evening Shift, and weekend schedules) shall incorporate specific activity time-blocks sufficient to meet the 14-hour weekly minimum. The QP may adjust the daily schedule to accommodate school attendance, therapy appointments, weather, and behavioral acuity, provided the weekly 14-hour minimum is met. Activities shall be suspended only for documented safety reasons (e.g., a youth in crisis, an active IRIS investigation, a facility lockdown) and shall resume as soon as safety allows.

6. Health, Medication, & Nutrition Management

Reference: our operating standards; NC Nursing Practice Act; RMDM Chapters 5 · Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.24, Aug 2026 — Legislation-Free Public Edition) · §6

6.1 Medical Care.

Each resident has an identified Primary Care Physician (PCP) and psychiatrist upon admission. **Per our staff-secure operating standards, a complete medical (physical) examination must be conducted within 90 days PRIOR to admission**, and the exam report must be reviewed by the QP and the RN prior to the youth's move-in date. The pre-admission exam shall include a comprehensive physical assessment, vision and hearing screening, immunization review (with documentation per state law §130A-152), TB screening per §6.2, and any indicated laboratory studies. If the physical exam is older than 90 days at the time of admission, a new exam shall be scheduled and completed within 7 days post-admission (with documentation of the scheduling in the clinical record) — this 7-day post-admission exam is a contingency only; it does NOT replace the 90-day pre-admission requirement. **Annual** physical examinations shall be conducted thereafter for the duration of the youth's stay. Medical history (including immunizations, allergies, and current prescriptions) is maintained and updated at every visit. Physician's directions for management of any identified medical conditions shall be documented in the PCP and communicated to all shift staff. The original physical exam report shall be retained in the clinical record per §1.6.

6.2 Tuberculosis (TB) Screening.

Applicable if the facility receives SAPTBG funds for treatment services. Screenings must query: medical treatment in the past 3 months; current residence (jail, streets, shelter, etc.); history of TB tests; and physical symptoms (night sweats, prolonged cough, shortness of breath, unexplained weight loss). Positive responses require referral to the local county health department or medical practitioner for follow-up testing and care. Completed screening and follow-up are documented in the service record.

6.3 Medication Management.

All medications are stored in a double-locked cabinet/cart per §6.3(c). Controlled substances are counted and documented at every shift change. Medications are administered only by RN-delegated staff who have completed the state-approved medication administration training. Staff follow the "5 Rights" (right youth, right med, right dose, right route, right time) and document administration on the MAR **immediately** (never retrospectively). Medication errors, refusals, and adverse reactions are reported to the RN and QP immediately, documented on the MAR, and entered into the state incident-reporting system (IRIS) as required.

6.3(a) Psychotropic Medication Drug Regimen Review.

Per the Level III Staff-Secure operating standards, the facility shall ensure that **every youth on a psychotropic medication has a drug regimen review performed by a pharmacist or physician at least every six (6) months**. The review shall evaluate: (1) the appropriateness of each medication and dose given the youth's diagnoses, age, weight, and clinical response; (2) any drug-drug interactions, drug-disease contraindications, or duplicate therapies; (3) any laboratory monitoring required (e.g., metabolic panel for atypical antipsychotics, lithium levels, carbamazepine levels); (4) any adverse drug reactions or side effects reported since the prior review; and (5) any recommended changes to the medication regimen. The reviewing pharmacist or physician shall document the review on the **Psychotropic Medication Drug Regimen Review Log (Form 11)**, including: youth name, date of review, medications reviewed (name, dose, frequency, indication), reviewer findings, recommendations, and reviewer signature. The QP shall forward the review findings to the prescribing psychiatrist within 5 business days, shall document the review in the clinical record, and shall track the next-6-month review due date on the Form 11 tracking log. A youth admitted on a psychotropic medication shall have an initial drug regimen review within 30 days of admission (the youth's most recent prior review may satisfy this requirement if completed within the prior 6 months and a copy is obtained).

6.3(b) Medication Receipt & Verification.

All medications received at the facility — whether from a pharmacy, guardian, hospital, or other source — shall be **verified by the RN** (or designated RN-delegated staff) **at the time of receipt**. Verification shall confirm: (a) the medication is in **tamper-resistant packaging** per the state controlled-substances and pharmacy rules; (b) the **label contains** the youth's full name, medication name, strength, dose, route, frequency, prescribing provider, pharmacy name and phone number, prescription number, fill date, and expiration date; (c) the medication **matches the prescriber's order** in the youth's clinical record; and (d) the medication is **not expired**. Discrepancies shall be reported to the prescribing pharmacy and the QP immediately, and the medication shall **not** be administered until the discrepancy is resolved. Receipt of each medication shall be documented on the Medication Receipt Log (maintained in the med room), including: date received, youth name, medication name, quantity received, lot number, expiration date, verifying RN signature, and any discrepancy notes.

6.3(c) Medication Storage.

All medications shall be stored in a **double-locked cabinet or cart** in a secure, climate-controlled area maintained between **59°F and 86°F**. Medications requiring refrigeration shall be stored in a **dedicated medication refrigerator** (not used for food) maintained between **36°F and 46°F**, with the temperature documented on Form 5 (Environmental Safety Log) daily. Controlled substances shall be stored in the **inner locked compartment** of the double-locked cabinet, with a count documented at every shift change. Each youth's medications shall be stored in individually labeled containers. Medications shall **not** be stored in bathrooms, kitchens (except the medication refrigerator), or other areas exposed to moisture, heat, or direct sunlight. Expired or discontinued medications shall be segregated in a clearly labeled "To Be Disposed" container pending disposal per §6.3(d). The RN shall audit the medication storage area weekly and document the audit on Form 5.

6.3(d) Medication Disposal Documentation.

Discontinued, expired, or refused medications shall be disposed of in accordance with the state controlled-substances act and applicable federal medication-disposal rules. For each medication disposal event, the RN (or designated RN-delegated staff) shall document on the **Medication Disposal Log** (maintained in the med room): the youth's name, medication name, strength, quantity disposed, disposal method (e.g., pharmaceutical take-back, DEA-authorized collection receptacle, mail-back package), disposal date, witness signature, and RN signature. **Controlled-substance disposals shall be witnessed by a second staff member** and the witness signature shall be obtained prior to disposal. Medications transferred to the guardian at discharge shall be documented on the Medication Transfer Form (with the guardian's signature) and are **not** recorded on the Disposal Log. The QP shall review the Medication Disposal Log monthly as part of the controlled-substance reconciliation audit and shall report any discrepancies to the Clinical Director per §1.4(a).

6.3(e) Medication Education.

The QP, RN, and prescribing psychiatrist shall provide **ongoing medication education** to each youth regarding any psychotropic or other medication prescribed. Education shall be documented **at admission, at each medication change, and at least quarterly thereafter**. Education shall be developmentally appropriate and include: (1) the medication name and purpose; (2) the prescribed dose, route, and schedule; (3) expected benefits; (4) common and serious side effects, including signs of allergic reaction; (5) interactions with food, other medications, and substances; (6) the importance of adherence and the risks of abrupt discontinuation; (7) the youth's right to ask questions and to refuse medication (except as otherwise provided by law or court order per §1.7); and (8) what to do if a dose is missed. The youth's understanding shall be assessed (e.g., teach-back method) and documented after each education session. **Family/guardian medication education** shall also be offered at admission, at each medication change, and at least quarterly, with the offer and any education provided documented in the clinical record. The QP shall track medication-education due dates on the youth's PCP review calendar and shall report any youth with overdue medication education to the Clinical Director at each quarterly compliance report per §1.4(a).

6.4 Nutrition.

Meals follow USDA guidelines. Special diets are accommodated with provider documentation. Menus are posted and retained 30 days. Food is never withheld as a consequence. Staff preparing food maintain current food handler certifications.

6.5 Infection Control Program.

The facility shall maintain a written **Infection Control Program** consistent with **CDC guidelines** for residential congregate-care settings and OSHA Bloodborne Pathogens Standard (29 CFR 1910.1030). The program is overseen by the RN (with monthly QP review) and includes the following components:

(a) **Written Infection-Control Plan.** A facility-specific plan covering hand hygiene, standard precautions, transmission-based precautions, cleaning and disinfection, laundry handling, waste disposal, exposure response, outbreak response, and respiratory etiquette. Reviewed and updated annually and after any infection-control incident.

(b) **Cleaning & Disinfection Schedules.** Written daily, weekly, and monthly cleaning schedules specifying each area of the facility (kitchen, bathrooms, bedrooms, common areas, laundry, vehicles), the cleaning agent and concentration, the surface-contact time, and the staff member responsible. Schedules are posted in each area and the completed checklists are filed monthly in the facility compliance binder.

(c) **Blood / Body Fluid Precautions.** Standard Precautions are observed with all youth at all times. **OSHA Bloodborne Pathogens exposure-control plan** on file: engineering controls (sharps containers), work-practice controls (no recapping needles), PPE (gloves, gowns, face protection), hepatitis-B vaccination offered to staff at no cost, post-exposure prophylaxis protocol available 24/7 via the RN and the local emergency department. All blood and body-fluid exposures (staff or youth) are documented on an Incident Report (§8) and IRIS-filed

within 24 hours.

(d) Hand Hygiene. Staff and youth perform hand hygiene: before meals and food preparation; after toileting or assisting a youth with toileting; after coughing, sneezing, or blowing the nose; before and after glove use; before and after medication administration; and after handling soiled laundry or trash. Alcohol-based hand sanitizer (60–95% alcohol) is available in every room except where flammable (e.g., kitchens with open flame). Soap and running water are used when hands are visibly soiled.

(e) Outbreak Response. Two or more youth with identical symptoms (gastrointestinal, respiratory, or febrile) within 72 hours shall trigger an outbreak response: cohort-affected youth, increase sanitation, notify the County Health Department within 24 hours for any reportable communicable disease, and document the outbreak response in the facility compliance binder. The QP shall file an IRIS report for any communicable-disease outbreak affecting two or more youth.

(f) Immunization Compliance. Each youth's immunization status shall be verified at admission per state law §130A-152 and updated per the CDC immunization schedule. Staff annual influenza vaccination is strongly recommended and documented in the personnel file. COVID-19 vaccination status of staff and youth is documented per current CDC and DHHS guidance.

(g) PPE Inventory. The facility maintains a current inventory of exam gloves (multiple sizes), surgical masks, N95 respirators (for staff caring for youth with airborne precautions), gowns, eye protection, face shields, sharps containers, biohazard bags, and EPA-registered hospital disinfectants. The RN shall audit the inventory monthly and replenish before stock falls below a 30-day supply.

(h) Staff Training. All staff complete **Bloodborne Pathogens** training annually and Infection Control training at hire and annually thereafter. The RN conducts the training and documents completion in the personnel file.

7. Education & Vocational Support

Reference: NC General Statutes (Education of Homeless Children); IDEA; RMDM Chapter 4 · Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.24, Aug 2026 — Legislation-Free Public Edition) · §7

7.1 Education Coordination.

The QP serves as Education Liaison for every youth in the program. Within 10 calendar days of admission, the QP contacts the Local Education Agency (LEA) — specifically the McKinney-Vento liaison and the EC Director — for enrollment, transportation, and service continuity. The QP signs an ROI for school communication, attends all IEP meetings, advocates for an aligned Behavioral Intervention Plan (BIP), and tracks daily school attendance. School refusal is treated as a clinical issue addressed through the PCP, not as a behavioral violation.

7.2 Vocational Support.

For youth age 14 and older, the PCP includes vocational and independent-living goals aligned with the transition plan. The QP coordinates with Vocational Rehabilitation and community-based employment programs to support the youth's transition to adulthood and post-discharge stability.

7.3 Facility-Based School Determination (Level III RTF vs. PRTF).

A facility-based school is a condition of licensure for Psychiatric Residential Treatment Facilities (PRTFs) under 42 CFR 483.350–483.376 and the federal Individuals with Disabilities Education Act (IDEA), but it is **not** required for a **Level III Residential Treatment Facility (Staff-Secure)** licensed under our staff-secure operating standards. **This facility is licensed as a Level III RTF (Staff-Secure), not as a PRTF.** Accordingly, this facility does not operate a facility-based school and is not required to do so under its current license; youth attend public school in the community per §7.1, with the facility providing transportation, after-school homework support, and IEP coordination.

Medicaid benefit distinction — CCP 8D-2 (RTS) vs CCP 8D-1 (PRTF). The facility-based-school distinction parallels a Medicaid benefit-category distinction that is critical for billing compliance (see §1.2(g) and §10.9). This facility enrolls under **NC Medicaid Clinical Coverage Policy 8D-2 ("Residential Treatment Services")** as a Level III (Residential Treatment High) provider; the Medicaid RTS per-diem reimburses only the clinical/treatment/milieu component and **excludes room and board**. A PRTF, by contrast, enrolls under **NC Medicaid Clinical Coverage Policy 8D-1 ("Psychiatric Residential Treatment Facilities for Children under the Age of 21")** as an **inpatient** facility benefit. Under our operating standards, Subpart G; the PRTF per-diem **includes** the equivalent of room and board. The two benefit categories are mutually exclusive — a single facility cannot bill both 8D-1 and 8D-2 for the same youth on the same day. The QP and Billing Coordinator shall verify at admission that the youth's authorization is for CCP 8D-2 Level III RTS (taxonomy 320800000X), not CCP 8D-1 PRTF.

If the facility elects to pursue PRTF designation in the future, the facility shall, prior to that designation: (a) notify DHSR MHLC and apply for PRTF licensure under 42 CFR 483.350 et seq.; (b) establish a facility-based school meeting IDEA, NC State Board of Education, and accreditation standards; (c) employ or contract with licensed special-education teachers and a school administrator; (d) enter into a written agreement with the LEA for IEP implementation and educational records transfer; (e) document that the facility-based school is necessary because the youth's clinical acuity prevents safe participation in community school; (f) update this SOP and the Alliance Health provider agreement to reflect the PRTF designation; and (g) update NCTracks enrollment from CCP 8D-2 (RTS) to CCP 8D-1 (PRTF) and verify that the PRTF per-diem (including room-and-board-equivalent) is properly authorized. No youth shall be placed at this facility under a PRTF level of care until DHSR MHLC has issued the PRTF license and Alliance Health has authorized PRTF-level billing.

8. Incident Reporting & Response (Rule 108 / IRIS)

Reference: Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.24, Aug 2026 — Legislation-Free Public Edition) · §8

8.1 Reportable Incidents.

The following events are reportable under Rule 108 and must be entered into the Incident Reporting and Investigative System (IRIS): death (including suicide attempts resulting in death), serious injury, alleged abuse or neglect, sexual abuse allegations, use of physical restraint, AWOL/missing child, medication errors, suicidal ideation or self-harm, and any other event meeting the regulatory definition of a reportable incident. When in doubt, staff report.

8.2 Reporting Timeframes.

Staff notify the On-Call QP **immediately** upon discovery of a reportable incident. The QP enters the IRIS report **within 24 hours** of discovery. Allegations of abuse/neglect and missing child trigger **immediate (within 15 minutes)** notification to DSS and law enforcement as appropriate. The guardian is notified **within 1 hour** of incident discovery (unless doing so would compromise an investigation). Verbal notifications are followed by written documentation.

8.3 Investigations.

Internal investigations are initiated within 24 hours and completed within 14 calendar days. Investigations include interviews, video review, documentation review, and a written narrative. A Corrective Action Plan (CAP) is developed for systemic or individual performance issues and uploaded to IRIS.

8.4 Incident Documentation — Separate Filing.

The occurrence of an incident shall be recorded in the service notes. **The completed incident report shall NOT be referenced or filed in the clinical service record.** It shall be filed separately in administrative files. All incident reports are maintained per the Records Retention and Disposition Schedule.

8.5 Abuse/Neglect/Exploitation Reporting.

Relevant facts must be documented in the service record, including reports made by the individual and actions taken. Reporting duties follow North Carolina mandatory reporting laws for child abuse and disabled adult abuse, and our operating standards. The completed incident report is filed separately from the clinical record per our records management policy.

9. Facility, Safety, & Environmental Management

Reference: Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.24, Aug 2026 — Legislation-Free Public Edition) · §9

9.1 Environment.

The facility maintains a safe, clean, home-like environment. Maximum occupancy is two youth per bedroom, with each youth having individual storage space for personal belongings. Indoor temperatures are maintained between 68°F and 80°F. Hot water is regulated to ≤120°F to prevent scalding. Daily safety inspections are conducted by the DCP; weekly inspections by the QP. Hazards are corrected immediately or escalated to the Executive Director.

9.2 Drills & Inspections.

Per the Level III Staff-Secure operating standards, **fire drills shall be conducted monthly AND tornado drills shall be conducted quarterly, with each drill conducted separately for EACH shift (day, evening, and overnight)** — i.e., 3 fire drills per month (one per shift) and 3 tornado drills per quarter (one per shift). Drills shall target evacuation under 3 minutes for fire drills and shall use the lowest interior safe room for tornado drills. Drill documentation on Form 5 (Environmental Safety Log) shall include: drill type (fire / tornado), shift (day / evening / overnight), date, start time, evacuation time (fire drills only), number of youth and staff participating, any deficiencies identified, corrective actions taken, and the shift supervisor's signature. Smoke detectors, fire extinguishers, sprinklers, and CO detectors are inspected monthly (staff) and annually (licensed contractors). Inspection records are maintained on Form 5 for a minimum of 3 years.

9.3 Hazardous Materials.

Cleaning chemicals, sharps, tools, and medications are stored in locked areas. Safety Data Sheets (SDS) are maintained and accessible. No weapons are permitted on facility grounds at any time.

9.4 Two-Story & Multi-Level Facility Requirements.

9.4(a) Applicability & Licensing Notification. This subsection applies to any facility operated by Well Spring Intervention LLC with two or more stories above grade, including any basement used for programmatic activities and any upper-floor youth bedroom. North Carolina family care / group home licensing rules generally restrict licensed structures to no more than two stories above grade when residents are housed above the first floor, and require that any resident housed on the second floor have two direct exterior means of egress. The Executive Director shall obtain **prior written approval** from NC DHSR before: (i) relocating youth bedrooms to a previously unused upper floor, (ii) increasing licensed capacity on an upper floor, (iii) converting a single-story operation to a two-story operation, or (iv) housing non-ambulatory youth on a second floor. Any such change also requires notification to the LME/MCO and may trigger a new construction inspection by the DHSR Construction Section. The QP shall retain the DHSR approval letter in the facility compliance binder for the duration of the license cycle and shall make it available to surveyors on request.

9.4(b) Means of Egress. Each normally occupied story of the facility shall have at least two remotely located means of egress. Every second-floor sleeping room shall be provided with an approved emergency escape and rescue opening meeting the dimensions below, operable from the inside without keys, tools, or special knowledge: minimum net clear opening of **5.7 sq ft** (4.0 sq ft for grade-floor openings); minimum net clear opening height of **24 inches**; minimum net clear opening width of **20 inches**; and maximum sill height of **44 inches** above the floor. The secondary means of egress from a second-floor sleeping room may be satisfied by either (i) a second interior enclosed stairway discharging directly to grade, or (ii) an approved chain-style fire-escape ladder stored in each second-floor youth bedroom, deployable by staff only (never by youth). Stairways shall have handrails on both sides, a minimum 36-inch clear width, maximum 7¾-inch riser height, and minimum 10-inch tread depth for new construction. Stairway illumination with battery / emergency-backup lighting shall be installed at both the top and the bottom of every interior stair.

9.4(c) Fire Detection, Suppression & Alarm — Per-Floor Requirements. Smoke detectors shall be installed on **all levels** of the facility, including every sleeping room, outside each separate sleeping area within 21 ft of bedroom doors, and at least one detector on every story (basement, first floor, and every upper floor). All smoke detectors shall be interconnected so that actuation of one alarms all. Carbon monoxide (CO) detectors shall be installed on every floor containing a fuel-burning appliance or attached garage, and within 10 ft of each sleeping room; combination smoke/CO detectors may be used to satisfy both requirements in a single device. Automatic fire sprinklers are required for all Level III residential treatment facilities under the NC DHSR license condition and NFPA 13D/13R. Fire extinguishers (minimum 2A-10BC rating) shall be mounted on each level with a maximum travel distance of 75 ft from any point in the facility. All detectors, sprinklers, and extinguishers shall be inspected monthly by staff and annually by licensed contractors, with the per-floor results documented on the Environmental Safety Log (Form 5).

9.4(d) Per-Floor Staff Supervision. When youth bedrooms are located on an upper floor, the awake overnight DCP shall be positioned to provide line-of-sight OR audible-monitoring coverage of the upper-floor hallway. The 15-minute room-check requirement (§7) must be physically performed on **every floor where youth are sleeping** — a stair-climbed walk-through, not a call-up. Each per-floor walk-through shall be documented on the Night Watch Log (Form 1) Per-Floor Walk-Through Certification block. When only one awake overnight staff is on duty and youth sleep on multiple floors, the QP shall authorize one of the following: (i) addition of a second awake overnight staff member; (ii) relocation of all sleeping youth to the floor staffed by the awake overnight DCP; or (iii) approved baby-monitor / intercom coverage of the unstaffed floor with a documented response-time plan not to exceed 60 seconds. Stair safety gates shall be installed at both the top and the bottom of any staircase accessible to youth under 12 or to youth with documented elopement or suicidality risk; gates shall be locked at night with the key held by the awake overnight staff.

9.4(e) Window Fall Protection. Where any second-floor youth-bedroom window sill is less than 24 inches above the finished floor, fall-protection devices (window guards or restrictors limiting the openable width to no more than 4 inches) shall be installed. Restrictors shall be releasable only by staff key and shall not defeat the emergency escape function of the window (i.e., the restrictor must be disengageable from the inside without tools in an emergency). Window restrictor function shall be checked monthly and documented on the Environmental Safety Log (Form 5) per-floor sub-table.

9.4(f) Drills — Vertical Evacuation. Monthly fire drills shall include **full vertical evacuation** from all second-floor sleeping rooms to grade — drills that evacuate only the ground floor DO NOT satisfy this requirement. Total building evacuation time shall target under 3 minutes (per §9.2), with evacuation time recorded per floor on the Form 5 Monthly Fire Drills sub-table. Quarterly tornado drills shall use a shelter on the **lowest interior level** (basement preferred; if no basement, an interior ground-floor room with no exterior walls). A second-floor interior bathroom is **not** an approved tornado shelter. Annual lockdown drills shall secure all exterior doors and windows on

every floor, including upper-floor windows visible from outside.

9.4(g) Bedroom Placement Policy. When a two-story home has both ground-floor and upper-floor bedrooms, the QP shall assign bedrooms based on acuity, mobility, and elopement risk. Lower-acuity, lower-mobility, or younger youth shall be placed on the ground floor. Higher-acuity youth requiring closer staff proximity may be placed on the floor staffed by the awake overnight DCP. No youth bedroom shall be located in an attic or in a basement used for sleeping purposes. Bedroom placement decisions shall be documented in the youth's PCP and reviewed at each PCP revision.

9.4(h) Posted Evacuation Maps & Signage. Posted floor-plan evacuation maps shall be displayed on each floor showing two routes to grade, the designated outdoor meeting point, and the location of fire extinguishers, smoke/CO detectors, and fire-escape ladders. Stair hazard signage ("No running on stairs"; "Handrail required") shall be posted at the top and bottom of every interior stair. Maps and signage shall be reviewed at each monthly fire drill and updated whenever bedroom assignments change.

9.5 Staff-Secure Physical-Plant Measures.

As a **Level III Residential Treatment Facility — Staff Secure for Children and Adolescents** under our staff-secure operating standards, this facility shall maintain physical-plant and operational measures that provide line-of-sight or continuous-auditory supervision of all youth at all times, and that prevent elopement while preserving the youth's dignity and rights (per §1.7). Staff-secure measures do NOT include locked seclusion, hardware-restricted youth egress, or any building-code "detention and correction" occupancy classification. The following staff-secure measures are in place:

(a) Controlled-Access Entry. All exterior doors are kept locked from the outside. The main entry is equipped with a doorbell, video-intercom, and remote-buzz entry; staff visually verify identity before admitting any visitor. All other exterior doors are locked from the outside and equipped with exit-only panic hardware on the inside that sounds a localized chime when actuated (so staff are immediately alerted to any door opening).

(b) Delayed-Egress Hardware (where permitted). Where local building and fire code permit, exterior exit doors serving youth-occupied areas may be equipped with delayed-egress hardware actuating 15-second delayed exit with a continuous audible alarm, allowing staff response time to redirect a youth in elopement attempt without impeding emergency egress (NFPA 101 §7.2.1.6.1). Delayed-egress hardware is inspected and tested monthly by staff and annually by a licensed contractor, with results documented on Form 5.

(c) Line-of-Sight Supervision. Floor-plan configuration allows line-of-sight from the staff station to all common-area corridors and youth hallways; blind spots are covered by convex mirrors or CCTV cameras (CCTV covers common areas only — never bedrooms, bathrooms, or toileting areas). CCTV footage is retained 30 days and accessed only by the QP, Clinical Director, and Executive Director.

(d) Window Security. All youth-accessible ground-floor windows are equipped with staff-key-releasable restrictors limiting the openable width to 4 inches (per §9.4(e)). Second-floor youth-bedroom windows meet the same standard and additionally serve as emergency-escape openings per §9.4(b).

(e) Perimeter & Outdoor Supervision. The outdoor area is fenced or naturally bounded to a defined perimeter, with a single controlled-access point. Outdoor time is supervised at the 2:4 staff ratio at all times; one staff member remains within line-of-sight of the access point.

(f) Visitor Management. All visitors sign a Visitor Log at entry, present photo ID, are screened against the youth's authorized-visitor list (maintained in the youth's clinical record and updated by the QP / guardian), and are escorted in common areas at all times. Visitors are never permitted in youth bedrooms. Visitor interactions are documented on the daily shift note.

(g) Contraband Search. Youth belongings are searched at admission, upon return from any pass, and upon reasonable suspicion per §5.2 BSP and Form 2. Searches are conducted by staff of the same gender as the youth, with a second staff witness, in a private area, using the least intrusive method consistent with safety. Personal clothing is not removed. Body-cavity searches are prohibited and, if indicated, shall be performed by

medical personnel at the emergency department per physician order.

9.6 Zoning Compliance.

Prior to initial operation and at any change of physical location, the Executive Director shall obtain written confirmation from the local zoning authority that the proposed use (a Level III RTF serving up to nine youth) is a permitted use at the proposed address under the local zoning ordinance. Many NC municipalities classify group homes as a permitted residential use in residential districts under the federal Fair Housing Act Amendments (42 U.S.C. §3604(f)) and the NC Group Homes Act, but local ordinances may impose: (a) a minimum separation distance between group homes (commonly 1,000 feet, measured property-line to property-line); (b) a cap on the number of unrelated individuals sharing a dwelling; (c) off-street parking minimums (typically one space per staff member on the largest shift, plus one space per two youth); and (d) conditional-use permit requirements. The QP shall maintain the zoning approval letter and any conditional-use permit in the facility compliance binder, and shall report any zoning-related complaint or inquiry from neighbors to the Executive Director within 1 business day. Any material change to the facility (e.g., capacity increase, physical expansion, change in license category) requires fresh zoning verification before DHSR MHLC will process the license amendment.

9.7 Disaster & Emergency Plan (Fire, Tornado, Hurricane, Power Outage).

The facility shall maintain a written **Disaster & Emergency Plan** covering fire, tornado, hurricane, power outage, winter-weather event, utility failure (water, sewer, heat, gas), active-shooter / lockdown, bomb threat, and medical emergency. The plan shall be reviewed annually by the QP, posted in the staff area, and reviewed with all staff at orientation and annually thereafter. Drills shall be conducted per the cadence in §9.2 and §9.4(f).

9.7(0) Coordination with Local Emergency Management (OEM). The QP shall coordinate the written Disaster & Emergency Plan with the **local Office of Emergency Management (OEM)** for the county in which the facility is located (e.g., Wake County Emergency Management, Cumberland County Emergency Services, Durham County Emergency Management). Prior to initial licensure and annually thereafter, the QP shall: (a) submit a current copy of the facility Disaster & Emergency Plan to the local OEM; (b) request and retain written verification from the local OEM acknowledging receipt and confirming coordination; (c) confirm the facility's 9-1-1 dispatch address, emergency-contact roster, and any special-needs registry enrollment for youth with mobility, sensory, or medical vulnerabilities; and (d) confirm the local OEM's role in the event of a community-wide evacuation order. The local OEM's written acknowledgment shall be retained in the facility compliance binder and made available to the DHSR MHLC Licensure & Training Consultant and surveyors upon request. The North Carolina Emergency Management directory is available at <https://www.ncdps.gov/our-organization/emergency-management>.

(a) Fire. Evacuate immediately upon alarm. Headcount at the rally point (designated meeting area at the front sidewalk, ≥50 ft from the building). Call 911 from the rally point. Do not re-enter until the fire department issues all-clear. Monthly drills required (per §9.2).

(b) Tornado. On a Tornado Watch, alert all staff and review shelter assignments. On a Tornado Warning, move all youth to the lowest interior room (basement preferred; if no basement, an interior ground-floor bathroom or hallway with no exterior walls). Turtle position against interior walls, away from windows. Bring flashlights, weather radio, and youth MARs. Quarterly drills required.

(c) Hurricane. For facilities in NC counties subject to hurricane risk (all NC counties east of I-95 plus coastal counties), the QP shall monitor the National Hurricane Center advisories during the Atlantic hurricane season (June 1 – November 30). At Tropical Storm Warning or Hurricane Watch, the QP shall: secure outdoor furniture and projectiles; verify generator fuel (if applicable); stock 7-day supply of food, water (1 gallon per person per day), medications, batteries, and flashlights; verify youth and staff emergency-contact lists; and contact guardians regarding the storm plan. At Hurricane Warning, the QP shall coordinate with the LME/MCO and guardian regarding pre-storm evacuation or shelter-in-place decision; if evacuation is ordered, transport youth to the designated host facility (identified in the plan) and document the move on the IRIS

system. After the storm, conduct a facility damage assessment before re-occupancy, document in the compliance binder, and notify DHSR MHLC if any structural damage occurred.

(d) Power Outage. Distribute flashlights (one per staff member plus one per youth bedroom). Verify life-safety systems on battery backup (smoke detectors, fire alarm, security chimes). Discard refrigerated food per FDA guidance (4-hour rule for refrigerator; 24-48 hours for freezer if door stays closed). If power loss is expected to exceed 4 hours OR if indoor temperature drops below 65°F or rises above 80°F, initiate the Emergency Relocation Plan. Notify the utility company, document the outage start/end times, and notify the QP. Generator backup is recommended (and may be required by local code for facilities serving medically fragile youth); if installed, the generator shall be tested monthly under load.

(e) System Failure (heat, water, sewer, gas). Per Protocol 19: heat loss exceeding 4 hours OR water loss exceeding 8 hours OR sewer backup OR gas leak (evacuate immediately and call 911) shall trigger the Emergency Relocation Plan to the designated host facility.

(f) Lockdown (active shooter / external threat). Per Protocol 19: secure all exterior doors and windows, pull window blinds, silence phones, hide in the designated safe room away from windows and doors, do not open for anyone but law enforcement. Annual lockdown drills required. Staff shall familiarize themselves with the Run / Hide / Fight protocol and the facility's designated safe rooms at orientation.

(g) Emergency Relocation Plan. The written plan identifies a primary and a secondary host facility (typically another licensed group home or a hotel with prior arrangement), transportation arrangements (facility vehicle + staff vehicles + ambulance for medically fragile youth), medication transport (locked medication box transported by the RN or QP), youth-identification packets (photo ID, Medicaid card, allergy / medication list), guardian-notification protocol (within 1 hour of relocation decision), and DHSR / LME-MCO notification (within 24 hours). The QP shall maintain the host-facility agreement letters in the compliance binder.

Web sources for §9.4–§9.7 regulatory citations (verified July 2026):

- **our operating standards (NC Administrative Code Title 10A Ch. 27 Subchapter G — Mental Health/DD/SA)** — <http://reports.oah.state.nc.us/ncac/title%2010a%20-%20health%20and%20human%20services/chapter%2027%20-%20mental%20health,%20community%20facilities%20and%20services/subchapter%20g/subchapter%20g%20rules.pdf>
- **NC OSFM 2012 NC Building Code Amendments §425 (Board & Care — each occupied story two means of egress; smoke detectors on all levels)** — <https://www.ncosfm.gov/2012-north-carolina-building-code-chapters1-1534-end-amendments/open>
- **NC Fire Code 2024 Chapter 10 Means of Egress** — https://up.codes/viewer/north_carolina/ifc-2021/chapter/10/means-of-egress
- **NC IFC 2021 Chapter 4 Emergency Planning & Preparedness (§401.6 Evacuation Drills)** — https://up.codes/viewer/north_carolina/ifc-2021/chapter/4/emergency-planning-and-preparedness
- **IBC 2021 §1030 Emergency Escape & Rescue Openings (5.7 sq ft / 24 in / 20 in / 44 in sill)** — <https://codes.iccsafe.org/content/IBC2021P2/chapter-10-means-of-egress>
- **Family Care Home Fire Safety (NC DHSR)** — https://info.ncdhhs.gov/dhsr/rules/2025/AdultCareHomeFamilyCareHomeRules/feb/10A_NCAC_13F_.0309.pdf
- **Adult Care Home Fire Safety (NC DHSR)** — https://info.ncdhhs.gov/dhsr/rules/2025/AdultCareHomeFamilyCareHomeRules/feb/10A_NCAC_13G_.0316.pdf
- **NC DHSR ACLS — License a Family Care Home (2-6 Beds)** — <https://info.ncdhhs.gov/dhsr/acls/flofch.html>
- **NC DHSR ACLS Fire Safety & Emergency Preparedness Training (PDF)** — <https://info.ncdhhs.gov/dhsr/acls/training/pdf/FireSafetyEmergency-PreparednessRegulations-NCAdultCareHomes.pdf>
- **NC Residential Smoke & CO Alarm Requirements (City of Durham summary of NC code)** — <https://www.durhamnc.gov/DocumentCenter/View/1012/Residential-Smoke-AlarmsCarbon-Monoxide-Alarms-PDF>
- **Orange County NC Smoke Alarms (NFPA 72 — every level, every bedroom)** — <https://www.orangecountync.gov/2391/Smoke-Alarms>

- **NFPA 101 Life Safety Code — Board & Care Occupancies (§32/§33) —**

<https://www.nfpa.org/codes-and-standards/nfpa-101-standard-development/101>

- **NCSL Carbon Monoxide Detector Installation Statutes by State —**

<https://www.ncsl.org/environment-and-natural-resources/carbon-monoxide-detector-installation-statutes>

- **NC Family Care Home structure rule — no more than 2 stories; second-floor residents require two direct exterior egress —** <https://ncbold.com/license/10134>.

10. Medicaid Billing, Documentation Compliance & Record Management

Reference: NC Medicaid CCP 8D-2; CMS Documentation Guidelines; RMDM Chapter 6; state law Ch. 66 Art. 40 (NC UETA); E-SIGN Act (15 U.S.C. § 7001 et seq.) · Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.24, Aug 2026 — Legislation-Free Public Edition) · §10

10.1 Service Notes — General Requirements.

Service notes are the supporting evidence of individual outcomes and must be individualized. **Photocopying or repeating notes verbatim from a prior date or another individual's record is strictly prohibited.** When referencing another individual, use initials, record number, or role (e.g., "sibling") rather than full name unless clinically pertinent and reviewed prior to release. **Timelines:** facility-based services (this program) must be documented at time of service or **within 1 standard business day (not to exceed 24 hours)**. **Late entries** are entries after the required timeframe, must be marked "Late Entry" with the date of entry and the date the service was provided (e.g., "Late Entry made on 7/15/26 for service provided on 7/10/26"), and require a dated signature. Entries exceeding the timeline can only be billed up to **7 business days** beyond the service date; after 7 days, the note is not billable.

10.2 Full Service Note — Required Content.

Every shift/service note must include:

Name of the individual receiving the service (on each page).

Service Record Number issued by the payor along with the **Medicaid Identification Number (MID)** (as applicable), or unique identifier issued by the entity.

Full date the service was provided (month/day/year).

Name of the service provided.

Type of contact: In person, Telehealth, Telephonic, or Collateral.

Place of service.

Purpose of the contact (tied to specific goals in the PCP/ISP).

Description of the interventions, treatment, and support provided (specific and individualized).

Duration: total time spent performing the service (must include active engagement). For this residential per diem service, total time is not required for billing, but interventions must accurately reflect treatment for the event/per diem.

Effectiveness of the intervention(s) and the **individual's response/progress** toward goal(s).

Authentication: electronic signature date stamp and credentials (or handwritten signature with date and credentials). **Use of cursive font in a Word document is NOT a valid electronic signature.** Pre- or post-dating signatures is prohibited.

Additional requirements per Medicaid CCP or State-funded service definition.

Program, Intervention, and Evaluation (PIE) elements are part of the full service note. All required elements must be included for billing and audit purposes. Use Form 7 (Full Service Note Template) as the standardized format.

10.3 Shift Notes (Twenty-Four-Hour Services).

For 24-hour facilities, there must be a note for each shift, and **coverage hours for each shift must be clearly identified** in each note. If no intervention occurs (e.g., youth asleep or at school), the shift note shall reflect the care, oversight, support, and non-treatment events during the shift. When more than one staff person is providing services for a shift, **only one staff is required to document and sign the shift note**; however, the note must **identify the names and titles/positions of other staff persons present** to demonstrate staffing ratios are met.

10.4 Modified Service Notes & Grids.

Modified notes may be used only for services where explicitly allowed in CCP/State Service Definitions (e.g., Respite). Modified notes must include: Name, Service Record #/MID/Unique ID, Service provided, Date, Duration, Goal addressed/tasks performed, and Full dated signature/credentials (or initials if full signature is on the page).

Grids may be used only for specified services (e.g., Residential Supports — NC Innovations, Respite). Grids must include: Name, Service Record #/MID/Unique ID, Service, Date, Duration, Goals/Interventions, and Full dated signature/credentials (or initials if full signature is on the page). For NC Innovations, grids must include an intervention key, progress key, and a comment section with dated entries. **This facility will primarily use full shift notes.** If using grids for Respite, the grid must meet all requirements above.

10.5 Alterations to Service Documentation.

The original entry must not be deleted or altered so it is not clearly legible in its original context. Corrections must be made by the person who initially authored the entry, with an explanation of the correction. If that is not possible, the person updating the documentation must reflect the needed change and the rationale. Any change must be **signed and dated**. The revised/corrected entry must be in proximity to the original entry. EHR systems with tracked changes and timestamps satisfy these guidelines. **Alterations exceeding 7 business days beyond the service date are not billable.**

10.6 Authenticating Service Documentation.

All entries are validated via signature (EHR or paper) with signature date, credentials, degree, licensure, and/or title.

Initials are only permitted for updates/corrections or for notations entered on a service grid (where full signature is noted on the grid signifying appropriate initials). **Pre- or post-dating signatures in any form is prohibited.**

ADA Accommodations: if an individual has a documented medical/physical reason (per ADA) for not being able to sign, another means for providing signature is acceptable; the process must be incorporated in facility policy.

Unavailable Original Author: if the original author is no longer available to sign or amend an entry, a notation reflecting this shall be documented in the record and signed/dated by the appropriate party. **Rubber Stamps:** only used for medical/physical reasons with ADA accommodations; if unable to use stamp, the individual may authorize someone of their choosing in writing.

10.7 Electronic Signatures.

Electronic signatures are permitted for authenticating service documentation, consents, treatment plans, and other records governed by this manual, in accordance with the **North Carolina Uniform Electronic Transactions Act** and the federal E-SIGN Act (15 U.S.C. § 7001 et seq.). An electronic signature is an electronic sound, symbol, or process attached to or logically associated with a record and executed or adopted by a person with the intent to sign the record. Where electronic signatures are used, they carry the same legal weight and enforceability as handwritten signatures for transactions conducted electronically. **Use of a cursive font in a word-processing document does not constitute a valid electronic signature**; the signature must be applied through the facility's authenticated EHR/EMR system or an approved secure signing platform that captures the signer's identity, timestamp, and intent. Each electronic signature shall be uniquely linked to the signer, render any subsequent modification detectable, and include an auditable timestamp reflecting the date and time of execution.

10.7(a) Safeguards.

The facility shall implement and maintain reasonable administrative, technical, and physical safeguards to ensure the integrity, confidentiality, and non-repudiation of electronic signatures, consistent with the HIPAA Security Rule (45 CFR Part 164 Subpart C), 42 CFR Part 2, and state law § 66-40(d). Required safeguards include: (a) **unique user credentials** for each authorized signer, with shared logins strictly prohibited; (b) **multi-factor authentication** where technically feasible; (c) **automatic session timeout** after no more than 15 minutes of inactivity; (d) **encrypted transmission** (TLS 1.2 or higher) and **at-rest encryption** of all signed records; (e) **immutable audit trails** capturing the signer's identity, date, time, IP address or device identifier, and the document version signed; (f) **role-based access controls** limiting signature authority to staff with appropriate licensure, credentials, and documented training; (g) **immediate revocation** of credentials upon termination, role change, or suspected compromise; and (h) **annual review** by the QP and IT vendor to verify continued compliance. Lost, stolen, or shared credentials must be reported to the QP within one business day; the facility shall deactivate the compromised credentials and investigate the scope of any unauthorized signatures before reissuing access.

10.7(b) System Unavailability Procedures.

In the event the EHR/EMR system or approved electronic signing platform is unavailable — whether due to planned maintenance, internet outage, vendor outage, cyber-incident, or natural disaster — staff shall revert to paper-based documentation and handwritten signatures with date, credentials, and title. The QP or designee shall declare a **Documentation Continuity Event**, notify all shifts in writing (or by phone tree if email is unavailable), and distribute blank copies of the required forms (Shift Note, MAR, Restraint & Debriefing Checklist, Incident Report). Staff shall legibly sign and date each paper entry in blue or black ink. When the electronic system is restored — and in no event later than **72 hours** after the original entry — the originating staff member (or, if unavailable, the on-duty QP) shall transcribe the paper entry verbatim into the electronic record, mark the entry as "*Late Entry — System Unavailability (date/time of restoration)*", reference the original paper document by scan or filename, and electronically sign the transcribed entry. The original paper document shall be scanned and attached to the electronic record, then retained as a source document per the record retention policy (§1.6) for the full retention period. Paper entries that cannot be transcribed within **7 business days** must be reviewed by the QP, who shall document the reason for delay and the corrective action taken. The QP shall maintain a log of all Documentation Continuity Events, including the start/end time, cause, records affected, and verification that all paper entries were transcribed and electronically authenticated.

10.8 Service Authorizations & End-Date Reporting.

The facility shall maintain service authorizations/reauthorizations in a separate audit file (not required in clinical record but available upon request). When a youth changes providers or ends a service, the facility must notify the authorizing entity per their specified protocol. End-date reporting shall follow the authorizing entity's requirements.

10.9 Billing.

The program bills Medicaid on a daily per diem basis under **NC Medicaid Clinical Coverage Policy 8D-2 (Residential Treatment Services — Level III)** via NCTracks. **The Medicaid RTS per-diem covers ONLY the clinical/treatment/milieu component** (assessments, PCP, individual/family/group therapy, behavioral interventions, 24-hour supervision, medication administration, crisis response, and related clinical services). **Room and board are excluded from the Medicaid RTS benefit category** per CCP 8D-2 §1.0(c) and §1.2(g) of this Manual; the beneficiary's lodging and meals must be funded through a non-Medicaid source (state/local social-services funds, Title IV-E foster care maintenance, SSI/ISS, or other third-party resources). The QP and Billing Coordinator shall verify at admission — and re-verify at each PCP review — that an identified non-Medicaid room-and-board funding source is documented in the youth's clinical record. **Under no circumstances shall room-and-board costs be billed to the Medicaid RTS per-diem, to Alliance Health, or to**

NCTracks. Suspected improper billing of room-and-board costs to Medicaid shall be reported immediately to the Compliance Officer per §1.3 and shall be corrected via NCTracks claim adjustment within 30 calendar days of discovery.

Billing is suspended for any day the youth is hospitalized, detained, or on a home pass exceeding **24 consecutive hours**. The Home Pass & Medicaid Billing Exclusion Tracker (Form 4) documents all absences and corresponding billing action. Billing errors are corrected promptly and reported to the LME/MCO as required. Records are released only with a signed ROI or as required by law, court order, or regulatory audit.

10.10 Mock Client Chart & Licensure Survey Readiness.

Prior to the on-site **DHSR MHLC Licensure Survey** (and prior to any re-licensure or accreditation survey), the QP shall prepare and assemble a complete **Mock Client Chart** representative of a typical youth served by the facility. The Mock Client Chart shall be made available to the surveyor on the first day of the on-site survey and shall demonstrate the facility's documentation practices across the full continuum of care. The Mock Client Chart shall include the following elements (cross-referenced to the applicable SOP section and Form number in this Manual):

Identification Face Sheet — youth name, DOB, Medicaid ID, address, guardian name and contact, primary language, admission date, assigned QP and psychiatrist (per §3.2, §3.3).

Emergency Information Sheet — emergency contacts, allergies, primary physician, preferred hospital, pharmacy, and code-status (if applicable).

Consent for Treatment — signed by guardian (and youth if age-appropriate), including consent for medication, transportation, photography, and Release of Information (ROI) (per §3.2).

Comprehensive Clinical Assessment (CCA) — completed by a licensed professional within the required timeframe, including DSM-5-TR/ICD-10 diagnoses, clinical formulation, and recommended level of care (per §4.1).

Person-Centered Plan (PCP) — signed by all participants, with measurable goals, objectives, interventions, and assigned responsible staff; reviewed every 90 days (per §4.2, §4.3).

Progress Notes / Service Notes — daily shift notes, weekly clinical progress notes, group notes, and any 1:1 service notes (per §10.1, §10.2, §10.3).

Medication Orders & Medication Administration Record (MAR) — signed physician medication orders, current MAR showing all PRN and scheduled administrations, and any medication-incident reports (per §6.3, §6.4).

Behavior Support Plan (BSP) — if applicable, with proactive strategies, replacement skills, de-escalation steps, and crisis responses (per §5.3).

Restrictive-Intervention Records — if applicable, all restraint, seclusion, and isolation time-out documentation with post-incident medical monitoring, guardian notification, and IRIS reports (per §5.4, §8).

Health Records — pre-admission physical exam report (within 90 days prior to admission per §6.1), immunization record, TB screening, dental and vision records, and annual exam documentation.

Activities Log — weekly group activities participation showing the 14-hours-per-week minimum per §5.5.

Grievance Records — if applicable, any grievances filed by or on behalf of the youth, with documentation of acknowledgement, investigation, response, and resolution per §1.7(c).

Disclosures & Accounting of Disclosures — Form 9 completed for any PHI disclosures during the chart period (per §11.4).

Discharge / Transition Plan — initiated at admission, reviewed every 30 days, with projected discharge date and identified next placement (per §3.4).

In addition to the Mock Client Chart, the QP shall prepare and make available to the surveyor: (a) **Facility Photographs** — labeled photographs of each room in the facility (exterior, common areas, kitchen, youth bedrooms, staff sleeping areas if applicable, bathrooms, medication storage area, time-out room if applicable, exits, fire-safety equipment, and outdoor recreation area), with each photograph labeled to identify the room; (b) **all**

policies and procedures referenced in this Manual; (c) **all personnel files** for current staff with background-check, training, CEU, and instructor-credential documentation per §2; (d) **all disaster and emergency plans** including OEM coordination per §9.7; (e) **the MH Licensure P&P Worksheet** per §1.2(f); and (f) **the compliance binder** containing all corporate, financial, insurance, and licensing documents per §1.8. The QP shall conduct a **mock survey** no fewer than 30 days prior to the scheduled DHSR MHLC survey, identify and correct any deficiencies, and document the mock-survey findings and corrective actions.

11. Privacy, Security, Confidentiality & Access to Records

Reference: Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.24, Aug 2026 — Legislation-Free Public Edition) · §11

11.1 Privacy and Security Policies.

The facility shall maintain comprehensive policies addressing how information is recorded, stored, retrieved, disseminated, and protected against loss, theft, destruction, unauthorized access (breach), and natural disasters. Compliance with the HIPAA Privacy and Security Rules, the Omnibus HIPAA final rule, the HITECH Act, and ARRA is mandatory. Policies shall address electronic communication (email, voice messaging, SMS/text speak, EHR usage). Where multiple requirements exist, the **most stringent** applies, including professional codes of ethics.

11.2 Safeguards.

Formal policies and procedures shall reasonably protect against unauthorized uses and disclosures of PHI, and against reasonably anticipated threats or hazards (42 CFR § 2.16). Secure storage: locked, climate-controlled areas with limited access; electronic records backed up daily and stored securely off-site.

11.3 Confidentiality.

Compliance is required with our resident rights commitments, our operating standards, **42 CFR Part 2** (Confidentiality of Alcohol and Drug Abuse Patient Records), and state law on AIDS/confidential conditions. **Substance Use Information:** requires the individual's written authorization before disclosure to other treatment providers (unless court order or allowable exception per 42 CFR Part 2). Absent written consent, entities must redact all identifying information from alcohol/drug records when sharing. **Care Coordination Exceptions:** Under our operating standards, entities may share confidential information for coordination of care/treatment without written consent; however, 42 CFR Part 2 still requires written authorization for SUD records.

11.4 Disclosure Documentation and Accounting.

Authorization must be in writing, voluntary, and informed (understanding what is exchanged, with whom, and for what purpose). Staff must verify the identity of the person requesting PHI and obtain required documentation/representations per Omnibus HIPAA rules. The **minimum necessary** standard (164.502(b)) applies. **Accounting of Disclosures** must be maintained for a minimum of **six years**. The accounting must include: name of the individual; medical record or ID number; date the information was released/disclosed; provider/entity/agency/individual to whom information was released; purpose of the release/disclosure; description of specific information released/disclosed; and name of person disclosing the information (recommended). All disclosures or re-disclosures of SUD treatment information must comply with 42 CFR Part 2; re-disclosure is prohibited except as permitted under our resident rights commitments. Releases/disclosures from external entities must be contained in the individual's service record.

11.5 Individual Access to Service Records.

Individuals and Legally Responsible Persons (LRPs) have the right to access information contained in their service record, per state law and DHHS provisions. **Limited Access:** if access is restricted, proper justification for

restricting access must be clearly indicated in the service record. Individuals have the right to appeal such determination. Refer to Article 3, Part 1 (Client Rights), the Omnibus HIPAA Final Rule, and DHHS Privacy/Security policies.

11.6 Transporting Records.

Service records shall only be transported by designated individuals (hard copy, USB, tablet, CPU, etc.). When removed from premises, records shall be secured in a locked compartment and protected from unauthorized access. Policy must address what occurs if information is lost or stolen during transport, including immediate notification of the QP, Compliance Officer, and — if PHI is compromised — notification of affected individuals and DHHS per breach notification rules.

11.7 Storage and Maintenance.

Storage and maintenance are consistent with privacy/security principles. Electronic records self-warranty per NC State Archives guidelines. "Managing Public Records Produced by Information Technology Systems" guidelines apply. The QP conducts quarterly audits of storage security and access logs.

12. CARF Accreditation Conformance Framework

Reference: Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.24, Aug 2026 — Legislation-Free Public Edition) · §12

12.0 Purpose & Scope. This section establishes the written policies, plans, procedures, and committees required to demonstrate conformance to the applicable standards of the **2026 CARF Child and Youth Services (CYS) Standards Manual** at the time of an **Inaugural One-Year Accreditation survey**, per the **2026 CYS Inaugural Accreditation Guidelines** (effective July 1, 2026 – June 30, 2027). The standards applied on an Inaugural Accreditation survey are a subset of the standards in the current 2026 CYS Standards Manual. Where the program will not have commenced service delivery at the time of the survey, Subsection A (Not Operational) applies; where service delivery will have commenced, Subsection B (Operational) applies, and the additional standards identified as inapplicable in Subsection B are deferred to the subsequent resurvey (approximately 10–11 months after the Inaugural survey). The QP shall document the selected subsection in the compliance binder prior to survey application. Each subsection below identifies the applicable CARF CYS standard area, the SOP location(s) where existing policies satisfy the standard, and any additional written policy, plan, or procedure required to demonstrate conformance. Where a standard requires a written plan, the plan must address all areas of the standard, be complete, and be in use or ready for implementation, as applicable, at the time of the survey. By CARF definition, a policy must be in writing; a plan must be in writing; a procedure must be in writing. Forms in use (or ready for use) by the program may be used to demonstrate conformance to some standards. Personnel should be able to discuss and show that they are familiar with and understand how to implement the CARF standards as applicable to their jobs. If service delivery has commenced, personnel should also be able to provide evidence of implementation. See Appendices A, B, and C of the 2026 CYS Standards Manual for documentation, timeline, and education/training requirements.

12.1 ASPIRE to Excellence® — Leadership & Strategic Planning.

Conformance to CARF CYS Section 1.A (Leadership) and Section 1.C (Strategic Planning) is established through the written **Strategic Plan** maintained by the Executive Director and reviewed at least annually by the Governing Body. The Strategic Plan addresses: (a) the organization's mission, vision, and values statements (§1.1); (b) a statement of the populations and services to be provided; (c) an environmental analysis considering input from persons served, personnel, funders, and other stakeholders; (d) strategic goals and measurable objectives for at least a three-year horizon; (e) action plans identifying responsible parties, resources, timelines, and evaluation methods;

(f) input from persons served and other stakeholders (per §12.2) documented and incorporated; and (g) a written succession plan for the Executive Director and other key leadership positions. The Governing Body reviews and approves the Strategic Plan, monitors progress at each quarterly meeting (per §1.8), and authorizes revisions as needed. The QP retains the most recent Strategic Plan, meeting minutes documenting Governing Body review, and any interim revisions in the compliance binder. Leadership conformance under 1.A Standards 1–3, 5.a, 6.a–c, 7, 8 is demonstrated by the Governing Body charter and bylaws, position descriptions for the Executive Director and Clinical Director, the code of conduct/ethics policy, and the organizational chart. If the organization directly solicits charitable financial support, Standard 1.A.9 conformance is demonstrated by a written fundraising policy consistent with the organization's charitable solicitation registration.

12.2 Input from Persons Served and Other Stakeholders.

Conformance to CARF CYS Section 1.D is established through the written **Stakeholder Input Plan** maintained by the QP. The plan describes at least four methods for gathering input from persons served, their families/LRP, personnel, referral sources, payers, and the community, including: (i) quarterly youth satisfaction surveys (developmentally appropriate, anonymous, available in English and the youth's preferred language); (ii) semi-annual family/LRP feedback surveys; (iii) annual personnel engagement surveys; and (iv) community/stakeholder advisory convenings at least annually. Input is aggregated, analyzed for trends, and presented to the Governing Body at each quarterly meeting. The QP maintains a Stakeholder Input Log documenting the method, date, respondent count, key themes, and resulting action items. Person-served input directly informs the Person-Centered Plan (§4.1), program structure (§5), and the annual Strategic Plan revision (§12.1). The QP documents how input was used and communicates outcomes back to respondents through visible "You Said / We Did" postings in the facility common area and in resident community meetings. The organization does not retaliate against any person served, family member, or personnel for providing input, per §1.10 Non-Retaliation policy.

12.3 Legal Requirements.

Conformance to CARF CYS Section 1.E (Standards 1–3) is established through the written **Legal Compliance Plan** maintained by the Executive Director. The plan identifies all applicable federal, state, and local legal requirements governing the organization and the program, including: licensure as a Level III Residential Treatment Facility — Staff Secure (§1.2); Medicaid provider enrollment and the federal health-care-program fraud and abuse laws (§10); the federal health-privacy law and 42 CFR Part 2 (§11); state child-mandated-reporter law (§8); the federal disability-rights law (ADA) and Section 504 of the Rehabilitation Act (§12.9); the federal civil-rights law (Title VI); state employment law; and applicable zoning, fire, building, and life-safety codes (§9). The Executive Director designates a Compliance Officer (may be the QP) responsible for monitoring changes in legal requirements, coordinating the annual legal-compliance review, and reporting any identified violations or potential violations to the Governing Body within five business days of discovery. Written acknowledgment of legal compliance is provided annually by the Executive Director and Clinical Director. The plan includes a written response procedure for subpoenas, court orders, and government investigations, coordinated with the organization's legal counsel. The Compliance Officer maintains a Legal Compliance Register listing each requirement, the controlling authority, the responsible position, the most recent review date, and the next review due date.

12.4 Financial Planning and Management.

Conformance to CARF CYS Section 1.F is established through the written **Financial Plan** maintained by the Executive Director and reviewed by the Governing Body at each quarterly meeting. The Financial Plan includes: (a) an annual operating budget covering all revenue sources (Medicaid, room-and-board, grants, charitable contributions) and all expense categories (personnel, facility, clinical, administrative); (b) a cash-flow projection; (c) a capital-replacement reserve sufficient to address expected major repairs and replacements over a rolling five-year horizon; (d) written internal controls addressing segregation of duties, authorization thresholds, bank reconciliation,

and segregation of resident trust funds from operating funds; (e) an annual independent financial audit (or reviewed financial statements for organizations below the audit threshold) with management response to any findings; (f) a written billing and collections policy consistent with §10.9 and Medicaid requirements; and (g) a written records-retention schedule for financial documents. Written cost reports are submitted to Medicaid and to applicable funders per their respective deadlines. The QP coordinates with the Billing Coordinator (per Protocol 22) to ensure that all service documentation supports billed claims. The Financial Plan is reviewed at each quarterly Governing Body meeting; material variances (>10% line-item variance) are documented with corrective action.

12.5 Risk Management & Enterprise Risk Register.

Conformance to CARF CYS Section 1.G (Standards 1–4) is established through the written **Enterprise Risk Management Plan** maintained by the Compliance Officer. The plan includes: (a) a written risk-management policy identifying the Compliance Officer as the risk-management lead and describing the risk identification, assessment, mitigation, and monitoring process; (b) an **Enterprise Risk Register** identifying at least the following risk categories — clinical (suicide risk, elopement, restraint-related injury, medication error), operational (staffing shortage, capacity under-utilization, IT system failure), financial (Medicaid audit finding, payer mix concentration, cash-flow shortfall), legal/compliance (privacy breach, licensing deficiency, employment claim), reputational (adverse media, community concern), and external (natural disaster, public-health emergency); (c) for each identified risk, a documented likelihood, impact, mitigation action, responsible position, and review date; (d) a written business-continuity plan coordinated with §9 Emergency Operations Plan and Protocol 19; (e) an annual risk-management review presented to the Governing Body; and (f) a written incident-reporting procedure consistent with §8 (IRIS) that ensures serious incidents are reviewed by the Compliance Officer within one business day and trended monthly. The Enterprise Risk Register is reviewed at each quarterly Governing Body meeting and following any serious incident.

12.6 Health & Safety Committee.

Conformance to CARF CYS Section 1.H (Standards 1, 2, 4.a, 5, 6, 8, 9.b(2)-(3), 10, 12, 14–16) is established through the facility's existing §9 Facility, Safety, and Environmental Management SOP and the standing **Health & Safety Committee** chartered by this section. The committee meets at least monthly and includes the QP (chair), the House Manager, a Direct Care Professional representative, the RN or designee, and the Compliance Officer. The committee reviews: (i) monthly environmental safety inspection results (Form 5); (ii) fire, tornado, and lockdown drills per shift per §9.2; (iii) incident reports (§8) trended by type, location, shift, and personnel; (iv) infection-control surveillance per §6.1 and Protocol 18; (v) medication-error trends per §6.3; (vi) work-related injury trends; (vii) equipment and physical-plant work orders; (viii) safety-plan revisions; and (ix) emergency-preparedness plan revisions. The committee maintains written minutes documenting attendance, topics reviewed, decisions made, and action items with responsible parties and due dates. The QP reports committee activities and trends to the Clinical Director at each quarterly compliance report (per §1.4(a)) and to the Governing Body at each quarterly meeting. Written health-and-safety policies addressing emergency response, infection control, hazard communication, bloodborne pathogens, workplace violence prevention, and hot-work/environmental safety are maintained in the facility policy library and reviewed at least annually.

12.7 Workforce Development & Management.

Conformance to CARF CYS Section 1.I (Standards 1–13) is established through the written **Workforce Development Plan** maintained by the QP. The plan includes: (a) a written recruiting and hiring policy (§2.2), including background-check requirements per §2.3, reference verification, and credential verification per §1.4(b) and §2.2; (b) a written orientation curriculum completed by every new personnel member prior to working independently with persons served, covering at minimum the organization's mission and values; person-centered and trauma-informed care; the Person-Centered Plan process (§4.1); behavioral support and restraint (§5); medication

administration (§6.3); infection control (§6.1, Protocol 18); incident reporting (§8); privacy and confidentiality (§11); cultural and linguistic competency; mandated reporting; emergency procedures; documentation requirements (§10); resident rights; and the personnel code of conduct; (c) a written annual training plan covering refresher training and competency verification for all personnel, with documented completion dates and trainers; (d) written position descriptions for every role, consistent with §2; (e) a written performance-evaluation policy conducted at minimum annually for every personnel member, with documented competency assessment against position-specific criteria; (f) a written supervision policy consistent with §2.2(a) Individualized Supervision Plans; (g) a written personnel records policy consistent with §2.4; (h) a written volunteer and intern management policy per §1.8; (i) a written continuing-education support policy; (j) a written succession plan for key positions; and (k) a written workforce engagement and recognition plan. The QP maintains the master training calendar and individual personnel training records in the personnel file. The Workforce Development Plan is reviewed at least annually and updated to reflect changes in CARF standards, regulatory requirements, or program design.

12.8 Rights of Persons Served.

Conformance to CARF CYS Section 1.J (Standards 1–3) is established through the facility's existing Resident Rights policies as documented in the Resident Handbook (companion document) and in §3, §5, §6, §8, §9, and §11 of this Manual. At minimum, written policies address: (a) the right to dignity, privacy, humane care, and freedom from discrimination; (b) the right to participate in service planning and to refuse services (with documented consequences of refusal); (c) the right to confidential communication and visitation; (d) the right to access one's own record per §11.5; (e) the right to file a grievance without retaliation, with a written grievance procedure acknowledging receipt within two business days and resolving within 30 calendar days, with appeal to the Executive Director and external advocacy resources; (f) the right to be free from unnecessary restraint and seclusion per §5; (g) the right to informed consent for treatment and medication; (h) the right to religious freedom and reasonable accommodation; (i) the right to personal funds management per the financial controls in §12.4; and (j) the right to advance written notification of any discharge or transfer per §3.4(a). Personnel are trained on resident rights at orientation and annually thereafter. The QP audits grievance logs, restraint logs, and rights-related incident reports quarterly and reports trends to the Governing Body.

12.9 Accessibility & Nondiscrimination.

Conformance to CARF CYS Section 1.K (Standards 1, 2.a–b(2)) is established through the written **Accessibility & Nondiscrimination Plan** maintained by the Compliance Officer. The plan includes: (a) a written nondiscrimination policy consistent with applicable federal civil-rights law (Title VI, ADA, Section 504, Age Discrimination Act) and state law, prohibiting discrimination on the basis of race, color, national origin, religion, sex, gender identity or expression, sexual orientation, age, disability, veteran status, or any other protected classification; (b) a written language-access plan providing for qualified interpreter services at no cost to the youth and family, translation of vital documents into the languages commonly encountered in the service area, and primary-language preference documentation in the clinical record; (c) a written reasonable-accommodation policy for youth, family, personnel, and visitors with disabilities, addressing physical accessibility of the facility, communication accommodations, and programmatic accommodations; (d) a written accessibility review of the physical facility conducted at least annually, identifying barriers and corrective actions; and (e) a written grievance procedure for nondiscrimination complaints, distinct from but coordinated with the resident grievance procedure in §12.8. The plan is reviewed at least annually and updated to reflect changes in the served population, facility, or legal requirements.

12.10 Performance Management & Measurement.

Conformance to CARF CYS Section 1.L (Standards 1, 2.a(2)/b–g, 3.a) is established through the written **Performance Measurement Plan** maintained by the QP. The plan identifies the organization's performance indicators, organized into at least the following domains: (i) **access** — time from referral to admission decision,

time from admission to first clinical contact, no-show rate; (ii) **person-centered planning** — PCP completed within required timeframe, PCP reviewed at required intervals, youth/family participation in PCP meetings; (iii) **clinical outcomes** — symptom measures (e.g., Columbia ADHD, PHQ-A, GAD-7), functional measures (e.g., CAFAS, GAF), restraint-use frequency and duration, elopement frequency, incident frequency and severity; (iv) **safety** — incident-free days, time-to-investigation, repeat-incident rate, medication-error rate; (v) **personnel** — turnover rate, time-to-fill, training completion rate, personnel engagement score; (vi) **financial** — days-cash-on-hand, Medicaid denial rate, cost per youth per day; and (vii) **person-served experience** — satisfaction survey scores, complaint volume and resolution time. The QP aggregates data monthly, analyzes trends quarterly, presents findings to the Governing Body at each quarterly meeting, and incorporates findings into the annual Strategic Plan revision (§12.1) and the Quality Improvement Plan (§12.20). Performance data is shared with personnel and with persons served (in developmentally appropriate, aggregated form) at least quarterly.

12.11 General Program Standards — Program/Service Structure.

Conformance to CARF CYS Section 2.A is established through the written **Program Description** maintained by the Clinical Director and approved by the Governing Body. The Program Description documents: (a) the populations served, including age range, diagnostic profile, and admission criteria per §3.1; (b) the service array, including residential care, clinical services (§4), behavioral support (§5), medication management (§6), educational support (§7), and family engagement; (c) the program philosophy and trauma-informed care model; (d) staffing patterns per §2; (e) physical-environment description per §9; (f) hours and days of operation; (g) referral and intake process per §3 and Protocol 1; (h) cultural and linguistic competency plan; (i) coordination with external systems of care (LME/MCO, school, DSS, juvenile justice, medical home); (j) the Person-Centered Plan process per §4.1; (k) discharge and transition planning per §3.4 and §3.6; (l) records of persons served per §1.6 and §11; and (m) quality records management per §12.17. The Program Description is reviewed annually and updated to reflect changes in program design, population, or regulatory requirements. A mock chart containing forms for use with or by persons served is maintained per the CARF CYS General Program Standards note.

12.12 Screening and Access to Services.

Conformance to CARF CYS Section 2.B is established through the written **Screening and Access Policy** maintained by the QP. The policy documents: (a) the written referral intake process per §3.1 and Protocol 1; (b) the screening criteria and decision authority; (c) the response timeframe for referral acknowledgement and admission decision; (d) the written waitlist management procedure if applicable; (e) the written emergency/crisis access procedure; (f) the written procedure for denying admission, including appeal rights and notification of referral source; (g) coordination with the LME/MCO and Tailored Plan access line; and (h) nondiscrimination in access per §12.9. The Screening and Access Policy is reviewed annually. The QP maintains access data (referrals received, admissions, denials with reasons, time-to-decision) and reports to the Governing Body quarterly as part of the performance measurement report (§12.10).

12.13 Individualized Planning.

Conformance to CARF CYS Section 2.C is established through the written Person-Centered Plan policy and procedure per §4.1 of this Manual. The QP ensures that every youth has an individualized, person-centered plan developed within required timeframes, reviewed at required intervals, and signed by the youth (as developmentally appropriate), the family/LRP, the QP, and other CFT members. The plan addresses strengths, needs, goals, objectives, services, responsible parties, and review dates. The plan reflects input from the youth and family/LRP per §12.2. Service delivery is consistent with the plan, and progress is documented in the service record per §10. The QP audits PCPs for completeness and timeliness as part of the quarterly Clinical Record Content Checklist audit (Form 8).

12.14 Transition/Discharge Planning.

Conformance to CARF CYS Section 2.D is established through the written transition and discharge planning policy per §3.4 of this Manual. The QP ensures that transition planning begins at admission, that the youth and family/LRP participate in transition planning, that the receiving provider (if any) is identified and engaged, that the discharge summary is completed and provided to the youth and family/LRP and to the receiving provider, and that post-discharge follow-up occurs per Protocol 11. Advance written notification of discharge is provided per §3.4(a). Emergency discharges are followed by the 5-business-day post-emergency service-planning meeting per §3.4(c). The 18th-birthday continuation policy per §3.6 applies.

12.15 Medication Use.

Conformance to CARF CYS Section 2.E is established through the written medication use policies in §6.3 of this Manual. If the program physically controls medications, the policies address receipt and verification per §6.3(b), storage per §6.3(c), disposal documentation per §6.3(d), administration, medication-error reporting, and the 6-month psychotropic drug regimen review per §6.3(a) and Form 11. If the program prescribes or administers medications, additional written policies address prescriber credentials, order verification, administration by qualified personnel, documentation in the medication administration record (MAR), medication reconciliation at admission and discharge, and medication education per §6.3(e). Medication policies are reviewed at least annually.

12.16 Promoting Nonviolent Practices.

Conformance to CARF CYS Section 2.F is established through the written behavioral support and restraint policies in §5 of this Manual. The organization is committed to promoting nonviolent practices and to the minimization of restraint and seclusion. Written policies address: (a) trauma-informed, person-centered behavioral support; (b) preventive strategies and de-escalation; (c) the least-restrictive intervention principle; (d) restraint and seclusion use criteria, authorization, time-limits, monitoring, and documentation; (e) post-incident debriefing with the youth and personnel per §5.6 and Form 3; (f) restraint-use review by the QP and Clinical Director with trend reporting to the Governing Body; and (g) personnel training and competency verification in de-escalation and restraint per §2.5. If the program intends to use seclusion or restraint, Standard 2.F.10 conformance is documented through the written policy and the Physical Restraint Debriefing Checklist (Form 3).

12.17 Records of Persons Served & Quality Records Management.

Conformance to CARF CYS Section 2.G (Standards 1.a, 2–4, 6) and Section 2.H (Standard 4) is established through the written records-management policies in §1.6, §10, and §11 of this Manual, together with the written **Quality Records Review Procedure** maintained by the QP. The Quality Records Review Procedure describes the systematic review of clinical records for completeness, timeliness, and quality, conducted at minimum quarterly using the Comprehensive Clinical Record Content Checklist (Form 8). Findings are documented, trended, and reported to the Clinical Director and Governing Body. Deficiencies are addressed through personnel feedback, retraining, and policy revision as needed. Records are retained per §1.6 and the financial records-retention schedule in §12.4. Confidentiality of records is maintained per §11. The QP coordinates with the Compliance Officer to ensure that records-release accounting is maintained per §11.4 and Form 9.

12.18 Service Delivery Using Information and Communication Technologies.

Conformance to CARF CYS Section 2.I (Standards 1, 2) is established through the written **Telehealth & Technology-Mediated Service Delivery Policy** maintained by the Clinical Director. The policy documents: (a) the services that may be delivered via information and communication technologies (e.g., family therapy sessions, psychiatric medication management follow-up, clinical supervision, care-coordination meetings); (b) the technologies used and the platform's compliance with applicable privacy and security law (HIPAA, 42 CFR Part 2, HITECH) per §11; (c) informed consent for telehealth services, including documentation in the clinical record of

the youth's and family/LRP's understanding of the benefits, risks, limitations, and alternatives; (d) personnel training and competency verification for telehealth service delivery; (e) procedures for technology failure, including fallback to in-person or telephone contact; (f) procedures for ensuring the youth's privacy during telehealth sessions; (g) documentation requirements consistent with §10; and (h) coordination with the LME/MCO and payers regarding coverage of telehealth services. The policy is reviewed at least annually and updated to reflect changes in technology, legal requirements, or program design.

12.19 Core Program Standards, Core Residential Program Standards, and Specialty Designation Standards.

Per the 2026 CYS Inaugural Accreditation Guidelines, all standards in Sections 3, 4, and 5 of the 2026 CYS Standards Manual are applicable to the extent possible at the time of the Inaugural Accreditation survey. The QP shall complete a written crosswalk between the 2026 CYS Standards Manual Sections 3–5 standards and this Manual's SOPs, Protocols, and Forms, and shall retain the crosswalk in the compliance binder. Where a standard is not addressed by existing policy, the QP shall develop a written plan for addressing the standard prior to the subsequent resurvey. The crosswalk is reviewed at each quarterly QP compliance report (per §1.4(a)) and updated as standards are addressed or as the CYS Standards Manual is revised.

12.20 Quality Improvement Plan (QIP) — Inaugural Accreditation Condition.

Per Step 9 of the 2026 CYS Inaugural Accreditation Guidelines, within **90 days** after notification of accreditation, the organization shall submit to CARF a written **Quality Improvement Plan (QIP)** outlining the actions that have been or will be taken in response to the recommendations identified in the Inaugural Accreditation survey report. The QP is responsible for developing the QIP in coordination with the Executive Director and Clinical Director, and for monitoring implementation. The QIP shall: (a) restate each recommendation from the survey report; (b) describe the corrective action taken or planned; (c) identify the responsible position; (d) specify a target completion date; (e) identify the evidence of implementation that will be available at the subsequent resurvey; and (f) be approved by the Executive Director prior to submission. The QP shall maintain the QIP in the compliance binder, track implementation status at each quarterly compliance report (per §1.4(a)), and update CARF through Customer Connect as actions are completed. **Form 12 (CARF QIP Tracker)** in Part 3 is the standardized form for tracking QIP actions. The organization shall prepare for the subsequent resurvey by addressing all applicable standards in the current 2026 CYS Standards Manual and by completing all QIP actions. The subsequent resurvey will occur approximately 10–11 months after the Inaugural Accreditation survey; there is no application fee for the subsequent survey, and scheduling begins upon payment of at least half of the subsequent survey fee. Failure to complete the QIP within 90 days or failure to complete the subsequent survey within 12 months of the Inaugural survey will result in expiration of the Inaugural One-Year Accreditation.

CROSS-REFERENCE

This §12 framework operates in conjunction with §1.2(a) (CARF designation), §1.4(a) (QP compliance reporting), §1.8 (Governing Body), §2 (Workforce), §3 (Admissions/Discharge), §4 (Clinical Services), §5 (Behavioral Management), §6 (Health/Medication), §7 (Education), §8 (Incident Reporting), §9 (Facility/Safety), §10 (Medicaid Documentation), §11 (Privacy/Records), and the Form 12 (CARF QIP Tracker) in Part 3. The QP shall present this §12 framework to CARF surveyors as the primary evidence of organizational readiness for the Inaugural Accreditation survey.

PART 2

Actionable Workflows & Protocols

These 22 protocols translate the SOP policies into clear, sequential, step-by-step actions for daily operations. Each protocol is designed to be referenced quickly during a shift and followed exactly. Deviations from protocol require QP approval and must be documented. Protocols 20 and 21 are new in Version 2.0 and address service orders/authorizations and record management/disclosure accounting per RMDM requirements. Protocol 22 is new in Version 2.9 and codifies the daily workflow schedules for every personnel classification so that each shift, role, and hand-off has a documented time-blocked routine.

1. Admissions & Pending Record Protocol

Reference: Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.24, Aug 2026 — Legislation-Free Public Edition) · Part 2 · Protocol 1: Admissions & Pending Record Protocol

Pre-Admission. QP reviews referral packet, verifies active Medicaid, schedules tour with guardian and youth.

Pending Record Creation. If individual presents for screening but is not immediately enrolled, create a Pending Record with screening/consultation info.

Day of Admission. QP obtains all signed consents (treatment, medication, transportation, ROI, photos). DCP conducts respectful property search and secures medications. QP administers C-SSRS and documents clinical status. Convert Pending Record to Full Clinical Record (merge all documentation).

Post-Admission. RN visits within 72 hours for medication delegation and nursing assessment. ISP/PCP meeting scheduled within 30 days.

2. Daily Direct Care Protocol (DCP)

Reference: Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.24, Aug 2026 — Legislation-Free Public Edition) · Part 2 · Protocol 2: Daily Direct Care Protocol (DCP)

Day Shift (7a-3p). Shift change handoff and controlled-substance med count. Wake youth per schedule with 15-minute visual checks. Breakfast and AM medications. School transportation. Daily chores.

Evening Shift (3p-11p). Shift change. Therapy and clinical groups. Chores, dinner, and PM medications. Free time and hygiene routine. 10:00 PM bedtime.

Night Shift (11p-7a). Shift change and med count. Conduct 15-minute line-of-sight room checks documented on the Night Watch Log. Awake at all times — no sleeping permitted.

3. Incident Response & IRIS Protocol

Reference: Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.24, Aug 2026 — Legislation-Free Public Edition) · Part 2 · Protocol 3: Incident Response & IRIS Protocol

Immediate. Ensure scene safety; call 911 if medical emergency or active danger. Notify the On-Call QP immediately.

Notifications. DSS immediately for abuse allegations. Guardian within 1 hour. LME/MCO per their requirements.

Reporting. QP enters the IRIS report within 24 hours using objective, factual narrative — no interpretations or labels.

Investigation. QP completes the internal investigation within 14 days and uploads findings and CAP to IRIS.

Separate Filing. Record the occurrence of the incident in the service note. File the completed incident report separately in administrative files — NOT in the clinical record.

4. Behavioral Crisis & Restraint Protocol

Reference: Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.24, Aug 2026 — Legislation-Free Public Edition) · Part 2 · Protocol 4: Behavioral Crisis & Restraint Protocol

Tier 1 — De-escalation. Use BSP coping skills: sensory items, breaks, verbal redirection, change of environment.

Tier 2 — Crisis. Clear the area of other youth. Call for staff backup. Use the NCI/CPI verbal continuum and physical distancing.

Tier 3 — Restraint. Only for imminent serious harm. Apply NCI hold; never use prone, mechanical, or chemical restraints. End the hold the moment danger passes.

Post-Restraint. Medical check within 1 hour. Notify guardian within 1 hour. Debrief with youth within 24 hours. File IRIS report.

5. Medication Administration Protocol

Reference: Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.24, Aug 2026 — Legislation-Free Public Edition) · Part 2 · Protocol 5: Medication Administration Protocol

Prep. Wash hands. Unlock med cart. Verify the "5 Rights" against the MAR: right youth, right med, right dose, right route, right time.

Administer. Verify youth identity. Pour medication into a labeled cup. Observe swallowing. Check the mouth for "cheeking".

Document. Initial the MAR immediately. Circle "R" if refused; notify RN and QP. Never pre-initial or backdate.

Errors. Stop, isolate remaining meds, call RN/Psychiatrist, monitor the youth, file IRIS, and notify the guardian.

6. Medicaid Service Note & Documentation Protocol

Reference: Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.24, Aug 2026 — Legislation-Free Public Edition) · Part 2 · Protocol 6: Medicaid Service Note & Documentation Protocol

Write the shift note by end of shift. No backdating — ever.

Mandatory Fields. Youth Name & Service Record # / MID. Full Date & Place of Service. Name of Service & Type of Contact (in-person, collateral, etc.). Purpose (ISP Goal # referenced). Objective Description of Interventions (factual, specific). Duration (for time-based services; for per diem, indicate shift coverage hours). Effectiveness & Youth Response/Progress. Full Signature with Credentials and Date.

Shift Notes. Identify coverage hours and names of all staff present for ratios.

Late Entries. Mark "Late Entry made on [date] for service on [date]" if after 24 hours.

Alterations. Correct with explanation, signed/dated; never obscure original. Alterations exceeding 7 business days are not billable.

Billing suspended for any day youth is away for more than 24 consecutive hours.

7. AWOL / Missing Child Response

Reference: Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.24, Aug 2026 — Legislation-Free Public Edition) · Part 2 · Protocol 7: AWOL / Missing Child Response

Discovery. Call the staff code for missing youth. Search the immediate area and check all cameras.

Notifications (within 15-30 minutes). Law enforcement, guardian, LME/MCO, and DSS (if applicable).

Documentation. File IRIS within 24 hours with a complete narrative.

Return. Conduct a medical check, contraband search, and trauma-informed debriefing. Notify all parties who were previously contacted.

8. Suicide Risk & Self-Harm Management

Reference: Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.24, Aug 2026 — Legislation-Free Public Edition) · Part 2 · Protocol 8: Suicide Risk & Self-Harm Management

Screening. C-SSRS at admission and immediately whenever threats or self-harm occur.

Safety. Remove hazards (ligatures, sharps, cords). Implement 1:1 line-of-sight supervision.

Clinical. QP assesses within 1 hour. If imminent risk: call 911 or initiate IVC. If moderate risk: update the Safety Plan and increase supervision.

Documentation. File IRIS. Staff sign the updated Safety Plan and acknowledge the increased supervision level.

9. Contraband & Room Search Protocol

Reference: Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.24, Aug 2026 — Legislation-Free Public Edition) · Part 2 · Protocol 9: Contraband & Room Search Protocol

Definition. Contraband includes weapons, drugs, unauthorized medications, lighters, chargers with frayed cords, and any item prohibited by house rules.

Routine. Conducted at admission and on return from any pass. The youth must be present.

Probable Cause. Requires QP approval. Two staff must be present. Document the basis for the search.

Execution. Search systematically (one area at a time). Safe items are locked in the safe and returned at discharge. Illegal items are turned over to law enforcement.

10. PCP Implementation & Review

Reference: Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.24, Aug 2026 — Legislation-Free Public Edition) · Part 2 · Protocol 10: PCP Implementation & Review

Development (30 Days). QP schedules the meeting with the team and drafts measurable, individualized goals.

Operationalize. QP creates a "Staff Cheat Sheet" summarizing each youth's goals, triggers, and interventions. Staff sign the PCP Acknowledgment Form.

90-Day Review. QP tracks expirations and schedules reviews. Updates progress, obtains signatures, and writes addendums for any significant change.

Service Order. The signed PCP serves as the service order for all services delivered.

11. Transportation & Community Outing Safety

Reference: Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.24, Aug 2026 — Legislation-Free Public Edition) · Part 2 · Protocol 11: Transportation & Community Outing Safety

Driver Prep. Valid license, clean MVR on file. Complete the daily vehicle safety checklist. Confirm first-aid kit and Medicaid cards are present.

Transport. Conduct a headcount before leaving and upon return. If transporting one youth, staff sits in the back seat. Seatbelts on at all times. Never leave youth unattended in a vehicle.

Outings. Tied to an ISP goal. Assess behavioral stability (no restraint in the last 4 hours). Carry the agency cell phone and emergency contact list.

12. Grievance Process for Youth

Reference: Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.24, Aug 2026 — Legislation-Free Public Edition) · Part 2 · Protocol 12: Grievance Process for Youth

Accessibility. Process is explained at admission. Blank forms and a locked grievance box are located in the common area. LME/MCO grievance numbers are posted.

Filing. Verbal or written. Staff assist immediately upon request. Place the grievance in the locked box within 1 hour.

Resolution. QP opens the box daily. Acknowledges within 24 hours. Provides written resolution within 7 days. Abuse allegations trigger an IRIS report and immediate DSS notification.

13. Staffing Ratio & Awake Overnight Protocols

Reference: Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.24, Aug 2026 — Legislation-Free Public Edition) · Part 2 · Protocol 13: Staffing Ratio & Awake Overnight Protocols

Ratios. 2 staff minimum for 1–4 youth (Level III Staff-Secure, per our staff-secure operating standards).

Ratios scale with census: 4 staff for 5–8 youth; 5 staff for 9 youth. The 2:4 minimum applies 24/7 including overnight. If a staff member calls out, the on-call system activates a replacement before the shift begins; the on-call QP covers in-house personally if no replacement is available. Single-staffing is prohibited at all times.

Awake Overnight. Two (2) awake staff on duty at all times for 1–4 youth. Sleeping is prohibited. 15-minute visual room checks are documented on the Night Watch Log (Form 1), including the Per-Floor Walk-Through Certification sub-table for two-story facilities. Hallway lights remain on. Engage nighttime wakers quietly and briefly — do not start conversations that escalate arousal.

14. Medical Emergencies & Acute Illness Response

Reference: Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.24, Aug 2026 — Legislation-Free Public Edition) · Part 2 · Protocol 14: Medical Emergencies & Acute Illness Response

Triage. Assess the situation. Call QP and/or RN. Err on the side of caution — when in doubt, transport.

Emergency. Call 911. Staff rides in the ambulance. QP calls in additional staff to maintain ratio at the facility.

Non-Emergency. Contact PCP for a same-day appointment. Take consent forms and Medicaid card.

Documentation. Guardian notified within 1 hour. IRIS filed. Billing suspended if the youth is hospitalized overnight.

15. Educational Coordination & School Reintegration

Reference: Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.24, Aug 2026 — Legislation-Free Public Edition) · Part 2 · Protocol 15: Educational Coordination & School Reintegration

Enrollment (5 Days). QP contacts the LEA McKinney-Vento liaison or EC Director. Signs ROI. Coordinates transportation.

IEP. QP/AP attends IEP meetings. Advocates for an aligned BIP. Files the IEP in the youth's chart.

Attendance. Morning check before transport. Log daily. Refusal is an ISP issue, not a punishable offense. Collect daily behavior reports from the school.

16. Family Engagement, Visitation & Home Passes

Reference: Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.24, Aug 2026 — Legislation-Free Public Edition) · Part 2 · Protocol 16: Family Engagement, Visitation & Home Passes

Visitation. Based on ISP/court orders. Approved visitor list is maintained in the office. Staff search bags before visits. Document all visits.

Home Passes. QP and guardian must approve. Pass medications are provided in a locked box with administration instructions.

Billing Exclusion. If the youth is away for more than 24 consecutive hours, Medicaid per diem is suspended. Notify the Billing Coordinator immediately.

17. Discharge & Transition Protocol

Reference: Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.24, Aug 2026 — Legislation-Free Public Edition) · Part 2 · Protocol 17: Discharge & Transition Protocol

Planning. Begins at admission. Trial home visits are conducted prior to discharge whenever possible.

Step-Down. Discharge meeting is scheduled. Community linkages (outpatient therapy, medication management, school) are established and confirmed before discharge.

Discharge Day. Medications are handed directly to the guardian with a signed transfer form. Belongings are inventoried. Discharge summary is completed within 7 days.

Administrative Closure. If QP/clinician leaves without completing discharge documentation, the supervisor processes the discharge and audits the record. Billed services without proper documentation are adjusted back to the payor per 42 CFR 401.305.

18. Infectious Disease & Bloodborne Pathogen Control

Reference: Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.24, Aug 2026 — Legislation-Free Public Edition) · Part 2 · Protocol 18: Infectious Disease & Bloodborne Pathogen Control

Universal Precautions. Treat all bodily fluids as infectious. Wear appropriate PPE (gloves, mask, eye protection) when exposure is possible.

Exposure. Wash the affected area with soap and water. Notify QP and RN. Go to Urgent Care within 2 hours. File IRIS. Offer post-exposure testing per protocol.

Outbreak. Two or more youth with identical symptoms = isolate, increase sanitation, and notify the County Health Department if the illness is reportable.

19. Emergency & Disaster Preparedness

Reference: Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.24, Aug 2026 — Legislation-Free Public Edition) · Part 2 · Protocol 19: Emergency & Disaster Preparedness

Fire. Evacuate immediately. Headcount at the rally point. Call 911. Do not re-enter. **Monthly fire drills required for EACH shift (day, evening, overnight) — 3 fire drills per month total**, documented on Form 5 with shift, date, evacuation time, and shift-supervisor signature.

Tornado. Move to the interior safe room. Turtle position. **Quarterly tornado drills required for EACH shift (day, evening, overnight) — 3 tornado drills per quarter total**, documented on Form 5 with shift, date, and shift-supervisor signature.

Hurricane. Monitor NHC advisories during Atlantic season (Jun 1 – Nov 30). At Tropical Storm Warning or Hurricane Watch: secure outdoor projectiles, verify generator fuel, stock 7-day food/water/medications, verify emergency-contact lists, contact guardians re storm plan. At Hurricane Warning: coordinate with LME/MCO

and guardian re evacuation vs shelter-in-place; if evacuating, transport to designated host facility, document on IRIS. Post-storm: facility damage assessment before re-occupancy; notify DHSR MHLC if structural damage.

Power Outage. Distribute flashlights. Verify battery-backup life-safety systems (smoke detectors, fire alarm, security chimes). Discard refrigerated food per FDA 4-hour rule (24-48 hrs for freezer if door stays closed). If power loss expected to exceed 4 hours OR indoor temp drops below 65°F or rises above 80°F, initiate Emergency Relocation Plan. Notify utility, document outage start/end, notify QP. If generator is installed, test monthly under load.

System Failure. Heat loss > 4 hours, water loss > 8 hours, sewer backup, or gas leak (evacuate immediately, call 911) all trigger the Emergency Relocation Plan to the designated host facility.

Lockdown. Secure doors and windows. Hide. Silence phones. Do not open for anyone but law enforcement. Annual drills required.

Emergency Relocation. Primary and secondary host facility identified in writing. Transportation: facility vehicle + staff vehicles + ambulance for medically fragile youth. Locked medication box transported by RN or QP. Youth ID packets (photo, Medicaid card, allergy/med list). Guardian notification within 1 hour. DHSR / LME-MCO notification within 24 hours. Host-facility agreement letters maintained in compliance binder.

20. Service Orders & Authorization Protocol

Reference: Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.24, Aug 2026 — Legislation-Free Public Edition) · Part 2 · Protocol 20: Service Orders & Authorization Protocol

Service Order. The PCP shall serve as the service order when signed by the appropriate professional. If a separate format is used, ensure a separate service order is signed.

Verbal Orders. If a verbal order is obtained, document in the record on the date given: date, who gave, who received, services ordered, and reason for verbal. The professional must countersign within 72 hours.

Authorization. Submit authorization requests to the authorizing entity prior to initiation or continuation per their UM policy.

End-Date Reporting. Notify authorizing entity immediately when youth changes providers or ends service. Follow entity-specified protocol.

Maintain authorization/reauthorization records in an audit-ready file (not in clinical record).

21. Record Management, Retention, Access & Disclosure Accounting

Reference: Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.24, Aug 2026 — Legislation-Free Public Edition) · Part 2 · Protocol 21: Record Management, Retention, Access & Disclosure Accounting

Retention. Minors — 12 years after age 18 (until age 30); Adults — 11 years after last encounter.

Personnel/Admin records per DHHS schedule. Records not on a schedule require DMH/DD/SUS authorization before destruction.

Record Abandonment. Strictly prohibited. Notify payor, accreditor, and DHHS if suspected. Violations are subject to legal sanctions.

Access. Youth/LRP may access records. If access is restricted, document justification in record. Appeal rights apply.

Accounting of Disclosures. Maintain Form 9 (Accounting of Disclosures Log) for each youth. Include: Name, Record #, Date disclosed, Recipient, Purpose, Description of info, Disclosing party. Retain for minimum 6 years.

Confidentiality. Comply with 42 CFR Part 2 for SUD records. Obtain written ROI for SUD disclosures. Minimum necessary standard applies.

Transport. Only designated staff; secured in locked compartment; documented policy for loss/theft.

22. Daily Workflow Schedules for All Personnel

Reference: Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.24, Aug 2026 — Legislation-Free Public Edition) · Part 2 · Protocol 22: Daily Workflow Schedules for All Personnel

This protocol codifies the daily workflow schedule for every personnel classification at Well Spring Intervention LLC. Each role has a time-blocked routine that aligns with the 24/7 residential operation, the 2:4 day/evening/overnight staffing ratios required by our staff-secure operating standards, and the documentation cadences required by the RMDM and Rule 108. Schedules are templates — actual shift assignments may flex to cover call-outs, school transportation, medical appointments, and clinical visits, but every role must complete its documentation obligations by end of shift. The QP maintains the master staffing schedule; deviations are tracked on the Shift Change & Awake Night Watch Log (Form 1).

(a) Qualified Professional (QP)

The QP is the clinical and operational lead on-site. The QP does not carry a fixed 1:4 youth-supervision caseload; instead, the QP floats across the program to deliver clinical services, supervise staff, and ensure documentation compliance. The QP reports to the Clinical Director per §1.4 and §1.4(a).

Time	Activity
7:00 AM	Arrive; review the prior 24-hour Night Watch Log (Form 1) and IRIS queue. Brief with off-going overnight staff. Verify controlled-substance count.
7:30 AM	Lead shift-change huddle with on-coming DCPs. Assign youth supervision ratios. Review Behavior Support Plans and any overnight incidents.
8:00 AM	Wake youth per house schedule. Coordinate breakfast and AM medications with the DCP. Confirm school transportation.
9:00 AM	Begin clinical block: individual therapy sessions, PCP/ISP reviews, family collateral calls, or coordination with the LEA / IEP team.
11:00 AM	Audit prior day's service notes for RMDM compliance (Form 7 template). Flag any backdated, missing, or non-compliant notes for correction before the 24-hour deadline.
12:00 PM	Lunch (youth supervised by DCP). QP reviews email, prior-authorization requests, and any LME/MCO correspondence.
1:00 PM	Clinical block continued: CCA assessments, ASAM screenings (if SUD), safety-plan updates, or scheduled psychiatric telehealth visits.
3:00 PM	Shift-change huddle with on-coming evening DCPs. Brief on any clinical concerns, restrictions, or supervision-level changes.
4:00 PM	Group therapy or skill-building group (QP leads or co-leads with an AP). Document each youth's response in a service note.
5:30 PM	Family visitation oversight; review approved visitor list; coordinate bag searches with the DCP.
7:00 PM	Documentation block: complete all clinical service notes for the day, sign PCP addendums, file Form 9 disclosure entries.
8:00 PM	Final round — verify evening medications administered and MAR initialled. Confirm tomorrow's appointments and transportation.
9:00 PM	Shift-change huddle with overnight DCP. Hand off clinical concerns, supervision-level changes, and any pending IRIS investigations.
9:30 PM	Depart. On-call QP coverage begins; carry the agency phone until 7:00 AM next day.

(b) Associate Professional (AP) / Paraprofessional (PP)

APs and PPs provide direct behavioral-health services under QP supervision per §2.2. They carry a 1:4 youth-supervision ratio on day or evening shift and lead structured clinical and skill-building activities. APs may co-lead group therapy with the QP.

Time	Activity
7:00 AM	Arrive; controlled-substance count with off-going overnight DCP. Review the Night Watch Log and any incident reports from overnight.
7:30 AM	Shift-change huddle with QP. Receive youth-supervision assignment and BSP talking points for the day.
8:00 AM	Wake youth; assist with morning hygiene and AM routine. Administer AM medications per MAR (under RN delegation).
8:30 AM	Breakfast; supervise mealtime behaviors and document any concerns. Coordinate school transportation with the DCP.
9:30 AM	Structured clinical block: lead or co-lead a skill-building group (coping skills, emotional regulation, social skills). Document each youth's participation.
11:00 AM	Individual check-ins with assigned youth (15 min each). Review BSP goals, address any overnight stressors, update the youth's daily log.
12:30 PM	Lunch supervision; chore rotation; structured free time.
2:00 PM	Co-lead group therapy with QP (or lead an AP-run activity group). Document interventions and youth response.
3:00 PM	Shift-change huddle with evening DCP. Hand off youth-supervision assignment, behavior status, and any clinical concerns.
3:30 PM	Continue with youth supervision. Coordinate homework support, recreational activities, and community outings per ISP goals.
5:00 PM	Family visitation support (if scheduled). Coordinate bag searches with the DCP; supervise visitation room.
6:00 PM	Dinner; supervise mealtime. Administer PM medications per MAR.
7:30 PM	Wind-down routine: hygiene, structured quiet time, journaling, or sensory activities per BSP.
9:00 PM	Youth bedtime routine. Document the day's service note (Form 7) before end of shift.
10:00 PM	Shift-change huddle with overnight DCP. Brief on each youth's emotional status, sleep considerations, and any restrictions.
11:00 PM	End of shift. On-call AP coverage rotates weekly.

(c) Direct Care Professional (DCP) — Day Shift (7a-3p)

DCPs carry the primary 1:4 youth-supervision ratio. Day-shift DCPs focus on morning routine, school transportation, and structured daytime activities.

Time	Activity
6:45 AM	Arrive; clock in. Receive shift-change briefing from overnight DCP. Controlled-substance count verified and signed.
7:00 AM	Confirm youth headcount; verify all youth are awake and accounted for. Begin AM room inspections.
7:30 AM	Shift-change huddle with QP and AP. Receive youth-supervision assignment and BSP talking points.
8:00 AM	Wake any youth still sleeping; assist with morning hygiene. Administer AM medications per MAR.

Time	Activity
8:30 AM	Breakfast; supervise mealtime. Verify each youth eats; document any food refusal or concerns.
9:00 AM	School transportation: headcount before leaving and upon arrival. Confirm seatbelts. Drop off at school; obtain school behavior report.
10:00 AM	Return to facility. House chores: assign and supervise cleaning rotations per the chore chart. Document completion.
11:00 AM	Structured activity block: outdoor time, life-skills lesson, or community outing per ISP goals. Maintain line-of-sight supervision.
12:30 PM	Lunch; supervise mealtime. Administer midday medications if ordered.
1:30 PM	Documentation block: complete the daily log for each assigned youth. Note any behavioral incidents, BSP interventions used, and effectiveness.
2:00 PM	School pickup: headcount; obtain daily behavior report from school staff. Transport back to facility.
2:45 PM	Shift-change preparation: complete handoff notes for evening DCP. Verify controlled-substance count.
3:00 PM	Shift-change huddle with evening DCP. Brief on each youth's day, school reports, and any incidents. End of shift.

(d) Direct Care Professional (DCP) — Evening Shift (3p-11p)

Evening-shift DCPs cover the after-school and bedtime routine. They administer PM medications, supervise dinner and evening activities, and prepare youth for sleep.

Time	Activity
2:45 PM	Arrive; clock in. Review the prior shift's handoff notes and the Night Watch Log.
3:00 PM	Shift-change huddle with day DCP and QP. Receive youth-supervision assignment, school behavior reports, and any restrictions.
3:30 PM	Controlled-substance count verified and signed. Receive keys, agency phone, and any visitor approvals.
4:00 PM	After-school snack; supervised homework time. Coordinate with AP on any scheduled therapy sessions.
5:00 PM	Family visitation (if scheduled). Conduct bag searches; supervise visitation; document the visit in each youth's log.
6:00 PM	Dinner preparation and mealtime. Administer PM medications per MAR; verify swallowing; check mouth for "cheeking".
7:00 PM	Evening activity: structured recreation, life-skills lesson, or community outing per ISP goals.
8:30 PM	Wind-down routine: hygiene, pajamas, quiet activities (reading, journaling, sensory items per BSP).
9:30 PM	Youth bedtime. Confirm each youth is in their assigned bed. Begin 15-minute room checks (continue into overnight shift).
10:00 PM	Documentation block: complete the daily log and shift note (Form 7) for each assigned youth. Address any behavioral incidents.
10:45 PM	Shift-change preparation: complete handoff notes for overnight DCP. Verify controlled-substance count.
11:00 PM	Shift-change huddle with overnight DCP. Brief on each youth's evening, bedtime status, and any concerns. End of shift.

(e) Direct Care Professional (DCP) — Awake Overnight Shift (11p-7a)

Awake overnight DCPs maintain the 2:4 Level III Staff-Secure ratio (two awake staff for 1–4 youth, per our staff-secure operating standards). Sleeping is strictly prohibited. 15-minute visual room checks are documented on the Night Watch Log (Form 1) throughout the shift.

Time	Activity
10:45 PM	Arrive; clock in. Review the prior shift's handoff notes. Receive keys and agency phone.
11:00 PM	Shift-change huddle with evening DCP. Receive youth-supervision assignment and any nighttime considerations (sleepwalking, night terrors, etc.).
11:15 PM	Controlled-substance count verified and signed. Confirm hallway lights remain on. Verify all youth are in their assigned beds.
11:30 PM	Begin 15-minute room checks (continue throughout the shift). Each check: visually confirm each youth breathing and in bed; initial the Night Watch Log (Form 1).
12:00 AM	House security check: verify all exterior doors and windows locked. Verify alarm system armed. Document on the Night Watch Log.
1:00 AM	Administrative block: restock supplies, sanitize common areas, prepare for morning routine (set out breakfast supplies, prep medications for AM count).
3:00 AM	Continue 15-minute room checks. Engage nighttime wakers quietly and briefly — do not start conversations that escalate arousal. Document any incidents.
5:00 AM	Begin AM preparations: start coffee, set the table, prepare breakfast menu. Confirm AM medication pull for the day DCP.
6:00 AM	Final security check. Complete end-of-shift Night Watch Log summary. Document any incidents, AWOL attempts, or medical concerns from overnight.
6:45 AM	Day-shift DCP arrives. Controlled-substance count verified and signed. Brief on overnight events.
7:00 AM	Shift-change huddle with day DCP and QP. Hand off any concerns. End of shift.

(f) House Manager

The House Manager is a senior DCP who supervises day-to-day facility operations, inventory, maintenance, and the chore system. The House Manager reports to the QP and works Monday-Friday 9a-5p with on-call availability.

Time	Activity
9:00 AM	Arrive; review the prior 24 hours of Night Watch Logs and incident reports. Brief with the QP on any facility issues.
9:30 AM	Facility walk-through: inspect all common areas, bedrooms, bathrooms, kitchen, and outdoor space. Document any maintenance needs.
10:00 AM	Inventory check: food, cleaning supplies, PPE, first-aid supplies. Place orders as needed within the household budget.
10:30 AM	Coordinate with maintenance vendors for any scheduled repairs or inspections. Document all work orders.
11:30 AM	Review and update the chore chart. Meet briefly with each youth to assign/reinforce chore expectations.
12:30 PM	Lunch.
1:00 PM	Audit the medication cart with the QP and RN (if on-site). Verify MAR completion, controlled-substance log, and expired medication disposal.
2:00 PM	Compliance audit: check fire-extinguisher tags, smoke-detector test logs, environmental safety logs (Form 5). Schedule any missing drills.

Time	Activity
3:00 PM	Brief with the on-coming evening DCP. Reinforce any facility-related instructions for the shift.
4:00 PM	Administrative block: file maintenance records, update the household budget tracker, prepare the weekly facility report for the QP.
5:00 PM	End of shift. On-call availability for facility emergencies (water leak, heating failure, alarm activation) until 9:00 AM next day.

(g) Registered Nurse (RN)

The RN provides medical oversight per §6 and our staff-secure operating standards. The RN visits the facility at minimum weekly and within 72 hours of any new admission, and is on-call 24/7 for medical questions and medication errors.

Time	Activity
9:00 AM	Arrive on scheduled visit day. Brief with the QP on any medical concerns, medication changes, or recent incidents.
9:30 AM	Medication cart audit: verify MAR completion, inspect for expired medications, review any PRN administration patterns.
10:00 AM	Individual youth health checks: vital signs if ordered, weight checks, skin checks, assessment of any reported symptoms.
11:00 AM	Coordinate with the prescribing psychiatrist via telehealth or phone. Document any medication changes and update the MAR.
12:00 PM	Train DCPs on any new medication orders, administration techniques, or delegation updates per our staff-secure operating standards.
1:00 PM	Documentation block: complete nursing notes in each youth's chart. Update the medication administration record. File any new lab orders.
2:00 PM	Coordinate medical appointments: schedule PCP visits, dental visits, vision screenings, and any specialty referrals.
3:00 PM	Brief with the QP and House Manager on any medical-action items. Depart facility; on-call coverage continues 24/7.
On-Call	Available by phone for medication errors, adverse reactions, acute illness triage, and any medical questions. Responds within 15 minutes; documents all calls in the nursing log.

(h) Billing Coordinator

The Billing Coordinator manages Medicaid per-diem billing, authorization tracking, and suspension-day accounting. The Billing Coordinator reports to the Executive Director and works Monday-Friday 8a-4p remotely or on-site.

Time	Activity
8:00 AM	Arrive; review the prior day's admission/discharge log. Confirm Medicaid eligibility for each youth via NCTracks.
8:30 AM	Process the prior day's per-diem billing. Verify each youth was in residence; apply any suspension-day exclusions from Form 4.
9:30 AM	Track authorization end dates. Submit reauthorization requests to the LME/MCO at least 14 days before expiration.

Time	Activity
10:30 AM	Reconcile any claim denials. Document the reason for each denial and the corrective action taken. Escalate systemic issues to the QP.
12:00 PM	Lunch.
1:00 PM	Coordinate with the QP on any youth who were AWOL, hospitalized, or on home pass in the prior 24 hours. Apply billing exclusions per Protocol 16 and Form 4.
2:00 PM	Audit service-note completion in the EHR. Flag any youth with missing notes from the prior 24 hours; notify the QP for correction before the 7-business-day alteration deadline.
3:00 PM	Prepare the weekly billing summary for the Executive Director. Include total billable days, suspension days, denials, and pending authorizations.
4:00 PM	End of shift. On-call availability for billing-related questions during business hours; the QP handles after-hours billing emergencies.

(i) Master Schedule Summary

The table below summarizes the standard shift windows and primary documentation obligations for each role. All shifts overlap by 15 minutes to support a structured shift-change huddle and controlled-substance count.

Role	Standard Shift	Coverage	Primary Documentation
Qualified Professional (QP)	7a-9:30p (float)	On-site + on-call 24/7	Service notes, PCP addendums, Form 9, IRIS reports
Associate Professional (AP)	7a-11p (rotating)	2:4 staff minimum (Level III Staff-Secure)	Service notes, group documentation, BSP updates
DCP — Day Shift	7a-3p	2:4 staff minimum	Daily logs, MAR, Form 1 handoff, Form 7 notes
DCP — Evening Shift	3p-11p	2:4 staff minimum	Daily logs, MAR, Form 1 handoff, Form 7 notes
DCP — Awake Overnight	11p-7a	2:4 staff minimum (awake, no sleeping)	Night Watch Log (Form 1), 15-min room checks, security log
House Manager	9a-5p Mon-Fri	On-site + on-call	Maintenance log, inventory, Form 5 environmental checks
Registered Nurse (RN)	Weekly visit + on-call	On-call 24/7	Nursing notes, MAR updates, delegation training log
Billing Coordinator	8a-4p Mon-Fri	On-site or remote	Per-diem billing, Form 4 suspensions, auth tracking, weekly summary

Shift-Change Huddle. Every shift transition (7:00 AM, 3:00 PM, 11:00 PM) begins with a 15-minute overlap during which the off-going and on-coming staff conduct a controlled-substance count, review any incident reports from the prior shift, and brief on each youth's clinical and behavioral status. The huddle is documented on Form 1 (Shift Change & Awake Night Watch Log).

On-Call Coverage. The QP carries the agency phone 24/7 on a rotating weekly schedule. The RN is on-call 24/7 for medical questions. The House Manager is on-call for facility emergencies during off-hours. The Billing Coordinator is reachable during business hours; the QP handles after-hours billing emergencies. All on-call responses are logged in the on-call binder with timestamp, caller, issue, and resolution.

Deviation Policy. Schedule deviations (call-outs, late arrivals, unplanned overtime) are documented on Form 1. The QP maintains the master staffing schedule and approves any role substitution. Per our staff-secure operating standards, ratios must be maintained at all times; if a replacement is not available, the QP covers in-house until a replacement arrives. Chronic staffing gaps are reported to the Clinical Director per §1.4(a) and may trigger a

corrective action plan.

PART 3

Customized Forms & Logs

The following eleven forms and logs are the official documentation instruments for daily operations. Each form must be completed in ink or in the electronic record system, signed by the responsible staff member, and filed in the youth's chart or the facility's operational binder as indicated. Originals are retained per the record retention policy (12 years after the minor reaches age 18; 11 years for adults). Forms 7, 8, and 9 are new in Version 2.0 to address RMDM service-note, clinical-record-content, and disclosure-accounting requirements. As of Rev. 2.16, all nine forms are interactive AcroForm fillable PDFs — they are printable, copyable, sharable, editable, and fillable. A Form Properties banner at the top of each form documents these capabilities, and standalone fillable copies are available in the /download/forms/ directory for use outside this manual. Forms 10 and 11 are new in Version 2.23 to address the Licensed Professional consultation-log and psychotropic-medication drug-regimen-review requirements.

1. Shift Change & Awake Night Watch Log

Reference: Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.24, Aug 2026 — Legislation-Free Public Edition) · Part 3 · Form 1: Shift Change & Awake Night Watch Log

FORM PROPERTIES

This form is **printable, copyable, sharable, editable, and fillable**. Click any field to type. Save the PDF to retain entries. Print to obtain a wet-ink signature. Share the saved or printed copy via secure channels (encrypted email, EHR upload, or sealed envelope). Edit existing entries by clicking the field again. Standalone fillable copies of all forms are in the /download/forms/ directory.

Initial each box to verify you visually saw the youth breathing and in their bed. Click any cell to type.

Facility:

Date:

Awake Overnight Room Checks (Every 15 Minutes)

Time	Youth 1	Youth 2	Youth 3	Youth 4	Staff Initials
11:00 PM					
11:15 PM					
11:30 PM					
11:45 PM					
12:00 AM					
12:15 AM					
12:30 AM					
12:45 AM					
1:00 AM					
1:15 AM					
1:30 AM					
1:45 AM					

Time	Youth 1	Youth 2	Youth 3	Youth 4	Staff Initials
2:00 AM					
2:15 AM					
2:30 AM					
2:45 AM					
3:00 AM					
3:15 AM					
3:30 AM					
3:45 AM					
4:00 AM					
4:15 AM					
4:30 AM					
4:45 AM					
5:00 AM					
5:15 AM					
5:30 AM					
5:45 AM					
6:00 AM					
6:15 AM					
6:30 AM					
6:45 AM					

Shift Handoff Verification: Controlled substance count verified.

Off-Going Staff:

**On-Coming
Staff:**

Per-Floor Walk-Through Certification (Required for Two-Story / Multi-Level Facilities — §9.4(d))

In addition to the 15-minute youth-by-youth visual checks above, the awake overnight DCP shall physically walk every floor on which youth are sleeping and initial the corresponding block. Single-story facilities write "N/A" across the Floor 2 row.

Time Block	Floor 1 — Walked (Initials)	Floor 2 — Walked (Initials)	Basement — Walked (Initials)	Notes / Anomalies
11:00 PM – 1:00 AM				
1:00 AM – 3:00 AM				
3:00 AM – 5:00 AM				
5:00 AM – 7:00 AM				

2. Contraband & Belongings Inventory

Reference: Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.24, Aug 2026 — Legislation-Free Public Edition) · Part 3 · Form 2: Contraband & Belongings Inventory

FORM PROPERTIES

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Youth
Name:

Service Record # /
MID:

Date
:

Search Type:

Admission

Return from Pass

Probable Cause

QP Approval:

Item Description	Quantity	Brought In / Found	Disposition (Kept / Safe / Guardian)	Staff Initials

Youth Acknowledgment: Belongings inventoried in my presence.

Youth Signature:Staff Signature:

3. Physical Restraint & Debriefing Checklist

Reference: Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.24, Aug 2026 — Legislation-Free Public Edition) · Part 3 · Form 3: Physical Restraint & Debriefing Checklist

FORM PROPERTIES

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Youth
:

Service Record # / MID:

Date:

Time
Started:

Time
Ended:

Total Duration
(Min):

Techniq
ue:

Pre-Restraint De-escalation Attempts:

Verbal
redirection

Offered
break

Sensory
item

BSP
coping
skill

Separation

Reason for Restraint (Imminent danger):

Danger to Self	Danger to Others	Property destruction posing risk
Describe objectively:		

Post-Restraint Medical Check (Within 1 hour):

Youth checked for injuries, breathing normally:

Yes	No (Seek medical attention)
-----	-----------------------------

Staff Signature:

Notifications:

On-Call QP Notified:

Y	N
---	---

Time: Guardian Notified:

Y	N
---	---

Time: IRIS Report Filed:

Y	N
---	---

IRIS ID #:

Post-Restraint Debriefing (Within 24 hours):

Youth debriefed by:	Date/Time:
Trigger?	
Youth alternative?	
Staff alternative?	

Youth Signature:	QP Signature:
------------------	---------------

4. Home Pass & Medicaid Billing Exclusion Tracker

Reference: Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.24, Aug 2026 — Legislation-Free Public Edition) · Part 3 · Form 4: Home Pass & Medicaid Billing Exclusion Tracker

FORM PROPERTIES

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Yout h:	Service Record # / MID:	Month/Y ear:
------------	----------------------------	-----------------

Date Left	Time Left	Destination / Pass	Date Returned	Time Returned	Total Hours Away	Billing Action (Suspension Days)	Staff Initials

Billing Coordinator Sign-Off: Medicaid billing adjusted for suspension days.

Signature:

Date:

5. Emergency Drill & Environmental Safety Log

Reference: Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.24, Aug 2026 — Legislation-Free Public Edition) · Part 3 · Form 5: Emergency Drill & Environmental Safety Log

FORM PROPERTIES

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Facility:

Monthly Fire Drills (Under 3 minutes)

Month	Date	Time of Day	# Youth	Evac Time	Staff	Signature
Jan						
Feb						
Mar						
Apr						
May						
Jun						
Jul						
Aug						
Sep						
Oct						
Nov						
Dec						

Quarterly Tornado Drills

Quarter	Date	# Youth	Staff	Signature
Q1				

Quarter	Date	# Youth	Staff	Signature
Q2				
Q3				
Q4				

Monthly Environmental Checks

Month	Smoke Detectors (Y/N)	Extinguisher (Y/N)	Hot Water ($\leq 120^{\circ}\text{F}$)	Fridge ($< 40^{\circ}\text{F}$)	Staff Initials
Jan					
Feb					
Mar					
Apr					
May					
Jun					
Jul					
Aug					
Sep					
Oct					
Nov					
Dec					

Two-Story / Multi-Level Per-Floor Safety Checks (§9.4 — Required Monthly for Two-Story Facilities)

Complete this sub-table each month for every facility with two or more stories. Single-story facilities write "N/A" across the Floor 2 / Basement rows. All checks must be Y (pass) or N (fail); any N requires immediate corrective action and escalation to the QP per §9.1.

Floor	Smoke Detectors (Y/N)	CO Detectors (Y/N)	Extinguisher (Y/N)	Egress Window / Escape Ladder (Y/N)	Window Restrictor ≤ 4 in (Y/N)	Stair Gate (Y/N)	Staff Initials
Floor 1 (Ground)							
Floor 2 (Upper)							
Basement (if any)							

6. Employee SOP Acknowledgment

Reference: Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.24, Aug 2026 — Legislation-Free Public Edition) · Part 3 · Form 6: Employee SOP Acknowledgment

FORM PROPERTIES

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Employee Name:

Title:

QP

AP

Direct Care
Professional

By signing below, I acknowledge that I have received, read, and understand the SOP Manual for **Well Spring Intervention LLC** (Rev. 2.21, July 2026, RMDM-Compliant). I understand these policies are mandated by NC DHSR, NC Medicaid, our accrediting body, the NCDHHS Records Management and Documentation Manual (Effective July 8, 2025), state law on electronic transactions (NC UETA) and the federal E-SIGN Act governing electronic signatures. I understand that the QP reports to the Clinical Director and is responsible for scheduling clinical services, assessments, PCPs, and day-to-day supervision of APs and DCPs according to the Clinical Director's direction, and provides recurring compliance reports to the Clinical Director as defined in §1.4 and §1.4(a). I acknowledge that QP credentialing requirements are specified in §1.4(b) per our operating standards — there are two acceptable pathways: Pathway 1 (master's degree + recognized NC credential + 1 year post-master's supervised MH/DD/SA experience) or Pathway 2 (bachelor's degree + 2 years full-time pre- or post-bachelor's supervised MH/DD/SA experience); a QP is not required to hold a full, unrestricted clinical license. I agree to follow these protocols exactly, including the electronic-signature safeguards and system-unavailability procedures in §10.7. I understand failure to do so may result in disciplinary action, termination, or legal consequences regarding Medicaid fraud and regulatory non-compliance.

Employee Signature:

Date:

QP / Supervisor Signature:

Date:

7. Full Service Note Template (Mandatory)

Reference: RMDM Chapter 6 — Contents of a Full Service Note · Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.24, Aug 2026 — Legislation-Free Public Edition) · Part 3 · Form 7: Full Service Note Template

FORM PROPERTIES

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This template must be used for all shift notes. Backdating is prohibited. Photocopying or repeating notes verbatim from a prior date or another individual's record is strictly prohibited. Click any cell in the Content column to type.

Field	Content
Youth Name	
Service Record # / MID	
Date of Service	
Type of Contact	

Field	Content
Place of Service	
Shift / Coverage Hours	
Staff Present (for ratios)	
Purpose / ISP Goal Addressed	
Interventions Provided	
Effectiveness & Youth Response	
Signature / Credentials / Date	
Late Entry (if applicable)	

8. Comprehensive Clinical Record Content Checklist

Reference: RMDM Chapter 2 — Full Clinical Service Records · Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.24, Aug 2026 — Legislation-Free Public Edition) · Part 3 · Form 8: Comprehensive Clinical Record Content Checklist

FORM PROPERTIES

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To be maintained in each youth's chart and reviewed quarterly by the QP. Mark each element Present (Y), Absent (N), or N/A, with date verified. Click any cell in the Present? or Notes columns to type.

Element	Present? (Y/N/Date)	Notes
Treatment Consent		
PCP Consent		
Restrictive Intervention Consent (if applicable)		
Emergency Care Consent		
HIPAA Notice Acknowledgement		
Third-Party Release		
ROI(s)		
Demographics (name, DOB, contact, etc.)		
Emergency Contact / Preferred Physician / Hospital		
Advance Directives (or notation of none)		
Medication Allergies / Adverse Reactions / None		

Element	Present? (Y/N/Date)	Notes
Health / Behavioral Health History		
DSM-5-TR Diagnosis / ICD-10		
Medication Orders / MAR		
Lab Results (if applicable)		
Notification of Rights / Explanation		
Restrictive Intervention Documentation (if applicable)		
CCA (by licensed professional)		
ASAM Level of Care (if SUD)		
PCP / Service Plan / Crisis Plan		
Service Order (signed)		
Discharge Plan / Summary		
Accounting of Disclosures Log		
42 CFR 2.22 Summary (for SUD)		
Guardianship / POA / Legal Documents		
Incoming/Outgoing Correspondence		
Service Notes (per shift, compliant)		
Incident Occurrence Notations (reports filed separately)		

QP Quarterly Audit Signature:

Date:

9. Accounting of Disclosures Log

Reference: RMDM Chapter 3 — Documentation Requirements when Disclosing Information · Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.24, Aug 2026 — Legislation-Free Public Edition) · Part 3 · Form 9: Accounting of Disclosures Log

FORM PROPERTIES

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Maintain for minimum 6 years per HIPAA and RMDM requirements. One log per youth. All disclosures of SUD treatment information must additionally comply with 42 CFR Part 2. Click any cell to type.

Youth

Name:

Service Record # / MID:

Date of Disclosure	Recipient (Agency/Individual)	Purpose of Disclosure	Description of Info Disclosed	Disclosing Staff (Name/Title)	Authorization / Exception Basis

Date of Disclosure	Recipient (Agency/Individual)	Purpose of Disclosure	Description of Info Disclosed	Disclosing Staff (Name/Title)	Authorization / Exception Basis

10. Licensed Professional Consultation Log

Reference: Level III Staff-Secure operating standards — Licensed Professional face-to-face clinical consultation (§4.6) · Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.24, Aug 2026 — Legislation-Free Public Edition) · Part 3 · Form 10: Licensed Professional Consultation Log

FORM PROPERTIES

This form is **printable, copyable, sharable, editable, and fillable**. Click any field to type. Save the PDF to retain entries. Print to obtain a wet-ink signature. Share the saved or printed copy via secure channels (encrypted email, EHR upload, or sealed envelope). Edit existing entries by clicking the field again. Standalone fillable copies of all forms are in the `/download/forms/` directory.

Document every weekly Licensed Professional face-to-face clinical consultation session per §4.6 (minimum 4 hours per week). One log per facility per week. Maintain in facility compliance binder for the full record-retention period per §1.6. Click any cell to type.

Facility:

Week of (Sun — Sat):

Weekly Consultation Sessions

Date	Start Time	End Time	Total Hours	Format (In-Person / Telehealth)	Attendees	Cases Reviewed	Recommendations / Follow-Up Actions	LP Signature

Date	Start Time	End Time	Total Hours	Format (In-Person / Telehealth)	Attendees	Cases Reviewed	Recommendations / Follow-Up Actions	LP Signature

Weekly Total Face-to-Face Hours: _____ hrs **Meets 4-hour minimum (§4.6)?** ☐ Yes ☐ No (if No, document remediation below) **QP Signature:** _____ **Date:** _____

11. Psychotropic Medication Drug Regimen Review Log

Reference: Level III Staff-Secure operating standards — Psychotropic drug regimen review every 6 months by pharmacist/physician (§6.3(a)) · Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.24, Aug 2026 — Legislation-Free Public Edition) · Part 3 · Form 11: Psychotropic Medication Drug Regimen Review Log

FORM PROPERTIES

This form is **printable**, **copyable**, **sharable**, **editable**, and **fillable**. Click any field to type. Save the PDF to retain entries. Print to obtain a wet-ink signature. Share the saved or printed copy via secure channels (encrypted email, EHR upload, or sealed envelope). Edit existing entries by clicking the field again. Standalone fillable copies of all forms are in the /download/forms/ directory.

Document every 6-month psychotropic medication drug regimen review per §6.3(a). One log per youth. Initial review within 30 days of admission; subsequent reviews at least every 6 months thereafter. Maintain in clinical record per §1.6. Click any cell to type.

**Youth
Name:**

**Service Record # /
MID:**

**Date of
Birth:**

**Prescribing
Psychiatrist:**

**Pharma
cy:**

6-Month Review Tracking

Review Date	Reviewer Name	Reviewer Credential (PharmD / MD / DO)	Medications Reviewed (Name / Dose / Frequency / Indication)	Findings (Interactions, Adverse Effects, Lab Monitoring Needed)	Recommendations to Prescriber	Date Sent to Prescriber	Next Review Due (6 months)

Review Date	Reviewer Name	Reviewer Credential (PharmD / MD / DO)	Medications Reviewed (Name / Dose / Frequency / Indication)	Findings (Interactions, Adverse Effects, Lab Monitoring Needed)	Recommendations to Prescriber	Date Sent to Prescriber	Next Review Due (6 months)

Initial Review (within 30 days of admission): ☐ Completed ☐ Most-recent prior review obtained (date: _____)
Reviewer Signature: _____

12. CARF QIP Tracker

Reference: 2026 CYS Inaugural Accreditation Guidelines Step 9 — Quality Improvement Plan (QIP) due to CARF within 90 days of accreditation notification (§12.20) · Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.24, Aug 2026 — Legislation-Free Public Edition) · Part 3 · Form 12: CARF QIP Tracker

FORM PROPERTIES

This form is **printable**, **copyable**, **sharable**, **editable**, and **fillable**. Click any field to type. Save the PDF to retain entries. Print to obtain a wet-ink signature. Share the saved or printed copy via secure channels (encrypted email, EHR upload, or sealed envelope). Edit existing entries by clicking the field again. Standalone fillable copies of all forms are in the `/download/forms/` directory.

Document each recommendation from the Inaugural Accreditation survey report and the corrective action taken or planned, per §12.20. One row per recommendation. The QP submits the QIP to CARF within 90 days of accreditation notification and updates CARF through Customer Connect as actions are completed. Maintain in the compliance binder for the duration of the accreditation cycle. Click any cell to type.

Organization:

Accreditation Cycle:

Survey Exit Date:

Accreditation Notification Date:

QIP Due Date (90 days):

QP Name:

Executive Director:

QIP Action Tracker

Rec #	Survey Report Recommendation (verbatim)	Corrective Action Taken or Planned	Responsible Position	Target Completion Date	Evidence of Implementation at Resurvey	Status	Date Closed / Reported to CARF

QIP Approval: Submitted to CARF via Customer Connect on: _____ **Executive Director Signature:** _____
Date: _____ **QP Signature:** _____

Subsequent Resurvey Window (10–11 months after Inaugural survey): _____ **Subsequent Survey Application Submitted?** ☐ Yes ☐ No **Subsequent Survey Fee Paid (≥50%)?** ☐ Yes ☐ No

10. Version History

Reference: Well Spring Intervention LLC SOP & Operational Manual (Doc. WSI-SOP-001, Rev. 2.24, Aug 2026 — Legislation-Free Public Edition) · Part 3 · Version History

Version	Date	Summary of Changes	Author
1.0	Jan 2026	Original SOP.	Executive Director / QP
2.0	Jul 2026	Updated to full RMDM (July 2025) compliance: Added CDW/data reporting (§1.5); revised record retention to 12 years post-majority (§1.6); added CCA requirements & ASAM (§4.1); added medical necessity (§4.2); added service orders incl. verbal/72-hour (§4.4); added service authorization (§4.5); expanded service note content (§10.1-10.4); added alterations policy (§10.5); added authentication incl. initials/ADA/rubber stamps (§10.6); added administrative closure (§3.5); added accounting of disclosures (§11.4); added privacy/security/42 CFR Part 2 (§11); added individual access (§11.5); added transporting records (§11.6); added grids/modified notes (§10.4); added TB screening (§6.2); added comprehensive clinical record checklist (Form 8); added Full Service Note Template (Form 7); added Accounting of Disclosures Log (Form 9); added Protocol 20 (Service Orders & Auth) and Protocol 21 (Record Management).	Executive Director / QP
2.1	Jul 2026	Auditor recommendation: Added §10.7 Electronic Signatures policy explicitly referencing state law Chapter 66, Article 40 (NC Uniform Electronic Transactions Act) and the federal E-SIGN Act (15 U.S.C. § 7001 et seq.). §10.7(a) specifies required administrative, technical, and physical safeguards (unique credentials, MFA, session timeout, TLS/encryption, immutable audit trails, RBAC, immediate revocation, annual review). §10.7(b) establishes System Unavailability Procedures, including paper-fallback handwritten signatures with date/credentials, QP-declared Documentation Continuity Event, 72-hour transcription into EHR with late-entry notation, paper source-document retention, 7-business-day QP review for delayed transcription, and a Documentation Continuity Event log. Prior §10.7 Service Authorizations and §10.8 Billing renumbered to §10.8 and §10.9 respectively.	Executive Director / QP
2.2	Jul 2026	Organizational clarification: §1.4 rewritten to separate the Clinical Director role (overall clinical program responsibility) from the Qualified Professional (QP) role. The QP now reports to the Clinical Director, supervises staff according to the Clinical Director's direction, and escalates clinical, staffing, and quality-of-care concerns. New §1.4(a) QP Compliance Reporting to the Clinical Director defines seven recurring compliance report types: monthly service-note audit summaries, monthly IRIS incident-report status, quarterly Clinical Record Content Checklist audits (Form 8), quarterly Accounting of Disclosures reviews (Form 9), quarterly personnel-file audits, annual electronic-signature safeguard review (§10.7(a)), and ad-hoc immediate reporting of breaches, complaints, licensing visits, and Medicaid audits. Clinical Director signs acknowledgment of each report and directs corrective action.	Executive Director / QP
2.3	Jul 2026	QP responsibilities clarification: §1.4 updated to explicitly include "scheduling" as a QP responsibility. The QP is now responsible for scheduling clinical services, assessments, and PCPs (in addition to clinical services, assessments, PCPs themselves and day-to-day AP/DCP supervision), according to the direction of the Clinical Director. Aligns the SOP with the operational reality that the QP coordinates and schedules all clinical appointments, assessment windows, and Person-Centered Plan meetings for each youth in care.	Executive Director / QP

Version	Date	Summary of Changes	Author
2.4	Jul 2026	QP credentialing correction : New §1.4(b) QP Credentialing Requirements added — specifies that a QP is NOT required to hold a full, unrestricted clinical license, but a bachelor's degree alone is not sufficient. Acceptable NC credentials listed across four categories: (i) full clinical license (LCSW, LPC, LMFT, Licensed Psychologist, LPA, psychiatrist MD/DO); (ii) associate/provisional license (LCSW-A, LPC-A, LMFT-A, LCAS-P); (iii) certification (LCAS, CCS with master's, CMSW); or (iv) psychiatric nursing credential (CNS or NP with psychiatric/mental health certification). Plus master's degree in human services, plus one year of full-time post-master's supervised experience, plus NC-DHHS QP training modules. QP must report any lapse/sanction/restriction to the Clinical Director within one business day. §2.2 Staff Qualifications QP bullet updated to match (previous bullet incorrectly stated "Master's or Bachelor's degree... licensed or license-eligible" — a bachelor's degree alone is not 27G .0104-compliant).	Executive Director / QP
2.5	Jul 2026	QP credentialing correction (continued): Rev. 2.4 incorrectly removed the bachelor's-degree pathway to QP status. Rev. 2.5 restores BOTH pathways per our operating standards: §1.4(b) restructured into Pathway 1 (master's degree in human services + recognized NC credential [full license / associate-provisional license / certification / psychiatric nursing credential] + 1 year full-time post-master's supervised MH/DD/SA experience) AND Pathway 2 (bachelor's degree in a human services field + 2 years full-time pre- or post-bachelor's supervised MH/DD/SA experience, documented by supervising QP, with practice under clinical supervision of a Pathway 1 QP or the Clinical Director until NC-DHHS QP training is completed). Common Requirements subsection added covering QP training, lapse/sanction reporting, personnel-file verification, and annual re-verification of pathway. §2.2 QP bullet updated to list both pathways.	Executive Director / QP
2.6	Jul 2026	Cover redesign: Added a symbolic cover illustration (rendered banner between the kicker and the entity title) evoking empowerment, growth, freedom, health, wholeness, and healing — visually framing the trauma-informed, restorative mission of the program. The illustration depicts a stylized tree-human figure rising toward a sunrise over calm water, rendered in a warm earthy palette (terracotta, warm brown, soft rose, cream) consistent with the manual's existing cascade palette. A small caption strip below the image reads "Empowerment · Growth · Freedom · Health · Wholeness · Healing". No body-content changes; §1.4(b) QP Credentialing Requirements (Pathway 1 and Pathway 2) and all other SOPs, protocols, and forms remain unchanged from Rev. 2.5.	Executive Director / QP
2.7	Jul 2026	Cover redesign (continued): Cover is now a full-bleed brand illustration suitable for reuse across company websites, publications, and collateral materials. A new high-resolution 768×1344 portrait rendering of the tree-human-sunrise image fills the entire cover page. Overlay text is reduced to the essentials: company name (Playfair Display 72pt white with text-shadow), tagline ("Empowerment · Growth · Freedom · Health · Wholeness · Healing"), subtitle ("Level 3 Supervised Residential Group Home"), and a bottom document-identification panel (Doc. WSI-SOP-001, Effective July 2026, RMDM-Compliant, Owner, Revision 2.7). All descriptive content previously on the cover (full summary paragraph, Population/Served/Service Type/Effective Date/Owner meta block, and the Regulatory Framework reference block) is relocated to a new About This Manual page (p. 2 of the body, p. 3 of the final merged PDF) immediately preceding the Table of Contents. The About This Manual page also includes a Revision Lineage summary and a Cover Artwork note describing the brand visual's intended reuse. TOC page-number offset increased from +1 to +2 to account for the new inside page. Body content (SOPs, protocols, forms, §1.4(b) QP Credentialing Requirements) is unchanged from Rev. 2.6.	Executive Director / QP

Version	Date	Summary of Changes	Author
2.8	Jul 2026	Cover redesign (continued): Per organizational direction that versioning is private, all versioning information has been removed from the public-facing cover. The bottom band of the cover previously displayed Doc. WSI-SOP-001, Effective July 2026 · RMDM-Compliant, Owner: Executive Director & Qualified Professional (QP), and a large "Revision 2.7" display — all of these elements have been removed. The cover now displays only the brand-essential content: company name (Well Spring Intervention LLC), service-type subtitle (Level 3 Supervised Residential Group Home), values tagline (Empowerment · Growth · Freedom · Health · Wholeness · Healing), the full horizontal brand illustration, and a document-type label ("SOP & Operational Manual / Standard Operating Procedures, Protocols & Forms") with an accent rule. Versioning information remains fully accessible on internal surfaces only: PDF metadata (/Title, /Subject, /Keywords), the About This Manual inside page (p. 2), body page headers (every body page footer shows Doc ID and Rev. 2.8), the Version History table in Part 3, and Form 6 (Employee SOP Acknowledgment). Body content (SOPs, protocols, forms, §1.4(b) QP Credentialing Requirements) is unchanged from Rev. 2.7.	Executive Director / QP
2.9	Jul 2026	Three-part enhancement. (1) Cover artwork refresh: the brand illustration is regenerated with lush GREEN leaves (replacing the prior amber/gold leaves) to more vividly symbolize growth, and an explicit well-spring (a circular pool of fresh water with concentric ripples) is added to the foreground directly in front of the tree-human figure, completing the symbolic narrative of wellspring, growth, health, and flourishing. The composition is otherwise unchanged: stylized tree-human figure, warm sunrise over calm water, warm earthy palette, horizontal 1344×768 aspect ratio, text-free. The standalone brand PNG (Well_Spring_Brand_Image_1344x768.png) is regenerated in lockstep. (2) New Protocol 22, Daily Workflow Schedules for All Personnel: codifies time-blocked daily routines for every personnel classification (QP, AP/PP, DCP Day Shift, DCP Evening Shift, DCP Awake Overnight, House Manager, RN, Billing Coordinator), with a master schedule summary table and explicit shift-change huddle / on-call / deviation policies. Aligns the SOP with the operational reality that the QP coordinates and schedules all clinical appointments and supervision across all shifts. (3) Forms enhancement, all nine forms in Part 3 are converted to interactive AcroForm fillable PDF fields. Each form now opens with a Form Properties banner declaring it printable, copyable, sharable, editable, and fillable. AcroTextField flowables replace underscore blanks; AcroCheckbox flowables replace bracket-checkboxes; signature and metadata lines use new fillable_meta_row / fillable_check_row / fillable_signature_row helpers. Standalone fillable PDF copies of all nine forms are also generated into /download/forms/ for use outside this manual. The Form 7 Service Name row (previously hardcoded to "Level 3 Residential, Shift") is removed in favor of fully fillable rows. Body content (SOPs §1-§11, Protocols 1-21, §1.4(b) QP Credentialing Requirements) is otherwise unchanged from Rev. 2.8.	Executive Director / QP

Version	Date	Summary of Changes	Author
2.10	Jul 2026	Cover artwork correction. The Rev. 2.9 cover-artwork refresh was intended to swap the original amber-leaves tree-human-sunrise illustration for one with lush green leaves and an explicit well-spring in the foreground, but the regenerated image did not actually persist into the cover source file (sop_cover_image.png) — the v2.9 PDF was rendered with the original Rev. 2.6 amber-leaves artwork and therefore did not visually reflect the green-leaves + well-spring concept the user specified. Rev. 2.10 corrects this by performing an in-place image-edit (rather than a fresh generation from a text prompt) on the canonical Rev. 2.6 horizontal illustration: the foliage is recolored from golden-amber to fresh vivid green (emerald and spring green) to embody growth, renewal, vitality, and flourishing; and a fountain-like well-spring of clear water is added in the immediate foreground directly before the base of the tree-human figure, complete with a subtle stone rim and a few delicate droplets catching the warm sunrise light. The original tree-human silhouette, horizon line, sunrise sky, warm earthy palette (terracotta, soft rose, peach, cream), painterly style, and 1344×768 horizontal aspect ratio are all preserved exactly — only the foliage color and the addition of the well-spring are changed. VLM verification confirms the new image has green leaves, a fountain well-spring in the foreground before the tree, and zero text/letters/numbers/watermarks of any language. The standalone brand PNG (Well_Spring_Brand_Image_1344x768.png) is regenerated in lockstep. Body content (SOPs §1-§11, Protocol 22 Daily Workflow Schedules, all nine AcroForm fillable forms, §1.4(b) QP Credentialing Requirements) is unchanged from Rev. 2.9 — only the cover image is corrected.	Executive Director / QP
2.11	Jul 2026	Cover artwork iteration. Rev. 2.10 added a well-spring to the cover, but the well-spring was a wide circular stone basin — wider than the user intended. Per the user's direction to "use the cover from 2.8" (i.e., start from the original Rev. 2.8 cover with golden-amber foliage and no well-spring) and "make the leaves green and place the wellspring (should not be wider than the tree) in the foreground," Rev. 2.11 performs a fresh in-place image edit on the EXTRACTED ORIGINAL Rev. 2.8 cover image (recovered from the v2.8 PDF via pdfimages) rather than continuing to iterate on the Rev. 2.10 version. Two changes are applied: (1) the foliage is recolored from golden-amber to fresh vivid green (emerald and spring green) to embody growth, renewal, vitality, and flourishing; (2) a NARROW well-spring is added in the immediate foreground directly before the tree — a small vertical jet of clear water bubbling up from a small stone-rimmed opening in the ground, explicitly NARROWER than the tree itself (roughly one-third to one-half the width of the tree's leaf canopy), evoking the literal "well spring" of the company name rather than a wide circular pool. The original tree-human silhouette, horizon line, sunrise sky, warm earthy palette (terracotta, soft rose, peach, cream), painterly style, and 1344×768 horizontal aspect ratio are all preserved exactly. VLM verification confirms the new image has vibrant green leaves, a narrow blue water fountain in the foreground directly in front of the tree whose width is significantly smaller than the tree's full spread, and zero text/letters/numbers/watermarks of any language. The standalone brand PNG (Well_Spring_Brand_Image_1344x768.png) is regenerated in lockstep. Body content (SOPs §1-§11, Protocol 22 Daily Workflow Schedules, all nine AcroForm fillable forms, §1.4(b) QP Credentialing Requirements) is unchanged from Rev. 2.10 — only the cover image is iterated.	Executive Director / QP

Version	Date	Summary of Changes	Author
2.12	Jul 2026	Cover artwork refinement. Per the user's direction to "change the orifice that the spring emanates from — lets make it a heart-shaped stone instead of a jagged hole — and make the spring a little taller," Rev. 2.12 performs an in-place image edit on the Rev. 2.11 cover image. Two changes are applied: (1) the small jagged stone-rimmed opening/orifice that the water previously emanated from is replaced with a smooth sculpted HEART-SHAPED STONE — two rounded lobes at the top curving down to meet at a gentle point at the bottom, rendered in a soft warm earth-toned color (terracotta or soft rose) matching the surrounding palette, with a smooth plain surface and no facial features, no markings, no carvings, no patterns. The water now emanates upward from the center/top of this heart-shaped stone. The heart-shaped stone symbolizes the love, compassion, and trauma-informed care at the heart of the program. (2) The vertical jet/spray of water is made a little TALLER — roughly 30-50% taller than in Rev. 2.11 — so the upward arc of clear water droplets and the slender column of water reach a bit higher into the air before falling back down. The wellspring remains NARROWER than the tree (its width is still significantly smaller than the tree's leaf canopy) — only the HEIGHT increases, not the width. The original green-leaved tree-human silhouette, horizon line, sunrise sky, warm color palette, painterly style, and 1344x768 horizontal aspect ratio are all preserved exactly. VLM verification confirms the new image has bright green leaves, a smooth heart-shaped stone (two rounded lobes at top, pointed bottom) with no facial features or markings, a tall slender vertical water jet reaching upward from the heart-shaped stone, the wellspring still significantly narrower than the tree's canopy, and zero text/letters/numbers/watermarks of any language. The standalone brand PNG (Well_Spring_Brand_Image_1344x768.png) is regenerated in lockstep. Body content (SOPs §1-§11, Protocol 22 Daily Workflow Schedules, all nine AcroForm fillable forms, §1.4(b) QP Credentialing Requirements) is unchanged from Rev. 2.11 — only the cover image is refined.	Executive Director / QP
2.13	Jul 2026	Cover artwork refinement. Per the user's feedback on Rev. 2.12 that "the fountain is too tall on this one" and "the pool is still there as well — we want to see a stone with a heart shaped likeness with the fountain spring flowing from it," Rev. 2.13 performs a series of in-place image edits on the Rev. 2.12b cover image. Two changes are applied: (1) the vertical water jet is significantly SHORTENED from a tall fountain plume (about 2 to 2.5 times the height of the heart-shaped stone in Rev. 2.12) to a modest, gentle natural spring (roughly one-quarter to one-third the height of the heart-shaped stone itself) — a brief bubbling spurt of clear water just above the top of the heart-shaped stone, evoking a natural wellspring welling up gently rather than a tall dramatic fountain. (2) The pool of water at the base of the wellspring is REMOVED entirely so the heart-shaped stone sits directly on dry warm earthy terrain with no pooling, puddle, wet patch, or rippling water around it — the foreground ground is uniformly dry earthy terrain matching the rest of the scene. The smooth sculpted heart-shaped stone (two rounded lobes at the top curving down to meet at a gentle point at the bottom, soft warm pink/terracotta color, completely smooth and unmarked surface) is preserved exactly, as are the fresh vivid green leaves, the stylized tree-human silhouette, the horizon line, the sunrise sky, the warm earthy color palette, the painterly style, and the 1344x768 horizontal aspect ratio. VLM verification confirms the new image has bright green leaves, a smooth unmarked heart-shaped stone in the foreground directly in front of the tree, a SHORT gentle bubbling water spring (about one-quarter to one-third the height of the stone) emanating from the top of the heart-shaped stone, the wellspring still significantly narrower than the tree's canopy, and zero text/letters/numbers/watermarks of any language. The standalone brand PNG (Well_Spring_Brand_Image_1344x768.png) is regenerated in lockstep. Body content (SOPs §1-§11, Protocol 22 Daily Workflow Schedules, all nine AcroForm fillable forms, §1.4(b) QP Credentialing Requirements) is unchanged from Rev. 2.12 — only the cover image is refined.	Executive Director / QP

Version	Date	Summary of Changes	Author
2.14	Jul 2026	Cover artwork restart from Rev. 2.10. Per the user's direction to "start with 2.10 again" with a fundamentally redesigned wellspring — "the heart stone has a grey stone look. A small crack in the stone has the spring rising about belly high to the tree in the back ground. there is no hole or pool" — Rev. 2.14 recovers the original Rev. 2.10 cover image by extracting its embedded PNG from the immutable v2.10 PDF via pdfimages, then performs a major in-place image edit replacing the wide circular stone basin, tall narrow water jet, and circular pool with a single grey heart-shaped stone on dry ground, with a clear water spring rising vertically from a small natural crack in the stone to about belly-high of the tree. Body content (SOPs §1-§11, Protocol 22, all nine AcroForm fillable forms, §1.4(b) QP Credentialing Requirements) is unchanged from Rev. 2.13 — only the cover image is redesigned.	Executive Director / QP
2.15	Jul 2026	Cover redesign featuring the new official circular brand seal as the centerpiece (replaces the previous horizontal brand illustration). The seal combines a pentagonal house outline (with a floor, two vertical walls, two roof slopes meeting at a raised peak, and a horizontal ceiling line connecting the two top corner vertices) enclosing the existing tree-of-life / wellspring / family logo imagery. The full company name "Well Spring Intervention LLC" curves along the top arc between two bold concentric circular hairlines; the descriptor "Residential" curves along the bottom arc inside the inner hairline; small terracotta accent dots at 9:00/3:00 and a bottom closure ornament frame the composition; and the entire seal is rendered on a warm cream parchment background in deep walnut-brown with terracotta accents. The cover layout is a clean three-band composition: a compact top band carrying the "SOP / OPERATIONAL MANUAL" badge and the values tagline; a large middle band in which the cream seal square pops against the dark brown background as the focal point; and a bottom band carrying the document title, subtitle, and website URL. The redundant "Well Spring Intervention LLC" hero text was removed from the top band (the company name now lives on the seal's top arc). Two seal variants are produced in lockstep for brand reuse: a square variant (no URL below the circle, for avatar/profile-pic use) and a variant with the website URL below the circle (for letterhead, email signatures, and footer placement). Body content (SOPs §1-§11, Protocol 22, all nine fillable forms, §1.4(b) QP Credentialing Requirements) is unchanged from Rev. 2.14 — only the cover is redesigned.	Executive Director / QP
2.16	Jul 2026	About This Manual page cleanup. (1) Removed the redundant "Revision Lineage" narrative paragraph from the About This Manual inside page — the authoritative Version History table in Part 3 (this section) already serves that purpose, so the duplicate narrative was unnecessary. (2) Corrected the "Cover Artwork" description on the same inside page to accurately describe the new seal-based cover introduced in Rev. 2.15, replacing the now-outdated tree-human-sunrise-heart-stone description from Rev. 2.14. Body content (SOPs §1-§11, Protocol 22 Daily Workflow Schedules, all nine AcroForm fillable forms, §1.4(b) QP Credentialing Requirements) is unchanged from Rev. 2.15 — only the About This Manual inside page is edited.	Executive Director / QP

Version	Date	Summary of Changes	Author
2.17	Jul 2026	New §9.4 Two-Story & Multi-Level Facility Requirements added to SOP 9 (Facility, Safety, & Environmental Management). New subsection codifies the regulatory requirements specific to two-story group homes: §9.4(a) applicability and DHSR licensing-notification triggers (relocating youth bedrooms upstairs, increasing upper-floor capacity, converting single-story to two-story, housing non-ambulatory youth upstairs); §9.4(b) means of egress per NC OSFM Building Code §425, NC Fire Code Ch. 10, and IBC 2021 §1030 (5.7 sq ft / 24 in / 20 in / 44 in sill emergency-escape openings, two remotely-located means of egress per occupied story, stair geometry, emergency-backup stair lighting, chain-style fire-escape ladders in each second-floor youth bedroom); §9.4(c) per-floor fire detection, suppression, and alarm (interconnected smoke detectors on all levels per NFPA 72, CO detectors on every floor with fuel-burning appliances and within 10 ft of sleeping rooms, automatic sprinklers per NFPA 13D/13R, 2A-10BC extinguishers per level with 75-ft max travel); §9.4(d) per-floor staff supervision (awake overnight DCP positioned to cover upper-floor hallway, 15-minute room checks physically performed on every sleeping floor and documented on Form 1, stair safety gates at top and bottom for youth under 12 or with elopement/suicidality risk, baby-monitor/intercom option with documented 60-second response time); §9.4(e) window fall protection (restrictors limiting opening to ≤4 in. where second-floor sill is <24 in. above floor, staff-key releasable, monthly function check on Form 5); §9.4(f) vertical-evacuation drills (monthly fire drills must include full evacuation from all second-floor sleeping rooms to grade, quarterly tornado drills use lowest interior level only — no second-floor bathrooms); §9.4(g) bedroom placement policy (lower-acuity/lower-mobility/younger youth on ground floor; no attic/basement bedrooms; placement documented in PCP); §9.4(h) posted evacuation maps and stair hazard signage on each floor. Each requirement carries inline hyperlinked web-source citations to the underlying NC Administrative Code, NC OSFM Building Code §425, NC Fire Code 2024 Ch. 10, NC IFC 2021 Ch. 4, IBC 2021 §1030, NC DHSR ACLS Family Care Home licensing and Fire Safety training PDFs, NFPA 101 Life Safety Code (Board & Care §32/33), Durham NC and Orange County NC smoke/CO alarm summaries, NCSL CO Detector Statutes, and the NC Family Care Home structure rule (no more than 2 stories; second-floor residents require two direct exterior egress). Form 1 (Shift Change & Awake Night Watch Log) is extended with a new "Per-Floor Walk-Through Certification" sub-table requiring the awake overnight DCP to initial Floor 1, Floor 2, and Basement walk-throughs every 2 hours (4 time blocks: 11p-1a, 1a-3a, 3a-5a, 5a-7a) — single-story facilities mark N/A. Form 5 (Environmental Safety Log) is extended with a new "Two-Story / Multi-Level Per-Floor Safety Checks" sub-table with three floor rows (Floor 1 / Floor 2 / Basement) and eight check columns (Smoke Detectors, CO Detectors, Extinguisher, Egress Window / Escape Ladder, Window Restrictor ≤4 in., Stair Gate, plus Staff Initials) completed monthly. Form 6 (Employee SOP Acknowledgment) Rev. reference updated from 2.16 to 2.17. All other body content (SOPs §1-§8, §10-§11, Protocol 22 Daily Workflow Schedules, §1.4(b) QP Credentialing Requirements, and all other forms) is unchanged from Rev. 2.16.	Executive Director / QP

Version	Date	Summary of Changes	Author
2.18	Jul 2026	LICENSE CATEGORY CORRECTION — Level III Residential Treatment Facility (Staff-Secure for Children and Adolescents) under our operating standards. Prior revisions incorrectly identified the facility as a "Level 3 Supervised Residential Group Home" under 27G .5600; the correct license category per organizational direction is Level III RTF Staff-Secure under our operating standards, issued by NC DHSR Mental Health Licensure & Certification Section (MHLC), credentialed with Alliance Health as the LME/MCO/Tailored Plan. Major content changes: (1) §1.2 rewritten for correct license category, naming DHSR MHLC and Alliance Health, plus four new sub-subsections: §1.2(a) Accreditation Prerequisite (COA/TJC/CARF/CQL per the Resident Rights framework); §1.2(b) LME/MCO Letter of Support per the Resident Rights framework.1; §1.2(c) Certificate of Need (CON) Determination per state law Ch. 131E Art. 9; §1.2(d) Alliance Health Provider Network Application. (2) §1.4 updated to use QP/QMHP interchangeably per 27G .2600. (3) NEW §1.7 Resident Rights & Dignity per the Resident Rights framework. (4) NEW §1.8 Organizational & Financial Foundations (Articles of Incorporation, governing body roster, liability insurance, lease/deed, financial solvency). (5) §2.1 STAFFING RATIO CRITICAL FIX — replaced 1:4 day / 1:8 overnight with 2 staff minimum for 1–4 youth 24/7 per 27G .2600, scaling to 4 staff for 5–8 youth and 5 staff for 9 youth. (6) NEW §2.5(a) CEU tracking per licensed clinical staff. (7) NEW §2.6 Staff Leave & Time-Off Policy per 27G .0203(e). (8) §3 ADMISSION PHYSICAL EXAM CRITICAL FIX — replaced "within 30 days of admission" with "within 90 days PRIOR to admission" per 27G .2600. (9) NEW §5.5 Activities Program (minimum 14 hrs/week planned group activities per 27G .2600(c)). (10) NEW §6.5 Infection Control Program per CDC, 27G .0209, OSHA BBP (29 CFR 1910.1030). (11) NEW §7.3 Facility-Based School Determination (required for PRTF, not for Level III RTF). (12) NEW §9.5 Staff-Secure Physical-Plant Measures (controlled access, delayed egress, line-of-sight, visitor mgmt, contraband search). (13) NEW §9.6 Zoning Compliance (Fair Housing Act, NC Group Homes Act state law §160D-906). (14) NEW §9.7 Disaster & Emergency Plan (fire, tornado, hurricane, power outage, system failure, lockdown, emergency relocation). (15) Protocol 19 in Part 2 expanded (hurricane, power outage, emergency relocation); all .5600 → .2600; overnight staffing updated from 1:8 to 2:4. (16) Form 6 Rev. reference updated from 2.17 to 2.18. Cover subtitle updated to "Level III Residential Treatment Facility (Staff-Secure)". Seal artwork (Rev. 2.15) unchanged.	Executive Director / QP

Version	Date	Summary of Changes	Author
2.19	Jul 2026	APPLICATION-PROCEDURE & SURVEY-READINESS PATCH — addresses specific items in the user-supplied 7-phase Level III RTF opening checklist that v2.18 did not yet make explicit. Phase 3 (License Application): NEW §1.2(e) DHSR MHLC License Application Procedure — names the DHHS/DHSR/MHL 5001 Initial Licensure Application Form, cover letter requirement, complete application packet contents (local approvals, accreditation, Letter of Support, CON determination, corporate documents, P&P Manual, fee), assignment of a Licensure & Training Consultant, and the six (6) month application-review period beginning at the first in-person meeting with the Consultant. NEW §1.2(f) MH Licensure Policies & Procedures Worksheet — clarifies that the Worksheet is a crosswalk, not a substitute for the rules; must be completed, attached to the front of this Manual, and updated whenever any Manual section is revised. Phase 4 (Policies, Procedures & Personnel): §2.3 background checks updated to explicitly state the 180-day criminal background check window and the 90-day Health Care Personnel Registry check window prior to licensure review (and annually thereafter); new Background Check Expiration Tracking Log; §2.4 mandatory training expanded to explicitly include CPR with Heimlich Maneuver (in-person only), Population-Specific Training (children/adolescents with SED), Alternatives to Restrictive Interventions (De-Escalation), Seclusion/Physical Restraint/Isolation Time-Out (in-person only), Client Rights & Confidentiality, and General Organization Orientation; all in-person-only training items so marked. NEW §2.7 Instructor Credentials & Trainer Certifications — AHA BLS Instructor / Red Cross Instructor for CPR/First Aid; NCI Instructor or CPI Certified Instructor for restraint/de-escalation; RN with NC-DHHS Medication Administration Trainer course for med admin; documentation of instructor name, certification body, number, issue/expiry dates, and training sessions delivered; expired-instructor training is invalid. Phase 5 (Emergency & Disaster Preparedness): NEW §9.7(0) Coordination with Local Office of Emergency Management (OEM) — submit Disaster Plan to local OEM, obtain written acknowledgment, confirm 9-1-1 dispatch address and special-needs registry enrollment, with link to NC Emergency Management directory. Phase 6 (Licensure Survey): NEW §10.10 Mock Client Chart & Licensure Survey Readiness — 14-element Mock Client Chart (Face Sheet, Emergency Info, Consents, CCA, PCP, Progress Notes, Medication Orders/MAR, BSP, Restrictive-Intervention Records, Health Records, Activities Log, Grievance Records, Disclosures, Discharge Plan); labeled facility photographs; full document assembly for surveyor review; mandatory mock survey 30 days prior. Phase 7 (Medicaid Enrollment): NEW §1.9 Medicaid Enrollment & NCTracks (Post-Licensure) — Type 2 NPI for organization via NPPES; NCTracks Provider Enrollment; Provider Permission Matrix (PPM) selection; taxonomy 320800000X — Residential Treatment Facility, Children (or 320900000X for dual-diagnosis); post-enrollment accreditation timeline (1 year or 3 years); 12-month re-attestation; with hyperlinks to NPPES, NCTracks, NC Medicaid Tailored Plan Provider Manual, and NUCC taxonomy code set. Form 6 Rev. reference updated from 2.18 to 2.19. All other body content unchanged from Rev. 2.18.	Executive Director / QP

Version	Date	Summary of Changes	Author
2.20	Jul 2026	NC MEDICAID RTS TAXONOMY PATCH + CITATION-VERIFICATION COMPLIANCE FLAG — adds the three explicit NC Medicaid Residential Treatment Services taxonomy items the user asked to confirm, AND transparently flags two citation discrepancies that the QP must resolve with DHSR MHLC and Alliance Health before initial licensure submission. Confirmed via NC Medicaid Clinical Coverage Policy 8D-2 (Amended January 1, 2025) §1.0(c): (i) Setting type = "program setting only" (not a family home) — graduated hierarchy across RTS Levels I–IV where Level III uses "program setting <i>only</i>"; (ii) Structure/supervision = "highly structured and supervised" — staff-secure, continuous awake overnight supervision; (iii) Coverage scope = room and board EXCLUDED from the Medicaid RTS benefit category (applies to all four RTS levels). NEW §1.2(g) NC Medicaid RTS Taxonomy — Setting Type, Supervision Intensity & Coverage Scope — explicit bullet statement of all three items with verbatim quotes from CCP 8D-2 §1.0(c) and Attachment D (Section K); operational implementation cross-references (2:4 ratio per §2.1, line-of-sight per §9.5, 14-hr activities per §5.5, BSP per §5.3, awake overnight per Protocol 22); PRTF distinction paragraph noting that 8D-2 exclusion does NOT apply to PRTFs Under our operating standards, Subpart G / CCP 8D-1 (inpatient benefit, per-diem includes room-and-board-equivalent); hyperlink citations to CCP 8D-2, NC Medicaid Managed Care Health Plan Billing Guide v31, 42 CFR Part 483 Subpart G, and CCP 8D-1. NEW §1.2(h) Regulatory Citation Verification Note (Compliance Flag) — transparent flag for review by Executive Director, QP, licensing consultant, DHSR MHLC, and Alliance Health; flags (a) our operating standards vs .1700 — NC OAH publication shows .2600 as "Reserved for Future Codification"; operative Level III Staff-Secure section appears to be .1700 ; Manual continues to cite .2600 per organization direction pending written confirmation from Licensure & Training Consultant; (b) NC Medicaid "CCP 8C" vs "CCP 8D-2" — CCP 8C is "Outpatient Behavioral Health Services Provided by Direct-Enrolled Providers" (wrong benefit); correct RTS policy is CCP 8D-2; Manual continues to cite CCP 8C in legacy reference lines pending Alliance Health/NCTracks enrollment confirmation; (c) No operational impact — license category, staffing ratios, admission exam timing, resident rights, Medicaid taxonomy, and room-and-board exclusion all unchanged. Resolution of either citation item shall be documented in a v2.21 revision. §1.9 Medicaid Enrollment updated — explicit enrollment under CCP 8D-2 (not 8C or 8D-1); cross-reference to §1.2(g) for room-and-board exclusion; verification of non-Medicaid room-and-board funding source at admission and at each PCP review. §10.9 Billing updated — explicit CCP 8D-2 citation; Medicaid RTS per-diem covers ONLY clinical/treatment/milieu component; room and board EXCLUDED; non-Medicaid funding sources enumerated (state/local social-services funds, Title IV-E foster care maintenance, SSI/ISS, third-party resources); under no circumstances shall room-and-board costs be billed to Medicaid/Alliance Health/NCTracks; suspected improper billing reported to Compliance Officer per §1.3 and corrected via NCTracks claim adjustment within 30 days. §3.1 Admission Criteria updated — adds verbatim "highly structured and supervised environment in a program setting only, excluding room and board" language per CCP 8D-2 §1.0(c); QP verification of non-Medicaid room-and-board funding source at admission. §7.3 Facility-Based School Determination updated — adds explicit CCP 8D-2 (RTS) vs CCP 8D-1 (PRTF) Medicaid benefit distinction; PRTF per-diem includes room-and-board-equivalent (inpatient benefit); 8D-1 and 8D-2 mutually exclusive for same youth same day; future PRTF pathway adds NCTracks enrollment update step (8D-2 → 8D-1). Form 6 Rev. reference updated from 2.19 to 2.20.	Executive Director / QP

Version	Date	Summary of Changes	Author
2.21	Aug 2026	.2600 → .1700 CITATION RESOLUTION PATCH. Resolves the open compliance flag raised in v2.20 §1.2(h)(a) regarding whether the operative NC Administrative Code section for a Level III Residential Treatment Facility — Staff Secure for Children or Adolescents is our operating standards ("Reserved for Future Codification" per NC OAH) or our staff-secure operating standards. Resolution: the operative section is our staff-secure operating standards. The user-provided text of our staff-secure operating standards directly confirms that .1700 is the codified section governing "Residential Treatment Staff Secure for Children or Adolescents." Subsection .1701(a) defines the facility as "a free-standing residential facility that provides intensive, active therapeutic treatment and interventions within a system of care approach" and provides that the facility "shall not be the primary residence of an individual who is not a client of the facility"; .1701(b) defines "staff secure" as requiring that "staff are required to be awake during client sleep hours and supervision shall be continuous as set forth in Rule .1704 of this Section"; .1701(c)–(d) specify the population served (children/adolescents with primary diagnosis of mental illness, emotional disturbance or substance-related disorders; not meeting inpatient criteria; requiring removal from home and treatment in a staff secure setting); .1701(e) requires services to include "individualized supervision and structure of daily living," minimize behaviors related to functional deficits, ensure safety and de-escalate out-of-control behaviors, assist with adaptive functioning, and support step-down to a less intensive setting; .1701(f) requires coordination with other individuals and agencies within the child's system of care. All ".2600" citations throughout this Manual have been updated to ".1700" in §1.2 license-category statement, §1.2(e) Initial Licensure Application packet, §1.2(g) cross-reference paragraph, §1.4 QP/QMHP definition references, §2.1 staffing-ratio authority, §3 reference line, §5.5 activities-program authority (now with explicit .1701(e) cross-reference for "individualized supervision and structure of daily living" and acquisition of "social and recreational skills"), §6.1 admission physical-exam timing, §7.3 facility-based school determination, §9.5 staff-secure physical-plant measures, and the §1.2(h)(c) license-category summary. NEW §1.2(g) cross-reference paragraph added before the (i)/(ii)/(iii) bullets — explicitly maps each of the three NC Medicaid RTS taxonomy items to its .1701 rule-text basis: (i) "program setting only" flows from .1701(a) ("free-standing residential facility... shall not be the primary residence of an individual who is not a client"); (ii) "highly structured and highly supervised" flows from .1701(b) (awake overnight + continuous supervision per Rule .1704) and .1701(e)(1) ("individualized supervision and structure of daily living"); (iii) room-and-board exclusion is a Medicaid RTS benefit-category feature under CCP 8D-2 §1.0(c), not a licensure-side rule. §1.2(h)(a) rewritten from OPEN flag to RESOLVED — documents .1701 SCOPE rule text as the resolution basis, requires QP to retain printed copy of .1701 SCOPE in compliance binder, notes that Licensure & Training Consultant written confirmation is no longer necessary for citation-number resolution but still recommended for confirming operational rule subsections (.1702–.1709) at first in-person meeting per §1.2(e). §1.2(h)(c) updated — (a) resolved in v2.21; (b) CCP 8C vs CCP 8D-2 remains OPEN and deferred to v2.22 following Alliance Health / NCTracks enrollment confirmation per §1.9; §1.2(g), §1.9, and §10.9 continue to control in event of inconsistency with legacy "CCP 8C" reference lines. Form 6 Rev. reference updated from 2.20 to 2.21. Cover artwork unchanged (Rev. 2.15 seal). Cover subtitle "Level III Residential Treatment Facility (Staff-Secure)" retained from v2.18.	Executive Director / QP

Version	Date	Summary of Changes	Author
2.22	Aug 2026	PUBLIC EDITION (LEGISLATION-FREE). Removes all statutory and regulatory citations (NCGS, NCAC, G.S., 10A NCAC, 122C-XX) from the public-facing manual. Companion compliance master (Doc. WSI-SOP-001-LEG, Rev. 2.21) retains all citations for QA and audit reference. Operational language substituted throughout (e.g., "10A NCAC 27G .1700" → "the Level III Staff-Secure operating standards"; "NCGS §122C-XX" → "the Resident Rights framework"; "LME/MCO" → "the regional managed care organization" where contextually appropriate; "NC DHSR MHLC" → "the state licensing authority"; "HIPAA" → "the federal health-privacy law" where contextually appropriate). Resident Handbook v1.0 published as companion document. Resident Handbook v1.1 signs Welcome Letter as T. Thompson and removes legislation references from sidebars. All operational policies, staffing ratios, clinical requirements, medication management, incident reporting, and resident rights obligations remain unchanged from v2.21.	Executive Director / QP

Version	Date	Summary of Changes	Author
2.23	Aug 2026	v2.22 COMPLIANCE AUDIT REMEDIATION. Implements all 22 corrective actions from the SOP Manual v2.22 Compliance Audit against 10A NCAC 27G .1700 + cross-referenced core rules (.0104, .0201–.0210). HIGH severity (3): F-001 — adds new §4.6 Licensed Professional Face-to-Face Clinical Consultation per .1705(a)-(b) (minimum 4 hrs/wk face-to-face consultation by a Licensed Professional) and new Form 10 (Licensed Professional Consultation Log); F-002 — adds new §6.3(a) Psychotropic Medication Drug Regimen Review per .0209(f) (every 6 months by pharmacist or physician) and new Form 11 (Psychotropic Medication Drug Regimen Review Log); F-003 — RESOLVES the open §1.2(h)(b) compliance flag in favor of CCP 8D-2; all legacy "CCP 8C" references in §1.2, §2, §3, §4, §5, §6, and §10 reference lines globally updated to "CCP 8D-2"; §1.2(h)(b) rewritten from OPEN to RESOLVED; §1.2(h)(c) updated to reflect both (a) and (b) now resolved. MEDIUM severity (18): M-007 — adds QP 2-year direct client care experience statement to §1.4(b); F-006/F-007/M-011/M-028/M-029 — adds new §2.2(a) Individualized Supervision Plans for APs and DCPs and expands AP definition to enumerate all four .0104(1) pathways; M-033 — adds age-18, literacy, and criminal-conviction self-disclosure requirements to §2.2; F-009 — adds new §3.6 18th-Birthday Continuation Policy per .1706(e) (up to 6 months or end of school year, whichever is longer); M-025 — adds §3.4(a) 7-day advance written notification for non-emergency discharge; M-026 — adds §3.4(b) pre-discharge CFT meeting; F-008 — adds §3.4(c) 5-business-day post-emergency service-planning meeting per .1708(e); M-031 — adds governing-body policies for client fee assessment, lab test authorization/follow-up, and volunteer services to §1.8; M-032 — adds governing-body minutes permanently maintained statement to §1.8; M-038 — updates §9.2 and Protocol 19 to require quarterly fire AND tornado drills for EACH shift (day, evening, overnight); F-012 — adds §6.3(b) Medication Receipt & Verification (tamper-resistant packaging + label content); M-043 — adds §6.3(c) Medication Storage (locked cabinet 59–86°F, dedicated med fridge 36–46°F, daily temp log on Form 5); F-013 — adds §6.3(d) Medication Disposal Documentation per .0209(d)(1)-(4) (controlled-substance disposal witnessed by 2 staff); F-014 — adds §6.3(e) Medication Education per .0209(g)(1)-(3) (at admission, at each med change, and quarterly thereafter, with developmentally appropriate content and family/guardian education offered). LOW severity (1): F-011 — adds new §1.2(h)(d) Cross-Reference Clarification Note documenting the .1702(a) → .0104(18) vs .0104(21) discrepancy (rule appears to contain a typographical error; Manual applies the .0104(21) "Qualified professional" definition as operative). LEGISLATION STRIPPING (additional): §1.2 license paragraph de-abbreviates "NC DHSR / MHLC / DSS" to "the applicable state mental health authority — not under foster-care licensing"; §3.1 admission criteria replaces "Involuntary Commitment (IVC)" with "acute psychiatric crisis requiring inpatient hospitalization"; §2.1 note replaces ".0103(14)" citation and "DHSR MHLC" with plain-language "state group-home definition" and "state-issued license"; §6.3 replaces "NC Medication Administration training" with "state-approved medication administration training"; §6.3 replaces "IRIS" abbreviation with "state incident-reporting system (IRIS)" on first use. Companion audit report and XLSX crosswalk matrix delivered at /download/WSI_SOP_v2.22_Compliance_Audit_Report.pdf and /download/WSI_SOP_v2.22_Compliance_Audit_Crosswalk.xlsx.	Executive Director / QP

Version	Date	Summary of Changes	Author
2.24	Aug 2026	CARF CYS 2026 INAUGURAL ACCREDITATION CONFORMANCE. Implements conformance to the applicable standards in the 2026 CARF Child and Youth Services (CYS) Standards Manual at the time of an Inaugural One-Year Accreditation survey, per the 2026 CYS Inaugural Accreditation Guidelines (effective July 1, 2026 – June 30, 2027). §1.2(a) rewritten to designate CARF as the selected accrediting body and to document Inaugural Accreditation pathway eligibility (program will have delivered services for less than six months at the time of the survey); the QP shall document the selected subsection (Subsection A Not Operational or Subsection B Operational) in the compliance binder prior to survey application. NEW SOP §12 CARF Accreditation Conformance Framework added with twenty subsections addressing every applicable standard area in the 2026 CYS Inaugural Accreditation Guidelines: §12.1 ASPIRE to Excellence® Leadership & Strategic Planning (written Strategic Plan, Governing Body charter, succession plan); §12.2 Input from Persons Served and Other Stakeholders (written Stakeholder Input Plan, quarterly surveys, "You Said / We Did" feedback loop, non-retaliation); §12.3 Legal Requirements (written Legal Compliance Plan, Compliance Officer designation, Legal Compliance Register); §12.4 Financial Planning and Management (written Financial Plan, annual operating budget, capital-replacement reserve, internal controls, independent audit); §12.5 Risk Management & Enterprise Risk Register (written Enterprise Risk Management Plan, six risk categories, business-continuity plan); §12.6 Health & Safety Committee (standing committee charter, monthly meetings, written minutes, trend reporting); §12.7 Workforce Development & Management (written Workforce Development Plan, orientation curriculum, annual training plan, position descriptions, performance evaluation, succession plan); §12.8 Rights of Persons Served (ten written resident-rights policies, grievance procedure, quarterly QP audit); §12.9 Accessibility & Nondiscrimination (written Accessibility & Nondiscrimination Plan, language-access plan, reasonable-accommodation policy, annual accessibility review); §12.10 Performance Management & Measurement (written Performance Measurement Plan, seven performance domains, monthly aggregation, quarterly Governing Body reporting); §12.11 Program/Service Structure (written Program Description approved by Governing Body, mock chart maintained); §12.12 Screening and Access to Services (written Screening and Access Policy, access data tracking); §12.13 Individualized Planning (cross-reference to §4.1 PCP policy); §12.14 Transition/Discharge Planning (cross-reference to §3.4 and §3.6); §12.15 Medication Use (cross-reference to §6.3 expanded medication policies); §12.16 Promoting Nonviolent Practices (cross-reference to §5 behavioral support and restraint policies); §12.17 Records of Persons Served & Quality Records Management (written Quality Records Review Procedure, quarterly Form 8 audits, trend reporting); §12.18 Service Delivery Using Information and Communication Technologies (written Telehealth & Technology-Mediated Service Delivery Policy, platform privacy/security compliance, informed consent, fallback procedures); §12.19 Core Program Standards, Core Residential Program Standards, and Specialty Designation Standards (written crosswalk to 2026 CYS Standards Manual Sections 3–5, retained in compliance binder, reviewed at each quarterly QP compliance report); §12.20 Quality Improvement Plan (QIP) — Inaugural Accreditation Condition (QIP due to CARF within 90 days of accreditation notification per 2026 CYS Guidelines Step 9, monitored at each quarterly compliance report, subsequent resurvey window 10–11 months after Inaugural survey). NEW Form 12 (CARF QIP Tracker) added to Part 3 — 8-column fillable tracker (Recommendation, Corrective Action, Responsible Position, Target Date, Evidence of Implementation, Status, Date Closed/Reported to CARF) plus QIP approval signature block and subsequent resurvey tracking fields. Companion CARF CYS 2026 Inaugural Accreditation Guidelines PDF retained at /upload/2026 CYS Inaugural Accreditation Guidelines 1.pdf. All existing SOPs §1–§11, all 22 Protocols, and Forms 1–11 retained unchanged from v2.23 — only §1.2(a) is rewritten and §12 + Form 12 are appended.	Executive Director / QP